Application Submitted to HRSA

Submitted to HRSA

Organization: CINCINNATI UNIV OF, CINCINNATI, Ohio

Grants.gov Tracking Number: GRANT14708020

EHB Application Number: 247763

Grant Number: 6 T73MC00032-34-03

Funding Opportunity Number: HRSA-26-019

Received Date: 7/10/2026 9:58:37 AM

A printer version document only. The document may contain some accessibility challenges for the screen reader users. To access same information, a fully 508 compliant accessible HTML version is available on the HRSA Electronic Handbooks. If you need more information, please contact HRSA contact center at 877-464-4772, 8 am to 8 pm ET, weekdays.

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OMB Number: 4040-0001
Expiration Date: 01/31/2029

APPLICATION FOR FEDERAL ASSISTANCE

SF 424 (R&R)

3. DATE RECEIVED BY STATE

State Application Identifier

1. TYPE OF SUBMISSION

☐ Pre-application ☒ Application ☐ Changed/Corrected Application

4. a. Federal Identifier T73MC00032

b. Agency Routing Identifier

c. Previous Grants.gov Tracking ID

2. DATE SUBMITTED

Applicant Identifier

5. APPLICANT INFORMATION

UEI: DZ4YCZ3QSPR5

Legal Name: UNIVERSITY OF CINCINNATI

Department: COM Pediatrics

Division: Sponsored Research Services

Street1: Digital Futures Rm 610, 3080 Exploration Ave.

Street2: PO Box 210222

City: Cincinnati County / Parish: Hamilton

State: OH: Ohio Province:

Country: USA: UNITED STATES ZIP / Postal Code: 45206-0222

Person to be contacted on matters involving this application

Prefix: First Name: David Middle Name:

Last Name: Gearring Suffix:

Position/Title: Director, Grants and Contracts

Street1: Digital Futures Rm 610, 3080 Exploration Ave.

Street2: PO Box 210222

City: Cincinnati County / Parish: Hamilton

State: OH: Ohio Province:

Country: USA: UNITED STATES ZIP / Postal Code: 45206-0222

Phone Number: 5135565969 Fax Number: 5135564346

Email: ospaward@uc.edu

6. EMPLOYER IDENTIFICATION *(EIN) or (TIN):* 31-6000989

7. TYPE OF APPLICANT: H: Public/State Controlled Institution of Higher Education

Other (Specify):

Small Business Organization Type ☐ Women Owned ☐ Socially and Economically Disadvantaged

8. TYPE OF APPLICATION:

☐ New ☐ Resubmission

☒ Renewal ☐ Continuation ☐ Revision

If Revision, mark appropriate box(es).

☐ A. Increase Award ☐ B. Decrease Award ☐ C. Increase Duration ☐ D. Decrease Duration

☐ E. Other (specify):

Is this application being submitted to other agencies? Yes ☐ No ☒ What other Agencies?

9. NAME OF FEDERAL AGENCY: Health Resources and Services Administration

10. ASSISTANCE LISTING NUMBER: 93.877

ASSISTANCE LISTING TITLE: Autism Collaboration, Accountability, Research, Education, and Support

11. DESCRIPTIVE TITLE OF APPLICANT'S PROJECT:

UNIVERSITY AFFILIATED CINCINNATI CENTER FOR DEVELOPMENTAL DISORDERS

12. PROPOSED PROJECT:

Start Date: 07/01/2026 Ending Date: 06/30/2031

13. CONGRESSIONAL DISTRICT OF APPLICANT

OH-001

14. PROJECT DIRECTOR/PRINCIPAL INVESTIGATOR CONTACT INFORMATION

Prefix: | First Name: Jennifer | Middle Name:

Last Name: Smith | Suffix:

Position/Title: Professor

Organization Name: Cincinnati Children's Hospital Medical Center

Department: Developmental and Behavioral Pediatrics

Division:

Street1: 3333 Burnet Avenue

Street2:

City: Cincinnati | County / Parish: Hamilton

State: OH: Ohio | Province:

Country: USA: UNITED STATES | ZIP / Postal Code: 45229-3026

Phone Number: 513-803-2365 | Fax Number:

Email: jennifer.smith4@cchmc.org

15. ESTIMATED PROJECT FUNDING

a. Total Federal Funds Requested	3,670,000.00
b. Total Non-Federal Funds	0.00
c. Total Federal & Non-Federal Funds	3,670,000.00
d. Estimated Program Income	0.00

16. IS APPLICATION SUBJECT TO REVIEW BY STATE EXECUTIVE ORDER 12372 PROCESS?

a. YES ☐ THIS PREAPPLICATION/APPLICATION WAS MADE AVAILABLE TO THE STATE EXECUTIVE ORDER 12372 PROCESS FOR REVIEW ON:

DATE:

b. NO ☒ PROGRAM IS NOT COVERED BY E.O. 12372; OR

☐ PROGRAM HAS NOT BEEN SELECTED BY STATE FOR REVIEW

17. By signing this application, I certify (1) to the statements contained in the list of certifications* and (2) that the statements herein are true, complete and accurate to the best of my knowledge. I also provide the required assurances * and agree to comply with any resulting terms if I accept an award. I am aware that any false, fictitious. or fraudulent statements or claims may subject me to criminal, civil, or administrative penalties. (U.S. Code, Title 18, Section 1001)

☒ **I agree**

**The list of certifications and assurances, or an Internet site where you may obtain this list, is contained in the announcement or agency specific instructions.*

18. SFLLL (Disclosure of Lobbying Activities) or other Explanatory Documentation

Add Attachment | Delete Attachment | View Attachment

19. Authorized Representative

Prefix: | First Name: Sandra | Middle Name:

Last Name: Newman | Suffix:

Position/Title: Assistant Director

Organization: UNIVERSITY OF CINCINNATI

Department: COM, Operations and Finance

Division:

Street1: 260 Stetson Street

Street2: Suite 4200

City: Cincinnati | County / Parish: Hamilton

State: OH: Ohio | Province:

Country: USA: UNITED STATES | ZIP / Postal Code: 45219-0553

Phone Number: 5135588894 | Fax Number:

Email: sandra.newman@uc.edu

Signature of Authorized Representative	Date Signed
Sandi Newman	07/10/2026

20. Pre-application Add Attachment | Delete Attachment | View Attachment

21. Cover Letter Attachment Add Attachment | Delete Attachment | View Attachment

OMB Number: 4040-0019
Expiration Date: 06/30/2028

Project Abstract Summary

This Project Abstract Summary form must be submitted or the application will be considered incomplete. Ensure the Project Abstract field succinctly describes the project in plain language that the public can understand and use without the full proposal. Use 4,000 characters or less. Do not include personally identifiable, sensitive or proprietary information. Refer to Agency instructions for any additional Project Abstract field requirements. If the application is funded, your project abstract information (as submitted) will be made available to public websites and/or databases including USAspending.gov.

Funding Opportunity Number

HRSA-26-019

Assistance Listing Number(s):

93.877

Applicant Name

UNIVERSITY OF CINCINNATI

Descriptive Title of Applicant's Project

UNIVERSITY AFFILIATED CINCINNATI CENTER FOR DEVELOPMENTAL DISORDERS

Project Abstract

Children and youth with autism/developmental disabilities (AS/DD) have complex needs, often requiring specialized services with providers from various disciplines across multiple systems. As these individuals move to adulthood, services become even more limited and systems more fragmented. There is a shortage of qualified providers in Ohio and across the country to provide evidence-based care to these individuals and their families. Cincinnati LEND aims to provide an interdisciplinary leadership training program combining academic, clinical, leadership, and community-based learning opportunities that are culturally and linguistically appropriate and person-/family-centered to increase the capacity of the workforce trained to meet the complex needs of this population and improve systems of care. Family members of individuals with AS/DD, advocates with lived experience, practicing professionals, and graduate students from various disciplines and programs will form an interdisciplinary cohort who will learn with, and from, each other under the mentorship of nationally recognized faculty. Cincinnati LEND trainees will demonstrate clinical competence in screening, evaluating, diagnosing, and providing evidence-based interventions in an interdisciplinary manner to individuals with AS/DD across their lifespan. In addition, they will participate in a robust leadership curriculum to build skills in policy, research, and quality improvement strategies that can lead to improved systems of care for these individuals. Cincinnati LEND trainees and faculty will continue to provide continuing education and technical assistance to family members and practicing professionals from various community organizations and agencies, as well as continue to disseminate project curricula, products, scholarly activity, and training approaches through publications in scholarly journals and presentations at conferences. Cincinnati LEND Faculty work as a cohesive team to develop and refine the Interdisciplinary LEND Curriculum (ILC), guided by the MCH Leadership Competencies v.5. The ILC consists of five elements: 1) Disciplinary Clinical Experiences, 2) Interdisciplinary Leadership Seminar, 3) Interdisciplinary Training Team, 4) Core Course and 5) Seminar in Evidence-based Methods. These components are intentionally sequenced across a 30-week academic year to support progressive development from foundational knowledge and self-awareness to advanced interdisciplinary practice, research, and systems-level leadership. Cincinnati LEND will achieve its goals through five core objectives: (1) training an interdisciplinary cohort of at least 29 long-term trainees annually across a minimum of 12 disciplines; (2) increasing the number of interdisciplinary diagnostic evaluations conducted by trainees and faculty; (3) integrating individuals with lived experience, including family members and advocates, as trainees and faculty; (4) providing continuing education that reaches at least 1,000 professionals and caregivers annually; and (5) delivering technical assistance through sustained engagement with at least 50 local, state, and national partnerships. These objectives are operationalized through an integrated set of didactic, clinical, leadership, research, and community-based activities, including interdisciplinary clinical teams, family mentoring, policy and advocacy training, and service-learning experiences. Using a comprehensive evaluation framework, each objective is supported by specific, measurable outcomes and continuous quality improvement processes, ensuring alignment between training activities, trainee competencies, and measurable impacts on workforce capacity and systems of care. By increasing workforce capacity, enhancing interdisciplinary collaboration, and strengthening systems of care, Cincinnati LEND will improve access to timely, high-quality services and advance outcomes for children and youth with AS/DD and their families.

OMB Number: 4040-0001
Expiration Date: 01/31/2029

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Project Director/Principal Investigator

Prefix: | * First Name: Jennifer | Middle Name:
* Last Name: Smith | Suffix:
Position/Title: Professor
Department: Developmental and Behavioral Pediatrics
Organization Name: Cincinnati Children's Hospital Medical Center
Division:
* Street1: 3333 Burnet Avenue
Street2:
* City: Cincinnati | County/ Parish: Hamilton
* State: OH: Ohio | Province:
* Country: USA: UNITED STATES | * Zip / Postal Code: 45229-3026
* Phone Number: 513-803-2365 | Fax Number:
* E-Mail: jennifer.smith4@cchmc.org

Credential, e.g., agency login:

*** Project Role:** PD/PI | **Other Project Role Category:**

Degree Type:
Degree Year:

***Attach Biographical Sketch**	1244-Biosketch_Smith_LEND.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

PROFILE - Senior/Key Person 1

Prefix: | * First Name: Stephanie | Middle Name:
* Last Name: Weber | Suffix:
Position/Title: Assistant Professor
Department: Developmental and Behavioral Pediatrics
Organization Name: Cincinnati Children's Hospital Medical Center
Division:
* Street1: 3333 Burnet Ave
Street2:
* City: Cincinnati | County/ Parish:
* State: OH: Ohio | Province:
* Country: USA: UNITED STATES | * Zip / Postal Code: 45229-3026
* Phone Number: 513-803-3630 | Fax Number:
* E-Mail: stephanie.weber@cchmc.org

Credential, e.g., agency login: STEPHWEBER

*** Project Role:** Other (Specify) | **Other Project Role Category:** Training Director

Degree Type:
Degree Year:

Attach Biographical Sketch	1245-3. Biosketch_Weber_LEND	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

BIOGRAPHICAL SKETCH

NAME: Smith, Jennifer
POSITION TITLE: Professor of Clinical Pediatrics
EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE	Completion Date	FIELD OF STUDY
West Virginia University, Morgantown, WV	BA	05/1997	Psychology
Radford University, Radford, VA	MA	05/2000	Clinical Psychology
Xavier University, Cincinnati, OH	PSYD	08/2004	Clinical Psychology
Munroe-Meyer Institute, Omaha, NE	Resident	06/2004	Pre-doctoral Internship
Columbus Children's Hospital, Columbus, OH	Fellow	08/2006	Post-doctoral Fellowship

A. Personal Statement

I am a Clinical Psychologist, Board Certified Behavior Analyst-Doctoral Level, applied researcher, and Professor of Clinical Pediatrics at the University of Cincinnati College of Medicine (UCCOM). I have been the Project Director of the Leadership Education in Neurodevelopmental and related Disabilities (LEND) program at Cincinnati Children's Hospital Medical Center (CCHMC)/University of Cincinnati since 2013. As the Principal Investigator (PI) on this T73 federally funded interdisciplinary training grant, I obtained continued funding through successful grant competitions in 2016 and 2021. As LEND Director, I have direct responsibility for all aspects of the program including executive decision making in allocation of grant budget funds. Our LEND program is nationally recognized, as evidenced by our involvement in the Association of University Centers on Disabilities (AUCD) Network, frequent requests for program consultation, and invitations to collaborate. I have been invited by MCHB to serve as a peer consultant for site visits to other LENDs in WV, IA, and NY. As the lead for the LEND Program Quality Improvement (LPQI) Network, I led a group of LEND directors across the country in demonstrating the impact of LEND training which was published in Pediatrics (2022). Leaders at MCHB have used this work to show the importance of LEND training in their federal advocacy efforts. Under my leadership, 359 trainees have received interdisciplinary training through the Cincinnati LEND program. These graduates have gone on to lead in hospitals, universities, schools, foundations, and communities in 29 states across the United States.

B. Positions and Honors

Positions

2006-2007	Supervising Psychologist, Helping Hands Center, Worthington, OH
2007-2009	Director of Curriculum Development, May Center, Randolph, MA
2009-2013	Staff Psychologist, Division of Developmental and Behavioral Pediatrics, CCHMC
2013-2020	Assistant Professor of Clinical Pediatrics, CCHMC
2020-2026	Associate Professor of Clinical Pediatrics, CCHMC
2026-Present	Professor of Clinical Pediatrics, CCHMC

Honors

2014	Award for Imagination and Courage, Jack H Rubinstein Foundation, CCHMC
2016	Dr. Jack Rubinstein Award Winner, Jack H Rubinstein Foundation, CCHMC
2023	Top Scientific Achievement in DDBP, CCHMC
2025	CCHMC Educational Achievement Award Nominee, CCHMC Office of Academic Affairs and Career Development

BIOGRAPHICAL SKETCH

NAME: Weber, Stephanie

POSITION TITLE: Associate Professor-Educator, Clinical Pediatrics

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE	Completion Date	FIELD OF STUDY
Saint Joseph's College, Rensselaer, IN	BS	05/2006	Psychology
University of Indianapolis, Indianapolis, IN	MA	05/2008	Clinical Psychology
University of Indianapolis, Indianapolis, IN	PSYD	08/2011	Clinical Psychology
The Devereux Foundation, Villanova, PA	Resident	08/2011	Predoctoral Internship
Cincinnati Children's, Cincinnati, OH	Fellow	07/2013	Postdoctoral Fellowship
University of Cincinnati Dept of Environmental Health, Cincinnati, OH	MPH	12/2020	Public Health

A. Personal Statement

I am a Clinical Psychologist with public health training, and Associate Professor-Educator in Clinical Pediatrics at the University of Cincinnati College of Medicine. My expertise spans evidence-based practice, quality improvement, the policy-making process, and screening, diagnosing, and treating developmental disabilities. Since 2013, I have been the Training Director of the Leadership Education in Neurodevelopmental and related Disabilities (LEND) program at Cincinnati Children's Hospital Medical Center (CCHMC)/University of Cincinnati and the University of Cincinnati Center for Excellence in Developmental Disabilities (UCCEDD), assuming additional responsibilities as LEND Associate Director in 2021. I oversee recruitment and retention of trainees, all curriculum development and implementation, research supervision. I direct our 300+ hour, graduate-level interdisciplinary training curriculum serving at least 29 trainees annually across 13 disciplines. We have graduated more than 300 long-term trainees since 2013, and I have directly mentored 121 trainees in clinical practice and research. My long-term professional goal is to advance high-quality interdisciplinary training and workforce development strategies that support healthy social-emotional and behavioral development in young children.

B. Positions and Honors

Positions

2023-	Associate Professor-Educator, Division of Developmental and Behavioral Pediatrics, CCHMC
2021-	Associate Director, LEND program
2014-	Act Early Ambassador to Ohio, Center for Disease Control and Prevention's "Learn the Signs. Act Early." program
2013-2023	Assistant Professor- Educator, Division of Developmental and Behavioral Pediatrics, CCHMC,
2013-	Training Director, LEND program
2013-	Training Director, UCCEDD
2013	Ohio Psychology License #7028

Honors

2025	*Impact & Inspire Award*, Community Relations, CCHMC
2019	*Jack H. Rubinstein Award*, Jack H. Rubinstein Foundation
2015	*Bridge Builder Award*, Cincinnati-Hamilton County Community Action Agency

Project/Performance Site Location(s)

Project/Performance Site Primary Location

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name: UNIVERSITY OF CINCINNATI

UEI: DZ4YCZ3QSPR5

* Street1: Digital Futures Rm 610, 3080 Exploration Ave.

Street2: PO Box 210222

* City: Cincinnati

County: Hamilton

* State: OH: Ohio

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code: 45206-0222

* Project/ Performance Site Congressional District: OH-001

Project/Performance Site Location 1

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name: Cincinnati Children's Hospital Medical Center

UEI: JZD1HLM2ZU83

* Street1: 3333 Burnet Avenue

Street2:

* City: Cincinnati

County: Hamilton

* State: OH: Ohio

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code: 45229-3026

* Project/ Performance Site Congressional District: OH-001

Additional Location(s)

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DZ4YCZ3QSPR5 **Enter name of Organization:** UNIVERSITY OF CINCINNATI

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 1 **Start Date:** 07/01/2026 **End Date:** 06/30/2027

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith						0.00	0.00	0.00

Project Role: PD/PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Speech Language Pathology (Raisor-Becker)	0.60			5,845.00	2,133.00	7,978.00

1 **Total Number Other Personnel**

Total Other Personnel 7,978.00

Total Salary, Wages and Fringe Benefits (A+B) 7,978.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment Delete Attachment View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2.	Foreign Travel Costs	
	Total Travel Cost	

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	122,368.00
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees **Total Participant/Trainee Support Costs**	122,368.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	3,791.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	596,921.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.		
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	600,712.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	731,058.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	36,769.00	2,942.00
		Total Indirect Costs	2,942.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)
DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	734,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	734,000.00

L. Budget Justification

(Only attach one file.) 1241-8. UC Budget Justification Narrat

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DZ4YCZ3QSPR5 **Enter name of Organization:** UNIVERSITY OF CINCINNATI

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 2 **Start Date:** 07/01/2027 **End Date:** 06/30/2028

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith						0.00	0.00	0.00

Project Role: PD/PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Speech Language Pathology (Raisor-Becker)	0.60			6,020.00	2,221.00	8,241.00

1 **Total Number Other Personnel**

Total Other Personnel 8,241.00

Total Salary, Wages and Fringe Benefits (A+B) 8,241.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment Delete Attachment View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2.	Foreign Travel Costs	
	Total Travel Cost	

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	122,368.00
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees **Total Participant/Trainee Support Costs**	122,368.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	5,644.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	596,636.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.		
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	602,280.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	732,889.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	13,885.00	1,111.00
		Total Indirect Costs	1,111.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)
DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	734,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	734,000.00

L. Budget Justification

(Only attach one file.) 1241-8. UC Budget Justification Narrat Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DZ4YCZ3QSPR5 **Enter name of Organization:** UNIVERSITY OF CINCINNATI

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 3 **Start Date:** 07/01/2028 **End Date:** 06/30/2029

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith						0.00	0.00	0.00

Project Role: PD/PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Speech Language Pathology (Raisor-Becker)	0.60			6,201.00	2,325.00	8,526.00

1 **Total Number Other Personnel**

Total Other Personnel 8,526.00

Total Salary, Wages and Fringe Benefits (A+B) 8,526.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment Delete Attachment View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2.	Foreign Travel Costs	
	Total Travel Cost	

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	122,368.00
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees **Total Participant/Trainee Support Costs**	122,368.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	5,643.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	596,330.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.		
9.		
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	601,973.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	732,867.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	14,169.00	1,133.00
		Total Indirect Costs	1,133.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)
DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	734,000.00

J. Fee

	Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	734,000.00

L. Budget Justification

(Only attach one file.) 1241-8. UC Budget Justification Narrat

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DZ4YCZ3QSPR5 **Enter name of Organization:** UNIVERSITY OF CINCINNATI

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 4 **Start Date:** 07/01/2029 **End Date:** 06/30/2030

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith						0.00	0.00	0.00

Project Role: PD/PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Speech Language Pathology (Raisor-Becker)	0.60			6,387.00	2,299.00	8,686.00

1 **Total Number Other Personnel**

Total Other Personnel 8,686.00

Total Salary, Wages and Fringe Benefits (A+B) 8,686.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment Delete Attachment View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2.	Foreign Travel Costs	
	Total Travel Cost	

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	122,368.00
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees **Total Participant/Trainee Support Costs**	122,368.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	5,644.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	596,156.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.		
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15.		
16.		
17.		
	Total Other Direct Costs	601,800.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	732,854.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	14,330.00	1,146.00
		Total Indirect Costs	1,146.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)
DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	734,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	734,000.00

L. Budget Justification

(Only attach one file.) 1241-8. UC Budget Justification Narrat Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: DZ4YCZ3QSPR5 **Enter name of Organization:** UNIVERSITY OF CINCINNATI

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 5 **Start Date:** 07/01/2030 **End Date:** 06/30/2031

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith						0.00	0.00	0.00

Project Role: PD/PI

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 0.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
1	Speech Language Pathology (Raisor-Becker)	0.60			6,578.00	2,388.00	8,966.00

1 **Total Number Other Personnel**

Total Other Personnel 8,966.00

Total Salary, Wages and Fringe Benefits (A+B) 8,966.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment Delete Attachment View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	
2.	Foreign Travel Costs	
	Total Travel Cost	

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	122,368.00
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees **Total Participant/Trainee Support Costs**	122,368.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	
2.	Publication Costs	
3.	Consultant Services	5,644.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	595,853.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.		
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11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	601,497.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	732,831.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	14,610.00	1,169.00
		Total Indirect Costs	1,169.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)
DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	734,000.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	734,000.00

L. Budget Justification

(Only attach one file.) 1241-8. UC Budget Justification Narrat Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

		Totals ($)
Section A, Senior/Key Person		0.00
Section B, Other Personnel		42,397.00
Total Number Other Personnel	5	
Total Salary, Wages and Fringe Benefits (A+B)		42,397.00
Section C, Equipment		
Section D, Travel		
1. Domestic		
2. Foreign		
Section E, Participant/Trainee Support Costs		611,840.00
1. Tuition/Fees/Health Insurance		
2. Stipends	611,840.00	
3. Travel		
4. Subsistence		
5. Other		
6. Number of Participants/Trainees		
Section F, Other Direct Costs		3,008,262.00
1. Materials and Supplies		
2. Publication Costs		
3. Consultant Services	26,366.00	
4. ADP/Computer Services		
5. Subawards/Consortium/Contractual Costs	2,981,896.00	
6. Equipment or Facility Rental/User Fees		
7. Alterations and Renovations		
8. Other 1		
9. Other 2		
10. Other 3		
11. Other 4		
12. Other 5		
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	3,662,499.00
Section H, Indirect Costs	7,501.00
Section I, Total Direct and Indirect Costs (G + H)	3,670,000.00
Section J, Fee	
Section K, Total Costs and Fee (I + J)	3,670,000.00

RESEARCH & RELATED Other Project Information

OMB Number: 4040-0001
Expiration Date: 01/31/2029

1. Are Human Subjects Involved? ☐ Yes ☒ No

1.a. If YES to Human Subjects

Is the Project Exempt from Federal regulations? ☐ Yes ☐ No

If yes, check appropriate exemption number. ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5 ☐ 6 ☐ 7 ☐ 8

If no, is the IRB review Pending? ☐ Yes ☐ No

IRB Approval Date:

Human Subject Assurance Number:

2. Are Vertebrate Animals Used? ☐ Yes ☒ No

2.a. If YES to Vertebrate Animals

Is the IACUC review Pending? ☐ Yes ☐ No

IACUC Approval Date:

Animal Welfare Assurance Number:

3. Is proprietary/privileged information included in the application? ☐ Yes ☒ No

4.a. Does this Project Have an Actual or Potential Impact - positive or negative - on the environment? ☐ Yes ☒ No

4.b. If yes, please explain:

4.c. If this project has an actual or potential impact on the environment, has an exemption been authorized or an environmental assessment (EA) or environmental impact statement (EIS) been performed? ☐ Yes ☐ No

4.d. If yes, please explain:

5. Is the research performance site designated, or eligible to be designated, as a historic place? ☐ Yes ☒ No

5.a. If yes, please explain:

6. Does this project involve activities outside of the United States or partnerships with international collaborators? ☐ Yes ☒ No

6.a. If yes, identify countries:

6.b. Optional Explanation:

7. Project Summary/Abstract 1242-Blank Document.pdf Add Attachment Delete Attachment View Attachment

8. Project Narrative 1243-Cincinnati LEND Project Narrativ Add Attachment Delete Attachment View Attachment

9. Bibliography & References Cited Add Attachment Delete Attachment View Attachment

10. Facilities & Other Resources Add Attachment Delete Attachment View Attachment

11. Equipment Add Attachment Delete Attachment View Attachment

12. Other Attachments Add Attachments Delete Attachments View Attachments

INTRODUCTION

This proposal describes a five-year strategy for building upon the extensive experience in interdisciplinary leadership training of the Leadership Education in Neurodevelopmental and related Disabilities (LEND) Program at Cincinnati Children's Hospital Medical Center (CCHMC) and the University of Cincinnati (UC). The program will be referred to as "Cincinnati LEND" throughout this proposal, and a chart of commonly used acronyms can be found on p. 37. Cincinnati LEND is co-located with the University of Cincinnati Center for Excellence in Developmental Disabilities (UCCEDD) in CCHMC's Division of Developmental and Behavioral Pediatrics (DDBP). The ongoing goal of Cincinnati LEND is to train the next generation of leaders to meet the complex needs of individuals with autism and other developmental disabilities (DD) and their families across the lifespan and improve the system of care for this population. These objectives are accomplished by providing a balance of academic, clinical, leadership, and community-based learning opportunities in an interdisciplinary and person-/family-centered manner.

NEED

Cincinnati LEND is one of two current LEND programs in Ohio. Cincinnati LEND serves Ohio, as well as the bordering states of Kentucky and Indiana. Ohio is a Midwestern state characterized by five major urban core areas (Cleveland, Cincinnati, Columbus, Dayton, and Toledo), alongside many rural counties. The state comprises 88 counties and has an estimated population of approximately 11.9 million residents (U.S. Census Bureau, 2025), of which 2.57 million (21.7%) are children under the age of 18. The median household income for Ohioans is $71,389, and 12.7% live in poverty. In terms of health, 10.4% of Ohioans under age 65 live with a disability, and 8.1% are without health insurance. Ohio's population is predominantly White (80.1%), followed by Black (13.6%), Asian (3.1%), and smaller proportions of other racial groups. Approximately 5.2% of the population in Ohio identifies as Hispanic or Latino. There are over 314,000 residents in Cincinnati, where the median household income is lower than Ohio ($52,909) and the poverty rate is more than double (25.5%). The racial and ethnic composition of the Cincinnati Metropolitan Statistical Area reflects the state's composition, though substantial variation exists across counties and the city of Cincinnati itself. The racial makeup of the city of Cincinnati is 49% White followed by 36.5% Black. Additionally, our catchment area includes a contiguous region of 32 counties in southeastern Ohio designated as Appalachian Ohio, an area that continues to face persistent economic disadvantages and barriers to health care compared to non-Appalachian regions (U.S. Census Bureau, 2025).

Unmet Needs

Unmet needs Cincinnati LEND will address. Cincinnati LEND addresses many unmet needs affecting children and youth with autism/DD including increasing workforce capacity, improving access to timely specialty care services that are high-quality and well-coordinated, and providing training in crucial leadership skills.

National, state, regional, and local need for professionals with expertise in autism/DD related to screening, diagnosis, and interdisciplinary service delivery.

National: In America, more children are being diagnosed with autism than ever before. The National Survey of Children's Health documents that 1 in 25 or 4.3% of children aged 3-17 years have a diagnosis on the autism spectrum (Child and Adolescent Health Measurement Initiative (CAHMI), 2023-2024). Recent prevalence data from the Centers for Disease Control and Prevention's (CDC) Autism and Developmental Disabilities Monitoring Network estimates that 1 in 31 children aged 8 years in the U.S. have been identified with a diagnosis on the autism spectrum (Shaw, 2025). In addition to autism, more

children are being identified with other DDs and delays including, but not limited, to attention deficit hyperactivity disorder (ADHD), Intellectual Disability, Down syndrome, Cerebral Palsy, speech delay, developmental delay, hearing and vision differences, and learning concerns. According to the CDC (2024), approximately 1 in 6 children or 17% of children aged 3-17 years in the U.S. has a DD.

State: The National Survey of Children's Health documents that 3.1% of children in Ohio between the ages of 3-17 years have a diagnosis on the autism spectrum (CAHMI, 2023-2024). According to the Ohio Department of Education and Workforce's Special Education Legislative Report (2026), 14.64% of all students with disabilities aged 3-21 who received special education services in the state in 2024-2025 were identified with Autism, representing the third highest disability type served. Students with disabilities account for 17.1% of all Ohio's preschool and school-age students aged 3-21 years.

Regional/Local: Data collected by the Ohio Department of Education and Workforce (2025) indicate that 21.6% (>7,600) of students in Cincinnati Public Schools (the "home" district for DDBP) are being served by the Ohio Department of Education as a student with a disability. DDBP, where Cincinnati LEND is located, serves over 10,000 patients per year, with 56% of these patients being identified with a diagnosis on the autism spectrum. Over half (56.2%) of patients served by DDBP use Medicaid.

Unmet needs affecting children and youth with autism/DD.

Workforce shortages: Current shortages of professionals are contributing to decreased access to and quality of healthcare to meet the complex needs of children and youth with autism/DD. Ohio has 249 primary care Health Professional Shortage Areas and 226 mental health care Health Professional Shortage Areas (Bureau of Health Workforce, 2025), a substantial increase in need from the 2020 needs assessment. According to the American Board of Pediatrics, Ohio had approximately 1.7 Developmental Behavioral Pediatricians (DBP) per 100,000 children in 2025. Although Ohio has a better per-capita availability of DBP certified professionals than other states, the referral population to DDBP includes Kentucky, West Virginia, and Indiana. These states have an extremely small number of DBPs for their populations (all below 0.6 per 100,000 children), thus adding substantially to the needs for specialty care within our referral area (American Board of Pediatrics Certification Management System, 2025). Similar shortages are noted in other specialty areas needed by children and youth with autism/DD, with demand significantly outweighing supply of providers. More than two-thirds of specialty centers serving children and youth with autism/DD across the U.S. report workforce shortages. This workforce shortage, plus the volume of children being referred for developmental concerns like autism leads to extreme burdens across the country (Cognoa, 2023).

Access to timely services: Access concerns are seen across the country, especially for diagnostic services to evaluate children for developmental concerns. An analysis of specialty care centers across the country reveals excruciating long wait times, with 25% of these centers having waitlists of longer than six months and over 21% having waitlists of longer than one year or that they were no longer receiving new referrals (Cognoa, 2023). A poll of developmental behavioral pediatricians at the Society for Developmental & Behavioral Pediatrics (SDBP) Annual Meeting in September 2023 showed even longer wait times, with more than half of these providers reporting waitlists of longer than nine months at their academic or hospital affiliated centers. In the U.S., less than a quarter (23.9%) of children on the autism spectrum are being diagnosed prior to the age of three years. More than a third (33.5%) are diagnosed when they are older than five years. Similar ages of autism diagnoses are found in HRSA Region V, which represents Ohio (CAHMI, 2023-2024). Due to these limitations, children with autism/DD are missing critical early intervention services.

Access to well-coordinated specialty services: Children and youth with autism/DD often present with complex health care needs and require a variety of services from multiple systems, including special education services, specialized medical care, and allied health services (e.g., speech, behavioral, and occupational therapies). When families are left to take responsibility for navigating these complicated service systems, they experience frustration and barriers in service access and provision. Close to a quarter (24.2%) of parents of children with special health care needs (SHCN) in Ohio report difficulty getting specialist care for their child's needs, with only 24.9% of parents reporting their child receives the special services to meet developmental needs, such as speech, occupational, or behavioral therapy. When care is provided, families report poor care coordination, with 39.2% of children with SHCN in Ohio not receiving needed care coordination. Even more alarmingly, 85.6% of parents with children with SHCN aged 0-17 years in Ohio report that they do not receive care within a well-functioning system. More than half (54.4%) of children with SHCN aged 0-17 years in Ohio did not have a medical home while 73.6% aged 12-17 years did not receive services necessary for the move to adult healthcare (CAHMI, 2023-2024).

Mental health needs: Mental health concerns are very common in children with autism. Up to 78% of children with autism have at least one mental health condition and nearly half have two (Kerns et al., 2020). The most common mental health conditions co-occurring with autism include behavioral disorders, anxiety, ADHD, and depression. Mental health systems in Ohio are already strained, with only 20.9% of the state's mental health needs being met due to workforce shortages (Bureau of Health Workforce, 2025). Children and youth with autism and co-occurring mental health conditions often do not receive access to appropriate mental and behavioral health services until it is too late, leading to higher rates of inpatient hospitalizations due to crisis situations (Pfingston & Sneed, 2025).

Training gaps: Not only do families of children with autism/DD express significantly higher needs for specialty services, but they also express difficulty finding providers with necessary training and skills to provide these services. This shortage of training is reported by physicians themselves who report a lack of confidence and knowledge due to inadequate training to provide effective care to children and youth with autism (Mazurek et al., 2020). As a result, children and youth with autism may not receive appropriate screening, evaluation, or treatment for their complex conditions, leading to poorer health outcomes. If healthcare professionals enter practice with the common core competencies needed to provide the safest, highest quality of person-/family-centered, culturally and linguistically appropriate clinical care, it is usually focused within their own disciplines. However, many professionals enter the healthcare system without any formal interdisciplinary leadership training. It has become even more necessary for professionals to understand how other disciplines approach care to address the multifaceted needs of these children and their families. Interdisciplinary programs such as LEND provide a model for the care coordination necessary for children/adolescents with autism/DD, leading to less duplication of services, quicker access to care, and better health outcomes (Kabarite et al., 2025). The Maternal and Child (MCH) field has recognized the need for interdisciplinary leadership training programs that have intentionality, planning, and accountability as cornerstones (Margolis, et al., 2013; McHugh, et al., 2016).

How Cincinnati LEND will address gaps in systems of care and the workforce for children/youth with autism/DD. Fulfilling unmet needs for well-coordinated specialty care and improving access to timely care to address the complex needs of children and youth with autism/DD will require both a workforce that has the knowledge and skills to both provide high quality person-/family-centered, culturally and linguistically appropriate services, and one that can provide effective leadership in addressing these major issues. Cincinnati LEND serves a vital role in addressing gaps in systems of care for medically underserved populations, specifically those with autism/DD. To address the severe shortage of trained professionals, Cincinnati LEND will continue to effectively recruit and train cohorts of trainees

from a wide variety of professional disciplines including graduate and postgraduate students, practicing professionals, individuals with disabilities, and family members, each prepared to provide evidence-based, interdisciplinary care for individuals with autism/DD. By training clinicians in efficient, team-based diagnostic and intervention practices, Cincinnati LEND increases regional and state-level capacity to reduce wait times for developmental evaluation and specialty care. Cincinnati LEND integrates mental and behavioral health training across disciplines, preparing trainees to recognize and address co-occurring mental health conditions that are prevalent but often untreated in individuals with autism/DD. In addition to clinical competence, Cincinnati LEND will continue to train MCH leaders who develop leadership skills that progress from self to others to the wider community based on the MCH Leadership Competencies (v. 5) (Maternal and Child Health Bureau (MCHB), 2026). The Cincinnati LEND curriculum directly prepares trainees to coordinate care across medical, educational, and community systems, addressing the documented gaps in care coordination in Ohio. Through didactic, clinical, and community-based experiences, Cincinnati LEND fills the training gap identified by providers who report inadequate preparation to care for individuals with autism/DD. Cincinnati LEND recognizes the importance of this project to align with MCHB's vision of "An America where all mothers, children, and families thrive and reach their full potential." Cincinnati LEND is proud to be a part of a national network that is developing leaders in MCH, building the MCH workforce, and impacting systems of care, particularly for those who are underserved, medically vulnerable, or live in areas with limited access to care.

APPROACH

Overall Methodology

Cincinnati LEND will address stated needs and meet the program requirements and expectations by establishing measurable performance goals, applying evidence-informed approaches, and using data to monitor outcomes and drive continuous improvement. To directly fulfill the purpose of the LEND program, Cincinnati LEND provides interdisciplinary, leadership-focused training that integrates didactic, clinical, community, and leadership experiences to prepare professionals to improve systems of care for individuals with autism/DD. Curriculum components are explicitly aligned with MCH Leadership Competencies (v5) and are sequenced to support progressive development from foundational knowledge and self-awareness to advanced interdisciplinary practice and systems-level leadership. **See Attachment 1: Work Plan** for a comprehensive overview of how the goals and objectives outlined directly align with the LEND program goals, including developing interdisciplinary leaders, increasing the autism/DD workforce, strengthening person-/family-centered care, and improving systems of care. Cincinnati LEND will address unmet needs of individuals with autism/DD and meet the project objectives through the activities, outcome measures, timeframe, and responsible staff/partners described.

Trainee recruitment and retention

Approach to recruit, select, and retain a qualified interdisciplinary cohort of Long-term trainees (LTTs). The size and composition of our LTT cohorts during the 2021-2026 cycle attests to our successful recruitment strategies. Recruitment strategies have been strengthened to optimize applicants' leadership potential, dedication to work in an interdisciplinary team, and commitment to serving individuals with autism/DD and their families. In collaboration with UCCEDD, we have worked to redesign our recruitment materials, overall branding, and website. We have worked to increase Cincinnati LEND's breadth and reach across avenues such as social media and conference marketing. LEND Leadership and Core Faculty have met with Deans, Department Chairs, and/or Program Directors for all

disciplines represented in the LEND program to date. Additionally, in 2025, Cincinnati LEND partnered with Akron Children's Developmental and Behavioral Pediatrics to support their training needs. Trainees from Akron Children's are recruited by Akron and Cincinnati faculty to virtually participate in Cincinnati LEND courses and complete their clinical work in various settings at Akron Children's. This partnership is coordinated by a DBP at Akron Children's who is a graduate of the Cincinnati LEND program and currently an Associate Professor of Pediatrics at Northeast Ohio Medical University. For recruitment of family trainees and trainees with disabilities, we have met with existing community partners, DDBP's Family Advisory Council (FAC), UCCEDD's Community Advisory Council (CAC), and county DD boards. We have continued to expand our network of community contacts for the purposes of recruitment. During all meetings, we emphasize the qualifications of the applicants we are seeking and the advantages of membership in our nationwide community of trainees, faculty, and people with autism/DD and their families. We highlight the many leadership positions which former trainees now hold in various local, regional, and national agencies and organizations. We specifically discuss the MCH Leadership Competencies and our program curriculum, emphasizing the unique attributes of the LEND curriculum to expand work within their graduate programs, especially in leadership, **interdisciplinary care, and person-/family-centered care**. Information sessions for individual programs are held at fitting times in the academic year. Several graduate programs have now added information about the LEND opportunity to their own recruitment materials. Additionally, residency and fellowship programs which recruit and place LEND trainees into the cohort emphasize LEND training in their recruitment and ongoing disciplinary accreditation materials. We have also produced two videos that are posted on YouTube. One video provides an overview of the LEND curriculum, while the other features multiple trainees discussing their LEND experience. Our application materials are designed to capture information specific to leadership qualities, dedication to individuals with autism/DD and their families, and interdisciplinary teamwork to ensure we obtain the most qualified candidates. A separate application supplement has been created for family trainees and trainees with disabilities to highlight their advocacy and community education accomplishments. After applications are reviewed by LEND Faculty, selected applicants are invited for interviews. Each applicant is interviewed by the Project Director (PD) and Associate/Training Director (AD/TD) and discipline-specific Core Faculty and/or clinical supervisors. Standardized interviewing techniques are used to assess candidates on various competencies including leadership potential, emotional intelligence competencies, experience with autism/DD, and overall fit with Cincinnati LEND using a Likert-scale form completed by interviewers. The PD and AD/TD review written interview feedback forms and make selections. To ensure retention and graduation of recruited trainees, leadership and faculty provide professional development mentorship both within and across disciplines. Each trainee develops an Individualized Leadership Plan (ILP) with faculty support and is afforded a multitude of scaffolded opportunities throughout the year based on the trainees' interests. As the PD and AD/TD are trained psychologists, mental health and wellness are of strategic focus in the program beginning during Orientation. These strategies have been successful as measured by 97% graduation during the previous grant cycle.

Composition of LTT cohort: LTTs include a) those who spend 300 or more hours in training in Interdisciplinary LEND Curriculum (known as "long-term LEND trainees") and b) those who participate in other specialized disciplinary or interdisciplinary training experiences (known as "other LTTs"). LTTs all demonstrate interest in a career serving individuals with autism/DD who represent various populations and satisfy other statutory requirements. Cincinnati LEND recruits at least 29 LTTs from at least 12 disciplines corresponding to funding tier 4, with at least one trainee with a disability and at least one family trainee participating each year. No single trainee discipline makes up more than 25% of a LEND

cohort per year. All disciplines participate in the cohort equally, providing unique contributions based on either their clinical or lived experience.

Graduate level trainees: Graduate level trainees are master's or doctoral candidates, or post-master's and Postdoctoral Fellows. The long-term LEND trainee cohort is recruited from graduate schools throughout the greater Cincinnati area, including University of Cincinnati, Xavier University, Miami University, Northern Kentucky University, Wright State University, University of Dayton, Mount St. Joseph University, as well as through national recruitment for fellowships (developmental-behavioral pediatrics (DBP), occupational therapy (OT), social work (SW), residencies (physical therapy (PT)), and externships (audiology).

Practicing professionals: Practicing professionals from the community with graduate degrees or commensurate work or leadership experience who wish to direct or redirect their work towards a leadership role related to individuals with autism/DD and their families participate in the LEND cohort.

Other LTTs: At least five "other LTTs" will participate in specialized, interdisciplinary training experiences each year. These include psychology pre- and post-doctoral trainees.

Trainees with a disability: Individuals who identify as having a disability serve as vital members of the Cincinnati LEND program. No educational minimum or current enrollment in an academic program is required for these trainees. Trainees with a disability must demonstrate an interest in developing their own leadership skills and sharing their lived experience. At least one trainee with a disability has participated in the Cincinnati LEND program since 2014. At least one trainee with a disability (Advocacy discipline) will participate in each year of this funding cycle.

Family trainees: Parents, caregivers, and siblings of individuals with autism/DD, known as family trainees, also serve as vital members of the Cincinnati LEND cohort. No educational minimum or current enrollment in an academic program is required for family trainees. However, family trainees must demonstrate an interest in developing leadership skills and sharing family perspectives with other trainees. Two or three family trainees have completed Cincinnati LEND each year since 2006. Two or three family trainees will participate each year during the upcoming five-year grant cycle.

Table 1 describes numbers, disciplines, and types of LTTs that will be recruited for the new project period.

Table 1: Long-term LEND trainees

# of trainees	Years	Discipline (acronym)	Level of training and institution
1-2	1-5	Advocacy (ADV)	N/A
1	1-5	Applied Behavior Analysis (ABA)	Various
1	1-5	Audiology (AUD)	Pre-doctoral; National audiology
1	1-5	Clinical Counseling (CC)	Various
1-2	1-5	Disability Studies (DS)	Various
3	1-5	Family (FAM)	N/A
2-3	1-5	Genetic Counseling (GC)	Pre-doctoral; UC Master's program
At least 2	1-5	Medicine/DBP	CCHMC DBP Fellows; community
1	1-5	Graduate Nursing (GN)	Pre-doctoral; various programs
1	1-5	Occupational Therapy (OT)	Post-graduate National Fellowship
1	1-5	Physical Therapy (PT)	Post-doctoral National Residency
3-4	1-5	Psychology (PSY)	Pre-doctoral; various programs
1	1-5	PSY Fellow	Post-doctoral; various programs

1	2-5	Public Health (PH)	Pre-doctoral; UC Master's program
1-2	2-5	Social Work (SW)	Pre-doctoral; various programs
1	1-5	Social Work Fellow (SW)	CCHMC Social Work Fellowship
3	1-5	Speech-Lang Path (SLP)	Pre-doctoral; various programs
1-3	1-5	Other *as approved	Varies
5 or more	1-5	Other LTT	Varies
Total: At least 29	**1-5**	**Total: At least 12, up to 17**	

Medium-Term and Short-Term Trainees: In addition to the LTT cohort and "other" LTTs, at least 15 medium-term (MTT; 40 to 149 hours) and 5 advanced medium-term trainees (AMTT; 150 to 299 hours) and at least 70 short-term trainees (STT; less than 40 hours) will participate in specialized interdisciplinary training. These trainees will be recruited to participate in portions of LEND interdisciplinary, clinical, and research activities through local graduate programs and other CCHMC training programs. Practicing professionals from CCHMC or other community agencies in LEND disciplines may also be MTT or STT. We have created a "menu" of training options for non-LTT called "LEND Lite." This menu includes an overview of the Course Objectives, schedule, and topics for individual courses or disciplinary clinical work or research. Some LEND Lite trainees elect to gain research experience and/or build leadership skills in one or more of our courses (described further in Training and education). All trainee participation in LEND Lite is approved by the AD/TD, often following individualized meetings with trainees, and Core Faculty within their discipline serve as the point of contact for these trainees. Specific agreements have been established through Audiology, Genetic Counseling, OT, and Social Work for LEND Lite trainees.

Training and education

Cincinnati LEND's curriculum provides a balance of academic, clinical, and community experiences and training content designed to build LEND trainees' knowledge and skills in providing services to individuals with autism/DD and their families.

Cincinnati LEND's content and instructional methodology is based on best evidence for training future leaders and meeting the needs of adult learners, especially related to technological advancements (Baumgartner & Carr-Chellman, 2024; Knowles, et al., 2014). The content is presented in ways that are suitable for trainees with different knowledge, skill levels, and backgrounds. We draw from Experiential and Transformative Learning theories by providing theoretical knowledge and practical experiences as well as creating the circumstances for learners to engage in critical self-reflection of assumptions and discourse on socially significant topics (Dillard, et al., 2024; McClain, 2024). These transformational experiences lead learners into the future prepared to provide high-quality screening, evaluation, and services to individuals with or at-risk for autism/DD and their families, create and lead innovative systems of care, develop evidence-based policy, challenge status quo and eliminate barriers to care, and train subsequent generations of leaders. Our LEND program is highly accomplished based on a long-standing academic affiliation between the UC Department of Pediatrics, DDBP, and collaborations with other regional universities and local, state-level, and national MCH-related and disabilities-related organizations which underscores our local, regional, state, and national impact.

The *Interdisciplinary LEND Curriculum (ILC)* is the centerpiece of Cincinnati LEND. LTTs from a minimum of 12 disciplines participate together in the nine-month, at least 300-hour ILC while concurrently pursuing graduate or post-graduate training in an MCH profession, parenting or being a sibling of a child with autism/DD, living with a disability, or working in the field. The ILC consists of five

elements: *Interdisciplinary Leadership Seminar (LEAD)*, *Interdisciplinary Training Team (ITT)*, *Core Course (CORE)*, *Seminar in Evidence-based Methods (SEBM),* and *Disciplinary Clinical Experiences*. See **Attachment 5** for review of the Curriculum Graphic. Cincinnati LEND meets every Tuesday and Thursday from mid-August through April, observing the UC holidays and breaks. Trainees participate in disciplinary experiences from 8:00am-12:00pm and as a cohort for afternoon courses. Tuesdays include LEAD from 1:00pm-3:00pm and ITT from 3:00pm-5:00pm. On Thursdays, trainees participate in CORE from 1:00pm-3:00pm and SEBM from 3:00pm-5:00pm. Opportunities for interdisciplinary teamwork are maximized throughout the curriculum. Trainees are divided into interdisciplinary teams for all group activities. All in-class, break-out discussion or experiential learning groups are thoughtfully prepared and assigned to include members of various disciplines to optimize interdisciplinary learning amongst trainees.

Cincinnati LEND's curriculum content and methodology are developed and implemented by the PD, AD/TD, and Core Faculty who comprise the Interdisciplinary Training Committee (IDTC). Detailed descriptions of the roles and responsibilities of program leaders, Core Faculty are provided in **Attachment 2: Staffing Plan and Job Descriptions**. The curriculum is guided by the MCH Leadership Competencies 5.0 (MCHB, 2026), publications on life course theory (e.g., Bazzano et al., 2025; Fine & Kotelchuck, 2010) and Community Factors of Health (Healthy People 2030), leadership works such as Bill George's *True North*, Association of University Centers on Disabilities' (AUCD) Neurodevelopmental Disabilities (NDD) Curriculum Resources, The Autism Collaboration, Accountability, Research, Education, and Support (CARES) Act of 2024, and other MCH initiatives such as the MCH Navigator Training Portal. All LEND content, materials, assignments, and correspondence is done through the Canvas by Instructure learning management system.

Academic

The topics required in the training and education section of the curriculum will be **bolded** throughout the next several paragraphs. The MCH Leadership Competencies 5.0 (MCHB, 2026) describe competencies required for an MCH leader and serve as the foundation of the Cincinnati LEND curriculum to enable trainees to build the knowledge and skills they need to become future leaders in MCH. All LTTs participate in the *Interdisciplinary Leadership Seminar*, a two-semester, 90+-hour series that engages trainees in individual and team-based experiential learning through seminar sessions, small group discussion, and team and individual projects. All trainees complete the StrengthsFinder 2.0 during their first few weeks of LEND and receive detailed reports describing their leadership strengths. In addition, all LTTs complete the MCH Navigator Self-Assessment during their first month of the program. The MCH Leadership Competencies provide the framework for the ILP developed by each trainee to guide their work during LEND and their future career. For the ILP, LEND trainees write SMART goals based upon five MCH Leadership Competency areas and reflection on their leadership strengths from StrengthsFinder 2.0. This project affords trainees the opportunity to learn and practice **performance measurement** through writing quantifiable goals and monitoring progress over time in their own work. The ILP guides leadership development through periodic reflection, self-assessment, application to content covered in *Leadership Seminar* and mentorship from LEND Leadership and Core Faculty promotes personal and professional growth. Progress on the ILP is monitored and facilitated through individualized mentorship sessions in October and April with LEND Leadership and at mid-year with the trainee's assigned Core Faculty. Through in-class sessions, we cover leadership topics such as Daniel Goleman's emotional intelligence skills and leadership styles, team functioning and dynamics, and identifying one's authentic leadership style based on *True North*. All trainees are provided with a copy of the *True North Fieldbook: Emerging Leader Edition* to use throughout the year and guide their Leadership Showcase activity at the end of the year. Our curriculum emphasizes the importance of

connecting with empathy and leading with compassion in healthcare based on work at the Empathy and Relational Science Program at Massachusetts General Hospital. We also provide didactics in managing crucial conversations, negotiation and conflict resolution, and administrative issues, specifically focused on team dynamics, clarification of roles and responsibilities, budgeting, and supervision/management. In conjunction with these sessions, trainees complete the Thomas-Kilmann Conflict Mode Instrument and are provided with their profile as well as a copy of the book *Crucial Conversations*. The AD/TD leads sessions on sharing diagnoses and difficult news with patients/families where we utilize actors from a local acting company to provide trainees with these simulated experiences. These sessions engage trainees in small interdisciplinary teams so they can learn how members of other disciplines may communicate with families and individuals with autism/DD, especially when the experiences are emotionally charged. Guided by Christina Maslach's *The Burnout Challenge*, we also engage trainees in discussions about burnout, compassion fatigue, and strategies to mitigate these professional hazards in themselves and their team members in their work as future MCH leaders.

To further prepare trainees for leadership roles and effective **interdisciplinary and person-/family-centered practice**, we provide innovative sessions on professionalism (interactive escape room activity) and effective communication for MCH leaders. Trainees then work with other members of their discipline, or individually in some disciplines, to prepare a disciplinary presentation. Each discipline group takes turns presenting to other trainees and faculty on educational and licensure requirements, common diagnostic approaches for autism/DD of their discipline, and general information about their work with individuals with autism/DD and their families. Non-clinical trainees, including family trainees and trainees with disabilities, provide an overview of their journey to the LEND program, discuss their research and/or advocacy interests, and a plan for how they will use the training experience to further their personal and/or professional goals. Additionally, we engage our Family Discipline Faculty and other UCCEDD Family Support Team members to provide sessions on Family-Centered Care and Charting the Life Course.

Cincinnati LEND values the provision of **culturally and linguistically appropriate services**. As a part of one of our early sessions in Leadership Seminar, we invite a partner from CCHMC's Language Access Services to discuss legal expectations and best practices in meeting language and communication needs of patients and families. We also discuss language differences as a barrier to quality healthcare. LEND trainees frequently work with qualified medical interpreters in various clinical settings. Many LEND trainees develop an ILP goal around improving their skill and confidence working with interpreters using various languages. In the past few years, we have increased the number of community outreach classes and Empowering Families Symposium sessions offered in Spanish, and we have strengthened our relationship with Su Casa Hispanic Center. In addition, more patient materials and resources are being translated into various additional languages. DDBP's Building BRIDGES (Belonging, Respect, Insights, Dialogue, Growth, Empowerment, and Support) Program, co-led by a Cincinnati LEND graduate, also works to understand the unique needs of individuals and communities we serve so that culturally and linguistically appropriate services are provided.

For trainees interested in building their skills in teaching, opportunities are available throughout the curriculum and based on trainees' interests. For example, PSY or DBP trainees co-teach in the mental health & DD session, SLP trainees present on language development, OT and PT trainees co-teach the session on Cerebral Palsy, and GC trainees support sessions on genetics.

Trainees complete various LEND Leadership Projects individually or in interdisciplinary teams which are designed to assist LEND trainees in building skills and knowledge in the MCH Leadership Competency areas. These include the Family Mentoring Project (FMP) and either the Community Leadership Project (CLP) or the Early Intervention Project (EIP), all discussed further below. In addition

to projects and in-class sessions, trainees enroll in the AUCD Emerging Leaders Community and participate in national and state-level meetings with other LEND and MCH trainees to solidify their connection with the national community of MCH trainees. At the end of the year, all LTTs have the choice of either creating and presenting an e-portfolio that summarizes their work and successes or giving a 10-minute Leadership Journey presentation guided by the *True North* curriculum.

All trainees participate in *Interdisciplinary Training Team (ITT)*, a two-semester, team-based experience (50-60 hours) that uses the structure of an **interdisciplinary team approach** to comprehensive assessment. The course begins with didactics on the comprehensive evaluation process for autism/DD, the basics of standardized testing, including psychometric properties, and common modifications that may be necessary in testing. Trainees discuss assigned readings regarding clinical assessment of young children and interdisciplinary team evaluations and application of life course theory to the evaluation process. Next, trainees complete supervised disciplinary evaluations of children who present with possible autism/DD and/or observe evaluations by other disciplines which occur during morning LEND hours. In afternoon ITT team meetings with all LEND disciplines represented, trainees discuss evaluation findings and work together, led by the family trainees and trainees with disabilities, to develop a person-/family-centered approach to the remaining evaluation visits, information sharing session (ISS), and follow-up with the family. Trainees learn about the methods and tools used by other disciplines through direct observation of evaluations by other disciplines, presentations by trainees, and online videos and resources. Team meetings provide an opportunity for trainees to develop skills for communicating across disciplines, facilitating meetings, managing conflict, and building an understanding of the change from a medical model to an **interdisciplinary team model** to include the family members and patients themselves. Trainees discuss how to use acquired **critical thinking skills** to apply these interdisciplinary experiences in settings with fewer resources and less protected time for interdisciplinary team meetings that they may experience in the future. Five trainees are selected from each team each semester to assume leadership roles and manage the team (Evaluation Coordinator, Family Advocate, Meeting Leader, Technology Leader, and Note Taker). By the end of the course, trainees can verbalize their own roles and responsibilities as well as those of other disciplines, and the basics of constructing an interdisciplinary team to meet patient needs. We have incorporated a new tool in our ITT called Empathy Mapping (Cairns et al., 2021), a visual tool used to gain a deeper understanding of family members' perspectives, needs, and challenges. Use of this tool has increased the role of our LEND family trainees who lead discussions of the Empathy Map during each ITT team meeting.

Interprofessional Case Simulation Series was designed to improve **interprofessional skills** in collaboration with the Nisonger Center LEND and Illinois LEND. Using videoconferencing technology, LEND trainees from different disciplines and programs engage in four (4) one-hour sessions across the training year. Case vignettes that were developed over multiple years in collaboration with individuals with lived experience are used to engage trainees in interactive discussions and case coordination planning. Trainees interact with additional individuals with lived experience and those from disciplines that may not be present in their home LEND program. They are also exposed to various state-level differences in care systems and national resources and supports for individuals with autism/DD and their caregivers.

Much of the didactic content of Cincinnati LEND is provided in *CORE Course*, a two-semester, 60-hour course that employs self-study, lectures, small and large group discussions, and experiential activities to provide an overview of autism/DD from a developmental perspective. All LTTs participate in this course, and it is a popular course for LEND Lite trainees (MTTs, AMTTs, STTs, and practicing professionals). Initial sessions ground trainees in theoretical concepts including the history of disabilities and the **various models of disability** (e.g., social, charity, neurodiversity, and medical) as well as

language in the disability field (e.g., person-first & identity-first terminology) and the **interaction of disability with other life experiences**. Using public health approaches and life course theory as a framework for the remainder of the course, sessions progress through a **lifespan developmental approach**, from early and school-aged childhood topics in fall semester to **services and supports in adulthood** by the end of the academic year. A **person-/family-centered, evidence-based approach** is incorporated throughout the course. Incorporating adult-learning theory, trainees engage in experiential activities and small group work to increase trainee knowledge and skills across foundational topics in the autism/DD field. (See **Attachment 5** for a sample syllabus with topics). Trainees complete essays at mid-term and at the end of year to demonstrate their learning and how they will apply gained knowledge to their clinical and community-based work.

The AD/TD, dually trained in clinical psychology and public health, leads sessions related to public health perspectives and **systems of care for children and youth with special health care needs** including those with autism/DD. Topics covered in these sessions include life course theory, community-based systems of services and factors that influence health, medical home, differences in health status, and MCH programs, including Title V programs. For example, CityMatCH's Life Course Game is played in small, interdisciplinary groups, and debriefing occurs as a large cohort with trainees and faculty to reflect on the cumulative impact of community factors that impact health. Public health sessions are based upon the curriculum resources co-authored by the AD/TD available in AUCD's National NDD Core Curriculum Resources on the topic. These include using case studies, patient journey maps, and pop culture movie clips for trainees to identify systems-level interventions and to develop ideal trauma-informed care pathways. Trainees discuss their plans as a large group and share ideas on expansion of current public health programming in our city, region, or state or creation of new programming. We also emphasize Ohio's Title V MCH priorities, including care coordination through a medical home for children with and without special health needs, infant safe sleep practices, breastfeeding guidelines, and promotion of developmental screenings for children ages 9-35 months (Ohio Department of Health, 2026). The Evidence-Based Case Discussion (EBCD) sessions also integrate community, societal, and systems factors throughout the experience.

Cincinnati LEND's curriculum highlights that effective systems of care for individuals with autism/DD and their families are developed and maintained in partnership with families and patients themselves, guided by the principles of **person-/family-centered care**. In LEAD and CORE sessions and through two family-focused projects, we provide opportunities to explore the importance of family-centered values and how organizations effectively embody these values. ITT offers trainees the opportunity to apply learning and develop skills necessary to be person-/family-centered. Readings and classroom presentations/panel discussions help trainees build knowledge and skills for person-/family-centered practice. Past trainees report significant learning on person-/family-centered care from having caregivers of individuals with autism/DD on ITT teams and in classroom panel discussions. The FMP is a leadership project that provides a longitudinal opportunity for trainees to understand the parent perspective about their child with a disability. Trainees complete visits within the family's home, a community location (e.g., store/mall, park, birthday party), medical or educational appointment or meeting (e.g., IEP meeting, speech therapy visit), or virtually to have a discussion of the child's diagnosis and treatment journey. Trainees reflect on this experience through written assignments. Trainees consistently rate the FMP as one of the most valuable learning experiences of LEND. The FMP also provides a way for 10-13 families per year to contribute to the training of LEND trainees. LTTs also participate in the Family-Centered Care Observations Project in which Family faculty and trainees observe clinical trainees and then provide feedback based upon implementation of practices such as shared decision making, prioritization of needs, tailoring recommendations, and connecting families to

resources, all core concepts of patient-/family-centered care (Institute for Patient- and Family-Centered Care, n.d.)

Cincinnati LEND demonstrates commitment to family-centered care as evidenced by taking the lead to develop and launch two programs within DDBP: FAC and Family Navigation (FN). DDBP-FAC's mission is to promote partnership between families and care providers to enhance family-centered practice. LTTs often participate in FAC meetings as part of ILP goals. Additionally, the FN program has served over 1,300 families since 2015 on care coordination and connection to educational and community services. Feedback from families has been extremely positive as evidenced by completion of multiple action plans to reach family goals. LTTs, especially Family trainees, meet with and observe the work of the Navigation team during LEND. Both programs have not only been in partnership with family members but have offered employment opportunities and honoraria to competitively compensate family members for their time and consultation. Throughout the current funding cycle, Cincinnati LEND staff and faculty have also worked to assure that patients and family members participating in LEND activities receive honorarium payments and reimbursement for travel expenses for their efforts as panelists, FMP mentors, and consultants to the program.

The **medical home model** and its value in providing continuous, accessible, safe, culturally and linguistically appropriate care and coordination for individuals with autism/DD is also emphasized and presented as a best practice model for primary care by the American Academy of Pediatrics (AAP). These concepts are introduced in sessions on public health and systems of care, applied in discussions and planning for patients and families in ITT, and further analyzed in sessions on Charting the Life Course and Medical Home.

CORE is structured along a **lifespan developmental approach** with prenatal and early intervention topics at the beginning of the year, school-age and adolescence covered in the middle, and **transition to adulthood** by the end of the training. Invited experts from community partnerships, the Moving to Adult Care (MAC) clinic, and Project SEARCH provide sessions on self-determination, self-advocacy, sexuality, healthcare and education transition, including post-secondary education, housing, and meaningful employment so that trainees learn about **health care and community transitions from youth to adult services and supports**. Advocates with disabilities participate as panelists in these sessions to enhance trainees' exposure to adults with autism/DD and hear their lived experiences, successes, and challenges with moving to adulthood. Trainees also hear from self-advocates during the Ohio DD Awareness and Advocacy Day during the presentations and visits with legislators. Some of our trainees also complete their CLP in settings that serve adolescents and adults with autism/DD. Additionally, at least one SEBM project each year focuses on youth and/or programming for young adults with autism/DD. We also work closely with the team from the Successful Transition of Autistic Youth (STAY) project funded by HRSA focused on developing, implementing, and evaluating a transition model in five neighborhoods in the city of Cincinnati. Trainees may also rotate through clinical experiences focused on this population (see ***Clinical***). In 2021-2022 and 2023-2024, we had Adult Services trainees who focused on the provision of services to transition-aged youth with Autism/DD. The Adult Services trainees worked with the Timothy Freeman, MD, Center for Intellectual and Developmental Disabilities to develop a toolkit that is now used to help patients who are transitioning from Cincinnati Children's to the Freeman Center. The Freeman Center provides quality, coordinated, interdisciplinary, patient-centered care for adults with intellectual and developmental disabilities in addition to training medical students and residents. The Cincinnati LEND and Freeman Center teams are exploring ways LEND trainees can be involved in clinical and research activities within the Freeman Center in upcoming years. Family Medicine fellows have completed clinical hours at the Freeman Center

while participating in portions of the LEND curriculum. We also have plans to have psychology trainees do rotations with the Freeman Center's Director of Mental and Behavioral Health Services in the future.

Training in **policy-making processes** occurs in a series of four, two-hour in-class sessions and additional experiential activities to provide trainees with opportunities to learn about **Title V MCH Services Block Grant Program and related MCH legislation** impacting the DD community as well as effective advocacy strategies. In this series, led by our LEND Policy Specialist, trainees practice communicating with community members, including state and federal legislators through in-class role-plays and visits to the Statehouse and Capitol Hill. The Policy Specialist works with the trainees to ensure they receive AUCD's Disability Policy News and Action Alerts as well as weekly "policy pieces" based on current topics in the DD field. To start the series, the Policy Specialist reviews the structure and legislative processes of Ohio and federal governments as well as state-level MCH and DD policy and priorities. In one session, trainees participate in an innovative role-play exercise where they represent groups of stakeholders and work to educate mock legislators (other trainees) as to why certain pieces of a Mr. Potato Head toy should be added, deleted, or moved. This session is rated highly every year, and trainees describe having a better understanding of educating vs. lobbying legislators after this session. Additionally, as an entire cohort of trainees and faculty, we attend the annual Developmental Disabilities Awareness and Advocacy Day at the Ohio Statehouse in Columbus, Ohio, where all trainees have at least two scheduled meetings with their state-level leaders to demonstrate their skills in communicating effectively with policymakers. Issues selected to discuss with policymakers are identified by Drs. Ayers, Weber, Smith, and policy specialists. ADV trainees research and develop info sheets under mentorship from Dr. Ayers. Prior to this event, we offer an in-class session in which trainees role-play educating mock legislators (Core Faculty) on current state-level DD policy. Trainees are also invited to apply to attend Disability Policy Seminar in Washington, DC. In the final session of the series, we engage the trainees in small, interdisciplinary groups to discuss common issues that arise in clinical practice, educational settings, and community living. They work together to develop an advocacy plan focused on the systems-level changes necessary to address the issues in the long-term. In addition to delivering the Policy curriculum, the LEND PD, AD/TD, Advocacy Faculty, and Policy Specialist have published this curriculum (Weber, et al., 2019). The LEND PD and Policy Specialist developed the policy measure to assess LEND trainees' knowledge, skills, and application of policy that is used in the LEND Program Quality Improvement (LPQI) initiative (see p. 31). The LPQI Research subcommittee, led by LEND PD, Dr. Smith, recently submitted a manuscript that is under peer review highlighting the impact of LEND training on trainee's knowledge, skills, and application of policy.

Cincinnati LEND offers multiple sessions in CORE related to best practices in screening, including discussions and practice with the Ages & Stages Questionnaire (ASQ), Modified Checklist for Autism in Toddlers (M-CHAT) and surveillance and screening resources available in Ohio's Sparkler app for families of young children. Trainees learn **best practices for autism screening, diagnosis, and evidence-based interventions at various levels of support needs across the lifespan** in various clinical encounters. DDBP follows a multidisciplinary assessment model for children with concerns for autism, reflecting key features outlined in the AAP Clinical Report on Identification, Evaluation, and Management of Children with Autism Spectrum Disorder (Hyman et al., 2020) and reaffirmed in November 2025. The "arena model" is an innovative clinic model in which many LEND trainees participate. "Arena model" is the term used to describe a multidisciplinary diagnostic evaluation model including assessments from multiple professionals (including a DBP, PSY, and SLP) occurring on the same day, either simultaneously or in short succession, involving history/physical examination, Autism Diagnostic Observational Schedule, 2nd Edition (ADOS-2) or speech/language testing, and the administration of developmental/cognitive, adaptive, and behavioral measures (Williams-Arya et al.,

2019). Trainees participating in diagnostic evaluations gain skills using various standardized measures, including the ADOS-2, Autism Diagnostic Interview-Revised (ADI-R), Developmental Profile 4 (DP-4), and multiple Wechsler Scales. They score, analyze results, and write comprehensive reports (or complete other suitable documentation for their discipline). Trainees may also gain experience in providing evidence-based intervention services through a variety of modalities including individual treatment, co-treatment with at least two disciplines, and group treatment. A variety of group treatments are provided through DDBP including those addressing anxiety, social skills, daily living skills, parent training, and behavioral concerns. Trainees in nursing and DBP also gain skills in performing routine follow-ups and providing medication management for a variety of presenting concerns.

Cincinnati LEND integrates the MCH Competency #7: Community Health Factors across courses, in clinical supervision, and across research and policy projects so that trainees learn about how the **social environment** impacts health. We provide learners with foundational understanding of the 10 essential public health services to promote population health. We discuss the impacts of socioeconomic status, education, place-based factors (e.g., community safety/violence, transportation), employment in the context of resources, and support for physical and mental health. Additionally, content focuses on risk and preventive factors, vulnerable periods, social support, and environmental influences on health. Mentorship with clinical supervisors and core faculty incorporates individualized instruction on how community factors influence health and access to services. Supervisor and faculty coaching across LEND experiences and projects affords trainees opportunities to identify strategies to address community factors to promote positive outcomes for individuals with autism/DD and their caregivers and communities. As will be further outlined below, our interdisciplinary research teams also work with community/policy partners to foster collective action and engage in research recruitment, data collection, and dissemination of findings *with* those in the impacted communities.

The structure of the Cincinnati LEND curriculum allows **emerging needs and special interests in the field of autism/DD** to be incorporated as necessary through didactic, clinical, and research experiences. CORE sessions have addressed topics including mental health concerns in individuals with autism/DD, suicidality, sexual health, and trauma-informed care, all essential for leaders in the autism/DD field. In SEBM, trainees have been introduced to topics including impacts of prenatal opioid exposure on development, issues facing adolescents/young adults with autism/DD, and Cerebral Visual Impairment (CVI). In response to rising youth suicide rates, CCHMC's Mental and Behavioral Health Institute (MBHI) is leading a system-wide shift toward proactive, evidence-based prevention using the Zero Suicide framework. LEND faculty and trainees receive training on Counseling on Access to Lethal Means (CALM), an intervention focused on reducing access to the most common and lethal methods of suicide, particularly firearms. Dr. Weber is collaborating with community partners to develop evidence-informed Trusted Adult Trainings, which aligns with the Ohio MCH priority to increase the percent of adolescents who have at least one adult outside the home who they trust. Additionally, elopement continues to be an issue that significantly impacts our community. In response to two children with autism dying by drowning after eloping from their homes, we have been diligent about ways to educate families and work with local law enforcement agencies on safety skills and systems improvements. These efforts have included educating state legislators on a safety alert bill, hosting an episode on the Cincinnati Children's Young & Healthy podcast, participating in two Facebook Live events with EmPath for Autism, developing and sharing an Emergency Response Plan Template, and designing and implementing water safety and adaptive swim lessons for children with autism/DD using a multidisciplinary approach.

Since the COVID-19 pandemic, **innovative and alternative methods of providing healthcare such as telehealth** have become a part of standard care across CCHMC. The Center for Telehealth

(CTH) provides the infrastructure to ensure convenient and timely care to patients and families within their homes. Video and voice data are encrypted so that video visits adhere to all the privacy regulations of an in-person visit. LEND faculty and trainees utilize telehealth visits on a regular basis depending on the needs and preferences of patients and families. Telehealth appointments have improved access to care for families who faced barriers in the past including geographic barriers, transportation barriers, time constraints, physical limitations, childcare challenges, as well as limited access to specialist care. Additionally, access to specialist care has also been improved through Cincinnati LEND's use of ECHO (Extension for Community Healthcare Outcomes) programs. ECHO represents an innovative tele-consultation/mentoring platform connecting local professionals with specialists at academic medical centers to provide best-practice treatment protocols, case-based learning, and co-management of cases. ECHO helps equip community-based providers with best-practice care and has proven particularly effective with underserved and rural communities.

Dr. Smith has served as the psychologist and co-principal investigator on the ECHO Autism for Primary Care Providers (PCP) program since 2017. The team is comprised of a social worker, nurse practitioner, family member, speech-language pathologist*, occupational therapist* and developmental-behavioral pediatrician* (*denotes Cincinnati LEND graduate). In 2025, the ECHO Autism team started an ECHO Autism for Allied Health Providers (AHP) including speech-language pathologists, occupational therapists, physical therapists, clinical counselors, and behavior specialists from various CCHMC locations and community agencies. The ECHO Autism programs have reached hundreds of primary care and allied health providers from across the country including Ohio, Kentucky, Indiana, Georgia, Pennsylvania, Michigan, North Carolina, Louisiana, California, and Alabama. Outcomes have been published in the *Journal of Developmental and Behavioral Pediatrics* by Smith et al. (2025).

Dr. Weber has served as co-principal investigator on the Project SCOPE (Supporting Children of the Opioid Epidemic) ECHO team since its development in 2018-2019. Project SCOPE, a collaboration between the two Ohio LEND programs and Wyoming's UCEDD, Wyoming Institute for Disabilities (WIND), was awarded a federal grant in 2019 to train up to 16 states in the Project SCOPE ECHO model over a three year-period. The SCOPE ECHO program focuses on reaching providers with the goal of building awareness and skills for supporting young children who have been impacted by the opioid crisis. Outcomes from SCOPE ECHO have been published in *Frontiers in Public Health* (Weber et al., 2024). Additionally, Dr. Weber co-developed Act Early ECHO in 2020 in collaboration between CDC's "Learn the Signs. Act Early." Ambassadors to Ohio, Virginia, and Wyoming. This ECHO emphasizes the five components of early identification, addressing systems-level barriers to quality early care and education, and supporting resiliency in caregivers and providers of young children. LEND trainees have participated in both ECHOs by developing content, collecting data, participating as hub team members/subject matter experts, and disseminating best practices.

Dr. Smith was awarded a LEND supplement through MCHB/HRSA in fall 2025 to develop an ECHO for Community Inclusion, using the ECHO model as a community of practice for those working within cultural and recreational organizations (e.g., zoo, theatres, museums, parks, etc.) across the greater Cincinnati region. This program was launched in April 2026. In addition to Dr. Smith as the team lead, the team is comprised of a DDBP SLP and Cincinnati LEND graduates in OT, PT, FAM, and ADV.

Critical thinking, science-based judgment, and research methods are the foundation of the *Seminar in Evidence-based Methods (SEBM) Course*. This 60-hour, two-semester series is conducted mostly in interdisciplinary project teams and provides a structured, faculty-supervised experience in the evidence-based cycle of inquiry that should be applied to answering a question, filling a knowledge gap, improving a process, developing a policy, or testing a hypothesis. SEBM also includes didactics, small group discussion and self-study concerning the following: protection of human subjects through CITI

training; participatory action research; basics of methodology and handling of data; questionnaire and survey design; reviewing and summarizing the literature; and quality improvement (QI) methodology in clinical and community-based settings. Faculty present their projects to the trainees in Week 1. Trainees then submit preference rankings based on topic interest or desire to expand skills in certain aspects of the research process. Three or four trainees are assigned to interdisciplinary teams, mixing disciplines and previous research experience across teams to make sure the experience is suitable for trainees with different knowledge, skill levels, and backgrounds. Faculty-led trainee teams develop a question, review and assess evidence-to-date, design and conduct a method of inquiry, collect and analyze data, and apply it back to the original question. Some projects have included grant writing or manuscript preparation as well. Teams are required to seek the input of a policy/community partner from a local, state, or national level disabilities and/or public health-related agency or organization in relation to the project. Teams develop project timelines and deliver mid-year oral group presentations on progress to date. They prepare a final poster and abstract to disseminate research findings at a joint session with The Ohio State University's Nisonger Center LEND Program trainees, which also includes an audience of state-level partners and policymakers.

LEND research projects are responsive to needs and designed to have a direct impact on the health and well-being of individuals with autism/DD and their families through translation of new knowledge into policy, practice, and systems change. See Table 2 for examples of recent project titles, dissemination, and impact. Through this approach, trainees expand critical thinking skills; increase knowledge on evidence-based approaches; enhance interdisciplinary teamwork skills; integrate policy considerations into their work; appreciate protections for human subjects in the design of projects; and apply various web-based resources to make research and other evidence-based projects more collaborative and accessible to audiences in the disabilities field. With ongoing faculty support in the next project period, further dedication will be focused on providing trainees with more exposure to dissemination of SEBM team results through a variety of means including peer-reviewed publication, policy analyses, social media, and materials for general populations.

Table 2. Select SEBM Team Projects from Current Funding Cycle

Title of Project	Multi-year Project	Funding Obtained	Local/regional presentation	National Presentation	Educational Material	Peer-reviewed publication
Families' experiences with the path to an autism diagnosis	✓	✓	✓	✓		✓✓✓✓
Evaluating computerized assessment of cognition in Down syndrome with caregiver administration	✓	✓	✓✓	✓	✓	✓
Developing and evaluating a Parent-Mediated Intervention for Autism	✓		✓	✓	✓	✓✓✓
Sensory Processing Patterns of Preschool-Aged Children with Prenatal Opioid Exposure	✓		✓	✓	✓	✓
Building a Curriculum for Caregivers to Support Autistic Adolescents Transition to Adulthood	✓	✓	✓		✓	

Understanding Needs & Services for Adult Healthcare: The Freeman Center	✓	✓	✓	✓	✓	
Understanding the Educational Needs of Caregivers of Children with CVI		✓	✓	✓	✓	
P2P Networks: Systems that Optimally Support Parents of Deaf & Hard-of-Hearing Children			✓	✓		
Perceptions of Accessibility and Inclusion in the greater Cincinnati region	✓	✓	✓	✓☑		

Key: ✓ = # of Presentation, Product, or Publication ☑ = International Presentation

Cincinnati LEND trainees are exposed to **how to use data for continuous QI in clinical and community-based practice** in coursework and clinical care. While many trainees observe and participate in QI efforts in their clinical work, QI methodology has also been incorporated into the LEND curriculum in a variety of ways. First, as part of the didactic sessions in SEBM, trainees are exposed to QI methodology such as Plan-Do-Study-Act (PDSA) cycles for increasing access to diagnostics and modifying treatment models. Additionally, trainees have participated in a Group Level Assessment (GLA) to identify ways in which the program can be improved from a trainee perspective including how to make content more suitable and accessible to trainees with different knowledge, skill levels, and backgrounds. Cincinnati LEND trainees also have access to the QI education offerings from CCHMC's James M. Anderson Center for Health Systems Excellence.

Evidence-Based Practice (EBP) is integrated into all CORE and ITT sessions. Additionally, seven sessions (14 hours) are offered throughout the year under the title Evidence-Based Case Discussions. These sessions cover a range of diagnoses, including Down syndrome, Fragile X syndrome, Spina Bifida, Feeding Disorders in developmental disabilities, Cortical Visual Impairment/Visual Disabilities, Developmental Coordination Disorder, and ADHD. Each session is led by professionals with specialized expertise in the featured diagnosis. In preparation, trainees review a scholarly journal article on a related developmental disability and examine a patient case study provided by the presenters. During the session, interdisciplinary clinical teams present evidence-based treatment practices currently used in real-world settings. These presentations are followed by perspectives from caregivers and individuals with lived experience who are compensated for their time and contributions. Trainees then break into small groups to discuss the case study, applying insights from the assigned article and responding to prompts about best practices for the patient in the case. The session concludes with a brief large group discussion, allowing participants to share reflections and integrate key learning points.

Clinical

Disciplinary Training Experiences: This experience varies in duration (116 – 232 total hours) over two semesters depending on the level of training and specific training requirements of each discipline. Clinical, practicum, and field placement opportunities are provided for both clinical and nonclinical trainees including family trainees and trainees with disabilities. The LEND Self-Advocacy Discipline Competencies are used as a supplement to the Cincinnati LEND curriculum to support trainees with disabilities. The Cincinnati LEND Family Faculty is actively involved in the AUCD LEND Family Discipline Network (LFDN) workgroup that meets frequently to identify and address issues unique to the Family discipline within LEND and support successful integration and growth of family trainees into LEND programs. This groupwork is currently working on LEND Family Discipline Competencies similar to the LEND Self-Advocacy Discipline Competencies used across the LEND network. Non-clinical trainees, such as FAM, ADV, DS, PH, and CE trainees receive individual and small group mentorship and supervision from their Core Faculty member to develop and achieve individualized goals

connected to their ILPs. For example, trainees have completed projects in the following areas: 1) developing a guide for new direct support professionals to learn best practices when working in someone else's home, 2) identifying accessible housing options in the greater Cincinnati area and partnering with local organizations with plans to expand these options, 3) writing and publishing in-depth policy analyses about changes in education policy that impact students with disabilities, 4) publishing in an academic journal about new theories about autism, and 5) developing and implementing exercise and dance programming for a wide array of children and youth with autism/DD. Family and Self-Advocate trainees also complete modules in Partners in Policymaking and create policy briefs for use in legislative visits.

The Cincinnati LEND setting within DDBP maintains extensive and innovative clinical training sites. DDBP services are an integral part of the community service delivery system for children and adolescents with autism/DD in the Tristate region of Southwestern Ohio, Southeastern Indiana, and Northern Kentucky. CCHMC's catchment area is situated in the middle of the Cincinnati-Middletown, Ohio-Kentucky-Indiana Metropolitan Statistical Area consisting of fifteen counties in Ohio, three in Indiana, and seven in Kentucky. During FY 2025, DDBP provided diagnostic and/or treatment services for 10,496 children, adolescents, and young adults through exemplary service programs which included interdisciplinary, evidence-based diagnostic and treatment programs. Professions involved in the diagnostic and treatment process include AUD, DBP, GN, OT, PT, PSY, SW, and SLP.

All LTTs in clinical disciplines are provided with extensive clinical training opportunities to increase their knowledge and skill in service delivery for individuals with autism/DD. Specifically, all trainees participate in live supervision most of the time with clinical supervisors, preceptors, or mentors. Trainees gain experience with a variety of patient populations including autism, Down syndrome, Spina Bifida, Complex ADHD, various genetic syndromes (e.g., Fragile X, Angelman, Rett), Intellectual Disability, and general developmental delays. Patients seen in the following clinics reflect the cultural, social, and ethnic diversity of the community. Trainees are afforded clinical and observational experiences in the following:

DDBP's Pediatric Clinic sees children from infants to young adults who are referred by their physicians when they have known or possible developmental delays, disabilities, learning difficulties or behavior challenges. Children and adolescents are seen for on-going follow-up care by DBPs and Nurse Practitioners. DDBP also has strong collaborations with the Adolescent Medicine, Child and Adolescent Psychiatry, Neurology, Genetics, and Gastroenterology divisions.

The Kelly O'Leary Center for Autism (TKOC), a member of the Autism Care Network since 2008, is a multidisciplinary diagnostic, treatment, and research program that is recognized as one of the elite autism centers in the U.S. TKOC is comprised of an experienced team of professionals from multiple disciplines including DBPs, nursing and nurse practitioners, OT, psychology, social work, and SLP. TKOC provides a comprehensive multidisciplinary diagnostic battery that follows evidence-based practice guidelines as well as individual, co-treat, and group therapy models in the areas of speech, occupational, and behavior therapies to address a wide range of concerns seen in children with autism.

The Jane and Richard Thomas Center for Down Syndrome offers an innovative approach to care to ensure the optimal health, developmental status, social-emotional functioning, and adjustment of children with Down syndrome and their families. Services include structured evaluations and ongoing monitoring, individualized treatment planning based on best practices and standards of care, care coordination, support, and assistance throughout childhood, education and advocacy, research opportunities, and planning for adult care.

Spina Bifida Center provides comprehensive, family-centered, interdisciplinary, and collaborative medical care to patients with spina bifida and other neural tube defects. In addition to

excellence in medical service and care coordination, the Center is a leader in spina bifida education and research.

The Complex Care Center provides family-centered medical care to children and adolescents with complex chronic medical, developmental, and mental health conditions. In addition to managing everyday health needs, the center serves as the central point of contact for coordination of medical needs by helping to coordinate care across subspecialists.

Moving to Adult Care (MAC) is a multidisciplinary clinic dedicated to helping teens and young adults with DD as they age into adulthood. MAC promotes self-management, advocacy, and continuity of care through a graduated process which involves education of patients, families, and their health care providers, coordination of ancillary services, and liaison with community resources. The model of care concentrates on the whole person, including moving from school to work, education and vocational training, community living, adult and social relationships, and pediatric to adult healthcare.

Next Step Clinic helps families of preschool-aged children with prenatal opioid exposure by providing comprehensive diagnostic evaluations to guide the "next step" in development. Patients are evaluated by pediatric providers including psychologists, developmental-behavioral pediatricians, occupational therapists, and speech-language pathologists who are highly trained to support children and families impacted by substance use disorders.

The Vinaya Rett Syndrome and Related Spectrum Disorders Clinic provides comprehensive, multidisciplinary care for children and adults with Rett syndrome and related spectrum disorders. Specialists work collaboratively to diagnose and manage patients' complex neurological and medical needs, with the goal of enhancing quality of life for them and their families. This clinic is designated as a Center of Excellence by the International Rett Syndrome Foundation (IRSF).

Developmental Assessment Team for Infants and Toddlers (DATIT) utilizes an interdisciplinary approach to assessment for infants up to 18 months and offers collaboration with the early intervention and home visiting programs.

Brief Intensive Behavioral Treatment (BIBT) Program, led by Dr. Laura Srivorakiat, Cincinnati LEND graduate, serves youth with autism/DD who have challenging behaviors and require a higher level of care than weekly outpatient therapy services. Patients are seen twice weekly for 90 minutes each over the course of seven weeks for moderate intensity behavioral intervention.

Early Intensive Behavioral Intervention (EIBI) Program, led by Dr. Kim Kroeger-Geoppinger, Cincinnati LEND graduate, is a small-group and intensive year-round program that uses applied behavior analysis (ABA) to serve young children with autism under the age of six.

In addition to clinics within DDBP, LEND trainees participate in clinical activities in various divisions across CCHMC including, but not limited to, CCHMC's following divisions: Audiology, Behavior Medicine/Clinical Psychology, Human Genetics, Occupational Therapy and Physical Therapy, Speech-Language Pathology, and Social Work. OT trainees also participate in CCHMC's Therapeutic Interagency Program (TIP), a mental health treatment program that specifically targets preschool children by integrating child protection, mental health and Head Start preschool resources. The goals are to ensure and monitor ongoing safety, provide mental health services and enhance developmental/cognitive development for children from families with a history of childhood neglect, child abuse, family violence, or other psychological trauma. In addition to educational services focused on pre-kindergarten readiness, children receive mental health therapy, OT, and PT services. The OT at TIP is a Cincinnati LEND graduate.

Timothy Freeman, MD, Center for Intellectual and Developmental Disabilities provides quality, coordinated, interdisciplinary, patient-centered care for adults with autism/DD in addition to training

LEND trainees, medical students, and residents. The Program Manager is a recent Cincinnati LEND graduate as well.

Akron Children's Developmental and Behavioral Pediatrics facilitates at least two trainees from Akron Children's to virtually participate in Cincinnati LEND courses. They complete their clinical work in various settings including The Tom and Jill Zidian Family Autism Center, Down Syndrome Program, Neonatal Follow-up Clinic, and the Neurobehavioral Health Unit. They also visit Cincinnati Children's in-person over two days to observe clinical services.

Community

Many disciplines, including PT, SLP, CC, and PSY, participate in field placements at schools throughout the greater Cincinnati region. Over the past year, Cincinnati LEND has been partnering with CCHMC's Division of Child and Adolescent Psychiatry. Cincinnati LEND had its first CC trainee who provided School-Based Mental Health Services within a school located in Cincinnati Public School District. A CC trainee has been recruited for the upcoming year and will be supervised by the recent Cincinnati LEND graduate.

All Cincinnati LEND trainees choose between participation in the CLP or EIP. Both provide experiential components focused on programs and public health services available to MCH populations. The CLP is a service-learning model in which trainees select a local organization that serves individuals with autism/DD to provide service hours. Examples include participating in Cincinnati Zoo's Zoo Troop for All, Cincinnati Ballet's CB Moves program, Cincinnati Art Museum's Sensory Saturdays, Visionaries and Voices art classes, Boltz to Nutz farm cooking classes, and Autism Connections' adult support group. Trainees provide a final oral presentation about their experience to the other trainees and faculty at the end of the year. In the EIP, trainees select a local county and are matched with Service Coordinators and Early Intervention (EI) providers to attend at least one transdisciplinary team meeting and at least two visits in the home, which could include an evaluation and one Individualized Family Service Plan (IFSP) development session. Trainees complete a modified Home Observation for Measurement of the Environment (HOME) questionnaire to guide their reflections and final assignment. Trainees who complete the EIP also complete a reflective assignment and provide a brief oral presentation on their experience to other trainees in the CLP/EIP final session.

Partnerships

Cincinnati LEND has developed and maintained collaborative relationships with community clinics, schools, and service agencies to enhance the training program. We have also partnered and coordinated with our state Title V programs, local advocacy organizations, and community agencies to identify community needs, improve access to services, and improve systems of care for individuals with autism/DD. In addition, we have recognized the importance of partnering with cultural institutions and businesses within our region to ensure that individuals with autism/DD are welcomed into community spaces. The collaborations have been established through many of the SEBM research projects, community education endeavors, and faculty work on boards, task forces, and committees. In addition, we have developed partnerships with academic departments within our local universities, as well as other LEND and UCEDD programs in Ohio and across the country. These partnerships have been a key element to our success in accomplishing the mission of our training program. **See Attachment 3** for a list of letters from key partners voicing support and commitment to the continued relationships. See below for specific partners.

<u>Title V, Part C, home visiting programs, and schools</u>: Ohio's Part C Early Intervention System in Hamilton, Butler, Warren and Clermont Counties, Help Me Grow and Every Child Succeeds Home Visiting programs, Pediatric Improvement Network for Quality (PINQ), Women, Infants, and Children

(WIC) Program, Ohio's Infant Hearing Program (IHP)/Early Hearing Detection and Intervention Program (EHDI), Hamilton County Head Start, Cincinnati-Hamilton County Community Action Agency, Ohio PROMISE Child Care Inclusion Program, Cincinnati Public Schools, Adams County Ohio Valley School District

State Agencies: Ohio Department of Health (ODH), Ohio Department of Education and Workforce (ODE), Ohio Developmental Disabilities Council, Ohio Department of Developmental Disabilities (DODD), Ohio Department of Children and Youth (DCY), Ohio Coalition for Education of Children with Disabilities (OCECD)

Agencies focused on supporting individuals to adulthood: UC Health's Timothy Freeman Center for Developmental Disabilities, UC's IDD Education Center, UC's Transition and Access Pathways (TAP) and IMPACT Innovation Programs, Developmental Disabilities Services (Hamilton, Butler, Warren, and Clermont Counties), Found Village, STAY project

Community Organizations: Central Clinic, Autism Connections, Down Syndrome Association of Greater Cincinnati, Spina Bifida Coalition of Cincinnati, Autism and We, Our Tribe, Siblings with A Mission, Regional Autism Advisory Council (RAAC), The Arc of Ohio, Ohio Center for Autism and Low Incidence (OCALI), EmPath for Autism, Hopeful Empowered Youth! (HEY!) Cincinnati, and 1N5

Autism CARES-funded programs: In 2023, DDBP was awarded a DBP Training Program grant through MCHB, one of 13 programs across the country. DBP Fellows participate in the full Cincinnati LEND curriculum during their first year of the fellowship. The leadership teams from Cincinnati LEND and DBP Training Programs partner on several activities and participate in monthly strategic planning meetings. DDBP is also a member of the Developmental Behavioral Pediatrics Research Network (DBPNet), a scientific and clinical research network promoting coordinated research activities across 16 academic DBP programs intended to address health issues for children with autism/DD. Dr. Weber, the LEND AD/TD serves as the Ohio Ambassador for the CDC's "Learn the Signs. Act Early." campaign, which has impact across the state by educating families and health care systems regarding early identification for children at-risk for DD. In this position, she leads the Act Early Ohio State Team, working collaboratively with state-level early childhood systems leaders.

University Centers for Excellence in Developmental Disabilities (UCEDDs): Cincinnati LEND functions as the UCCEDD's interdisciplinary training program. Cincinnati LEND and UCCEDD benefit from sharing physical space within DDBP leading to a long history of collaborating on education and training, scholarly activity, product development and dissemination, TA, and advocacy that will continue in the future. Dr. Ayers, Associate Director of UCCEDD and LEND Advocacy Faculty, is the director of the Center for Dignity in Healthcare and Community Living for People with Disabilities, a five-partner national coalition.

Other MCHB training and research investments, including other LEND training programs:

Cincinnati LEND has built collaborative partnerships with many LEND programs across the country. Dr. Smith has served as a peer consultant for MCHB site visits at the West Virginia, Iowa, and Brooklyn LEND programs. Dr. Weber is also invited by the Texas LEND program to present strategies regarding how to build belonging. Dr. Weber and Dr. Smith are invited annually by the Illinois LEND program to speak on conflict resolution and strength-based leadership. Cincinnati LEND is a member of the Great Lakes LEND Collaborative (GLLC), a collaborative of LEND programs in the states of Ohio, Illinois, Indiana, Michigan, Minnesota, and Wisconsin to identify and examine issues of common interest that can lead to collaborate on projects, share program experiences, and enhance networking and training opportunities for LEND trainees and faculty. Collaborative activities include participation in regularly scheduled conference calls and meetings, holding training events, sharing regional training opportunities and resources, and providing technical assistance (TA) to new LEND programs within Region V. In

addition to the annual poster session with Nisonger trainees, the two Ohio LEND programs often share ideas on MCH issues impacting our state and innovative interdisciplinary training approaches. The Cincinnati and OSU LEND programs also partner with the Illinois LEND program on the Interprofessional Case Simulation Series described on p. 10. In addition, Cincinnati LEND is an integral pipeline to Ohio's Family to Family Health Information Center (F2F) and Parent to Parent (P2P) Programs, as many LEND family graduates serve as staff members providing support, information, resources, and training to families of children and youth with special health care needs.

Universities: In addition to mentoring out LTT cohort, Cincinnati LEND faculty partner with Xavier University and UC Doctor of Occupational Therapy Degree (OTD) programs by serving as site mentors for doctoral student's Capstone Project and Experience. For example, a recent Capstone Project focused on the development and delivery of a training program to increase the knowledge and confidence of dance instructors to better support inclusive dance programming for children with autism/DD.

Community spaces: Dr. Smith, LEND Project Director, leads the Starting Our Adventure Right (SOAR) Program. SOAR promotes safe, comfortable, and inclusive opportunities, combined with careful training and education of community members so that individuals with DDs and their families can fully participate within their own communities. LEND trainees serve a variety of roles including volunteering for events, developing training materials and visual supports, and assisting in staff training activities. Over the past 13 years, we have become a trusted inclusion resource and fostered effective, collaborative relationships with over 20 community partners including Cincinnati/Northern Kentucky International Airport, Cincinnati Art Museum, Cincinnati Zoo and Botanical Gardens, Duke Energy Children's Museum, The Children's Theatre of Cincinnati, Cincinnati Ballet, Cincinnati Parks, and MadTree. Over 4,000 community members have received training and over 40,000 people have attended sensory-friendly performances, events, flight rehearsals, programming, and access days in the greater Cincinnati region through the SOAR Program.

Technical assistance and continuing education

Continuing Education (CE) and Professional Development

Cincinnati LEND will conduct a minimum of three CE activities per year (described below) to enhance skills and disseminate new information. We will track and report CE participation in the annual performance report using the National Information Reporting System (NIRS) database. The program will market CE opportunities internally through the internal CCHMC webpage as well as through email communication to various hospital divisions in addition to the distribution lists from Center for Telehealth and Physician Liaison Programs. Marketing materials will also be shared through social media accounts for DDBP, LEND, and UCCEDD as well as through caregiver and professional Listservs maintained by the LEND Family Outreach Associate. CE needs of the community will be assessed through LEND and UCCEDD Faculty and staff's participation on committees, task forces, and other community activities. and discussed monthly at the LEND IDTC meeting. Additionally, annually, the Interprofessional Continuing Education (IPCE) team conducts a needs assessment to identify the learning needs of professionals in our region. These results are shared with LEND Leadership who shares pertinent findings with Core Faculty for future planning purposes.

Empowering Families Symposium (EFS): Cincinnati LEND will partner various organizations including UCCEDD, CCHMC Family Resource Center, UC IDD Center, Autism Connections, Down Syndrome Association of Greater Cincinnati, Hamilton County DDS, and Ohio Coalition for Education of Children with Disabilities (OCECD) to host the Empowering Families Symposium. At least one in-person or virtual event will take place each year over the next five-year cycle. This community-partnered symposium allows family members of children and youth with autism/DD and practicing professionals to

access community resources, learn information, and network with other families. Topics will include educational supports, camps and recreational opportunities, behavioral and medical supports, advocacy, financial planning, and issues related to transition to adulthood. During in-person events, family members and community professionals in attendance will have the opportunity to engage with vendors from community agencies and companies that support families of children/youth with autism/DD. CE credits are offered for practicing professionals.

Community Outreach Classes: The LEND Family Outreach Associate and DDBP Community Outreach Coordinator collaborate with practicing professionals in DDBP and LEND trainees to provide classes, trainings, and workshops relevant to families of and professionals working with children who have autism/DD. The various topics related to the needs of children and adolescents with autism/DD include behavioral challenges, functional communication skills, feeding problems, anxiety, toilet training, executive functioning deficits, and other relevant topics. Classes primarily take place at DDBP but are also conducted virtually and at area schools and community agencies. Classes are offered at various times and on various days to ensure that the schedule meets the needs of the community.

Next Step Collaborative Conference: Cincinnati LEND will host the Next Step Collaborative Conference in collaboration with Next Step Collaborative each year of the upcoming grant cycle. This annual conference will bring together professionals from healthcare, education, and social services to explore innovative strategies for supporting children with prenatal substance exposure or whose caregivers have substance use disorders. CE credits are coordinated by the CCHMC IPCE Department.

Other: Dr. Weber is an approved instructor through the Ohio Childcare Resource and Referral Agency (OCCRRA) and the Ohio Child Welfare Training Program (OCWTP). As such, she creates on-demand in-person and virtual training opportunities for early childhood education and care providers, child welfare professionals, foster parents in Ohio to receive CE credits. As previously mentioned, ECHO Autism for Primary Care Providers (PCP) and Allied Health Providers (AHP), SCOPE ECHO, and Act Early ECHO offer various CE opportunities for participants. In addition, many LEND sessions are approved for CE credit for practicing professionals from over 10 disciplines, including education, social work, and clinical counseling.

Technical Assistance (TA)/Outreach

LEND Core Faculty and staff, as well as other DDBP faculty and staff involved in LEND training activities use their subject matter expertise to provide a variety of TA/outreach to partners to support and improve the system of care for individuals with autism/DD at the national, state, and local levels. Cincinnati LEND has specific expertise in the areas of early detection and intervention of ASD, community access and inclusion, mental health services, and policy. Cincinnati LEND has worked diligently to build relationships and become a trusted resource to various community organizations and agencies. TA activities include consultation regarding program development, program evaluation, needs assessments, policy and guideline formulation, as well as more formal advisory functions. TA needs of the community are assessed through a variety of means including, but not limited to, meetings of the RAAC, UCCEDD CAC, DDBP FAC, and advocacy activities performed by the LEND Policy Specialist and Advocacy Faculty. The LEND Data Coordinator (DC) is responsible for collecting and reporting TA data in NIRS for the annual performance report. Examples of the variety of our partnerships are listed below.

National: Examples of boards, taskforces, and workgroups that LEND Faculty and staff serve in leadership positions or participate on at the national level include: AUCD's LPQI Workgroup and Research Subcommittee (Chair) and Legislative Policy Affairs Committee (Member); LEND Self-Advocacy Discipline Network (Chair); LEND Impact Measurement Workgroup (Member); Accreditation Council for Genetic Counseling's Accreditation Committee (site visitor); Agency for

Healthcare Research and Quality's Technical Expert Panel (Panelist); American Academy of Cerebral Palsy and Developmental Medicine's Scientific Committee (Member); American Physical Therapy Association (APTA)'s Pediatrics (Chair); Disabled Parenting Project (Co-Researcher and Parent Advisor); National Down Syndrome Congress Policy Committee (Member); National Research Center for Parents with Disabilities (Co-Investigator and Consultant); National Society of Genetic Counselors: Advocacy Coordinating Committee (State of KY Lead); Patient-Centered Outcomes Research Institute (PCORI)'s Board of Governors; Pediatric Cortical Visual Impairment Association (Board Member); United Cerebral Palsy's Research Committee (Member).

State: Examples of boards, taskforces, and workgroups that LEND Faculty and staff serve in leadership positions or participate on at the state level include: Ohio Advocacy Protective Services Community Leadership Council (Member); ODH's Disability Advisory Committee (Member), Newborn Hearing Screening Advisory Subcommittee (Member); Ohio Health Policy Institute (Appointed Member); Opportunities for Ohioans with Disabilities Council (Member); Supported Decision-Making Network of Ohio Board (Leadership Committee member); Dolly Parton Imagination Library Ohio Board (Member); Family Resource Network of Ohio (Member); Nisonger UCEDD Community Advisory Committee (Member).

Local: Examples of boards, taskforces, and workgroups that LEND Faculty and staff serve on at the local level include: DDBP FAC; Living Arrangements for the Developmentally Disabled Board of Directors; YWCA of Greater Cincinnati's Domestic Violence Shelter Advisory Board; Hamilton County Family and Children First Council; CCHMC Ethics Committee; Timothy Freeman Center for Developmental Disabilities Advisory Board; UC Disability and Society Certificate Steering Committee; Greater Cincinnati Access and Inclusion Network; Children's Theatre of Cincinnati Sensory Friendly Programming Committee; Cincinnati Art Museum's Accessibility Advisory Board; Nuxhall Foundation Board of Directors; CCHMC Patient Family Experience Workgroup; Cincinnati Accessibility Board of Advisors; Hamilton County Accessibility Advisory Board, RAAC Executive Committee.

HIGH-LEVEL WORK PLAN

Activity (Staff)	J	A	S	O	N	D	J	F	M	A	M	J
Trainee recruitment (All)												
Clinics (Faculty, Supervisors)												
Leadership Seminar (Smith)												
ITT (Getz, LeCras, Mays)												
Core Course (Weber)												
SEBM (Weber)												
TA (All)												
CE (All)												
Reporting (Smith)												

Refer to detailed Work Plan in **Attachment 1**.

RESOLVING CHALLENGES

Based on our lessons learned from previous training years, there may be challenges we encounter in designing and carrying out activities in the work plan. The following section describes these potential challenges and our approaches to resolving them should they occur. To sustain the strengths of our program, adequate funding for faculty time is essential. The changing state and federal funding landscape presents ongoing concerns about the sustainability of high-quality programming if funds continue to be reduced. There are competing demands within the academic medical center and within each discipline that faculty must prioritize along with LEND activities. While these demands offer potential challenges,

they also showcase the skill set necessary to excel in one's discipline in a fast-paced medical center environment. This provides opportunities for trainees to learn early in their careers about the need for effective work/life fulfillment.

Our experience has informed several important opportunities to strengthen how we support the success and retention of trainees with disabilities, following trainees deciding to withdraw from the program due to personal circumstances. While program leadership anticipated some of the needs going into the training year, some of the trainees with disabilities have demonstrated more significant mental health challenges and executive functioning deficits than anticipated. Despite substantial support provided by our PD, AD/TD, Advocacy Faculty member, and peers, trainees were unable to manage program responsibilities with personal demands outside of the program. We recognized the need to improve our interviewing processes for trainees with disabilities so that we can be prepared to provide support or offer alternative training opportunities if an individual may not be ready to complete the full LEND training. We also learned ways to simplify some of the processes required to enter the program, including the medical screening, badging requirements, and other paperwork required by CCHMC. We have learned the importance of identifying needs and building effective accommodation plans that are responsive to trainee needs but also ensure full inclusion in the cohort model. We are actively working on these improvements for the upcoming funding cycle by seeking guidance from AUCD's Persons with Disabilities (PWD) Workgroup and its LEND Self Advocacy Discipline Committee, led by our Advocacy faculty, Dr. Ayers. We also recognize the need for intentional community-building and peer support for trainees with lived experience of disability. Cincinnati LEND will participate in a new regional peer mentoring program for self-advocate trainees in the GLLC. Using an adapted constellation mentoring approach (Rottier et al., 2025), self-advocacy faculty will support peer facilitation of monthly, virtual mentoring meetings for trainees across six LEND programs in Year 1, with opportunities for other programs in the GLLC to join in future years. Across the award period, staff from these programs will also collect pilot data to evaluate the regional peer mentoring component, disseminate findings, and support development for future programs, including self-advocate peer mentoring in other regions, regional alumni network activities, and community building for trainees in other disciplines.

Over the past several years, we have recognized the mental health needs of all our trainees, especially regarding elevated levels of anxiety. We have been addressing these concerns by emphasizing self-care within our curriculum and within our overall LEND culture. We have also added a Mental Health session into LEND Orientation where trainees are provided with resources and how to seek mental health services if interested. In addition, we have emphasized a culture of a growth mindset that is receptive to feedback due to trainees coming into our program with high levels of perfectionism and fear of failure. We have also noted the residual effects of the COVID-19 pandemic on our trainees' degree of clinical exposure and development of professional behaviors. Many of our trainees missed a critical period of undergraduate/graduate training and were limited to virtual formats instead of in-person clinical training prior to entering our program, which has resulted in the need for increased faculty involvement to ensure professional interactions with patients and families. Although some disciplines have been impacted more than others, all have been affected. This observation has led to fruitful discussions amongst our faculty and supervisors on how to adjust expectations and meet trainees where they are in their development as a clinician by providing extra support and scaffolding. We also maintain frequent communication with university programs to ensure continuous feedback regarding trainee performance.

PERFORMANCE REPORTING AND EVALUATION

Performance measurement and reporting

The following section describes how Cincinnati LEND **will track project-related processes, activities, and milestones and use data to identify actual or potential challenges to implementation**. Cincinnati LEND uses a multifaceted approach to measure the program's effectiveness and evaluate the proposed goals and objectives. Tracking outcomes requires active central oversight to ensure that data are accurate and will generate meaningful reports to determine if outcome targets are being met. Our evaluation strategy includes regular review of quantitative and qualitative data by LEND Leadership, with data management performed by the LEND DC. **Data will be stored securely** using password protected accounts in the NIRS database and REDCap. REDCap was developed specifically around HIPAA-Security guidelines. All servers and web-based information transmission to and from REDCap are encrypted. Faculty review trainee successes and challenges during monthly IDTC meetings. Core Faculty complete an Interim Progress Report at mid-year after collecting input from faculty and clinical supervisors on trainee's academic performance in LEND, clinical performance, and degree of overall professionalism. This form is reviewed with each trainee during their mid-year ILP meeting with Core Faculty. IDTC meetings are also used to review the program on an ongoing basis. Responsive revision of the curriculum takes place as needed based on this regular feedback, but no less than annually. The leadership team performs a formal annual review of all the evaluation data collected related to both the ILC and the specialized clinical experiences for LTTs. New data from the field and updates in national guidance concerning MCH leadership training also inform regular updates to the curriculum.

Cincinnati LEND will collect data on the **performance measures specified by HRSA**. The PD will use the Discretionary Grant Information System (DGIS) to submit required reporting through the Electronic Handbooks (EHB) by the **specified deadlines**. This will include the New Competing Performance Report at the start date of the project to update administrative data and complete projected yearly targets for Performance Measures. Each year's performance on the Performance Measures will be compared to the year's targets using data collected throughout the year using NIRS and internal data collection systems to complete the annual Non-Competing Performance Report as instructed. Data will be collected and analyzed by the LEND DC and then reviewed by PD, AD/TD, and Faculty before submitting by the specified deadline. **The PD and AD/TD will participate in Autism CARES Act evaluation activities** to inform the federal report to Congress, including the number of diagnostic evaluations conducted annually by Cincinnati LEND. The PD and AD/TD will submit a Project Period End Performance Report after the project ends. We will report on progress on any supplements that our program receives in a similar manner.

We will measure performance on key activities and program objectives as well as collect and report annually on measures that align with program goals and objectives including, but not limited, to the following:

Number of current LTT, MTT, STT and former trainees via follow-up surveys to limit loss to follow-up. NIRS database will be used to report required data on the number of LEND trainees (long, medium, and short-term) participating in the program each year. An essential element of program evaluation is a system to maintain contact with past trainees to gather longitudinal report-back about the post-LEND experiences of graduates of our LEND program. The online trainee survey system through NIRS has improved our ability to remain in contact with LTTs over the years and to have them participate in the Trainee Follow-up Survey which provides longitudinal follow-up data about graduates' employment, research, leadership, advocacy efforts, and scholarly activities. We aim for an 85% survey response rate. Under the supervision of the PD and AD/TD, the LEND DC follows a regular schedule of contact with trainees at intervals determined by MCHB to complete the trainee follow-up survey online.

We also strive to have yearly communication with all graduates to update their contact information. We continue to pursue our intensive effort to maintain contact with our graduates, starting in the first few weeks of the LEND program each year by introducing trainees to the importance of future survey participation and having them complete the survey at multiple time points throughout their training year. This affords LEND leadership the opportunity to introduce the importance of the survey and to answer trainees' questions. Efforts have been made to improve the accuracy of contact information for former LTTs for maintenance in the NIRS database. The following steps describe the process we use to obtain updated contact information: 1) A customized email is sent to all primary email addresses available; 2) If the survey is not completed within two weeks after the first email is sent, we send a second email to the same email address; 3) If the survey is not completed within two weeks of sending the second email, we follow this strategy in a stepwise fashion: a) Send email to secondary trainee email and text trainee at phone number available in NIRS or our database; b) If email or phone number is found not to be active/accurate OR the survey is not completed within the time allotted, we complete following strategies: Contact former supervisors to obtain current contact information (if available), conduct internet and social media searches for possible phone number or job listing to call or email; 4) If the survey is not completed within the reporting period, the former trainee is classified as not reachable.

Number of CE events and participants and quality of the CE provided. A standard evaluation form is completed by individual attendees to evaluate each CE activity. The forms are collected at the end of each activity by the faculty/staff responsible for the activity. The data are reported to the LEND DC who compiles the data into summary format and shares with LEND Directors to determine the degree to which training objectives were met. The DC also collects information about the number of CE activities, the focus of each activity, total number of participants for each activity, and type of Continuing Medical Education or CE units provided. These data are used to report the data required in MCH Performance Measures related to CE/training activities. For the Empowering Families Symposium, attendees rate each of the sessions on their overall level of satisfaction with the session, the degree to which their knowledge of topic increased, whether stated goals and objectives were met, as well as suggestions for future training topics. Attendees also evaluate the entire symposium by rating whether their knowledge of accessing resources and services increased, ability to advocate for child/self/patients increased, and whether they would recommend the event to others. For the Community Outreach Classes, attendees rate their level of satisfaction with information provided, the degree to which their knowledge about the topic increased, as well as three key points they learned by attending the session. Attendees are also asked to provide requests for future training topics. For the Next Step Collaborative Conference, participants rate each of the sessions on their overall level of satisfaction with the session, the degree to which their knowledge of topic increased, whether stated goals and objectives were met, as well as suggestions for future training topics. Additional knowledge questions may be required based upon the professional CE requirements.

Number and quality of TA activities and participants in program, including those geared toward autism/DD topics. All faculty/staff who provide TA through participation on committees, task forces, planning groups, etc., report data to the LEND DC including the recipient of the TA, the time involved, the number of individuals involved, and the major product/outcome or anticipated product/outcome. These data are used to assess the quality of TA activities and are reported as required in MCH Performance Measures related to TA including details on subject matter expertise and TA provided to the community.

Number of individuals taught to diagnose, screen, and/or provide evidence-based interventions on autism/DD. NIRS will be used to track these numbers. All LEND trainees will be exposed to these skills through a variety of academic and clinical opportunities. The extent to which they reach clinical competence on these skills will depend on their discipline and level of training.

Number of diagnostic evaluations Cincinnati LEND conducts each year. Cincinnati LEND will work with DDBP leadership and data analysts from CCHMC's James M. Anderson Center for Health Systems Excellence to extract data from the EPIC Electronic Health Record regarding the number of diagnostic evaluations conducted each year.

Details on the impact of LEND activities on the systems of care. See p. 33 for examples on how we measure the impact of Cincinnati LEND activities on the systems of care.

Program evaluation

Measuring program success depends primarily not on what the program and its faculty offer, but rather on what benefits accrue to program participants and to the individuals and communities we serve. Therefore, the evaluation of participant performance and impressions is a crucial component of evaluating the program. The longitudinal nature of this program, its required assignments, and use of collaborative learning methods allows extensive evaluation of participant performance and impressions. We use the Kirkpatrick Model (Alzate, 2026; Kirkpatrick & Kirkpatrick, 2016) to measure the impact of learning within the Cincinnati LEND program. Training content must be practical, interesting, enjoyable, and based on adult learning theory. The four levels of the Kirkpatrick model include **Reactions** (how do trainees and other participants feel about the experience, what do they like and dislike, are they satisfied?), **Learning** (how successfully did participants increase knowledge, improve skills, and experience attitude changes corresponding to program content?), **Behavior** (are skills and knowledge from the program transferred and used in participants' everyday MCH practice and valued years after training is completed?) and **Results** (are LEND graduates making an impact on MCH systems?). See p. 33 for more information regarding impact.

Approaches to assess experiences for STTs, MTTs, and LTTs.

Cincinnati LEND Program survey: LTTs complete this via *Constant Contact* at mid- and end-of-year to provide feedback on all aspects of Cincinnati LEND including all projects, activities, and sessions. LTTs are asked to rate how well Cincinnati LEND is preparing them to assume leadership roles related to the field of autism/DD in the areas of advocacy and policy, research, teaching, and clinical skills. Trainees are also asked to rate their level of satisfaction with the training, how much their knowledge has increased in various areas, which activities were most beneficial to their learning, and to provide feedback on what changes they would make to the program to improve the trainee experience.

Exit Interviews: Trainees provide verbal feedback at the end of their training year as a part of their final ILP meeting with the PD and AD/TD.

Session Evaluation Form: All trainees (STT, MTT, and LTT) evaluate each session of the Leadership Seminar, EBCD, and Core Course as well as select ITT and SEBM didactic sessions by completing a survey through *Canvas* following completion of each session. Items focus on clarity of learning objectives, preparation and delivery of the speaker(s), level of engagement as an audience member, application to own discipline, and usefulness of handouts and/or audio/visual supports. In addition, trainees rate the overall quality of the session and whether the session was conducted in a culturally and linguistically appropriate manner. The evaluations are compiled by the AD/TD and used both to provide feedback to the presenter(s) for quality improvement purposes and to identify whether sessions will continue to be provided the following year.

Disciplinary & SEBM Interim Presentation Evaluation Form (Faculty & Trainees): Trainees complete a self-evaluation after oral presentations to rate their own performance in the areas of speaking skills (rate, tone, volume, use of filler words), preparation and delivery of content, time management, accessibility of slides, and engagement with audience. Trainees also note skillful aspects of the

presentation as well as areas for improvement. Faculty complete the same form to rate the performance of each trainee and written feedback is shared with trainees for comparison.

Reflective Essays Rubric: Trainees write reflective essays at the end of each semester of CORE to identify "take-home" points from sessions most salient to them and describe how they are currently implementing or will implement the knowledge, skill, or resource in their discipline. Essays are graded by the AD/TD with written feedback provided to trainees based upon the Reflection Evaluation for Learners' Enhanced Competencies Tool (REFLECT; Wald et al., 2012). The last session of CORE is used to discuss the trainees' essays as well as obtain feedback from trainees about the quality of the various Core Course sessions.

SEBM Self-Assessment and Course Survey: Trainees complete this via *Canvas* at mid- and end-of-year to rate their level of knowledge or skill on the course objectives (See **Attachment 5: Syllabus**). Trainees also rate their level of satisfaction with the experience and the degree to which their knowledge about research increased.

SEBM Faculty Assessment: SEBM Team Leads complete this form at mid-year and end of the training year on each of their SEBM team members to assess trainee progress towards achieving the course objectives. Team Leads also provide feedback on how each trainee can continue to achieve these objectives at mid-year and in their future careers.

Discipline Specific Competency Evaluations: Clinical supervisors evaluate discipline specific competencies on an ongoing basis through observation of the trainee's assessments, written clinical reports, and discussion during supervision. The specific format for documenting discipline specific competencies varies among the disciplines based on the formats required by the trainee's university department/program. A similar process is used for LEND Core faculty to evaluate non-clinical trainees.

ITT Evaluation Form: A faculty member who co-coordinates the ITT course compiles feedback from faculty and supervisors in this form at the end of each semester to evaluate trainee's performance as a member of their team. Items focus on skills such as collaboration, respect for alternative viewpoints, participation in team discussions, as well as professional behaviors regarding punctuality, timely completion of tasks, and leadership competencies around person-/family-centered, and culturally and linguistically appropriate care. The trainee is also rated on their performance in a leadership role on the team as appropriate. The form is shared and discussed with each trainee at the end of each semester.

Observations across Disciplines Rubric: Trainees complete a written reflection for two observations per semester (four total) in which they observe other disciplines during patient care visits to gain firsthand knowledge of the practice of that discipline and the family experience of seeing that discipline. The faculty member who coordinates this project completes the REFLECT to rate trainee's skill in professional writing, critical thinking, and ability to answer reflection prompts comprehensively. The rubric and feedback are shared with each trainee.

LEND Leadership Self-Evaluation Form: Trainees complete this via *REDCap* at the end of their training year using retrospective pre-post methodology to assess their perceived level of competence and personal importance in the MCH competency areas of Self-Reflection, Ethics, Communication, Negotiation/Conflict Resolution, Community Health Factors, Lived Experience in MCH, and Interdisciplinary/Interprofessional Team Building.

Interdisciplinary Practice Self-Assessment: Trainees complete this via *REDCap* at the end of their training year using retrospective pre-post methodology to assess their attitudes/beliefs about interdisciplinary practice and the frequency of their use of interdisciplinary skills. This form is modeled after the work of Margolis et al. (2013).

Attitudes Towards Healthcare Teams Survey (ATHCT) & Interprofessional Collaborative Competencies Attainment Survey (ICCAS): Trainees complete the ATHCT and ICCAS at regular

timepoints via Qualtrics (collected by Nisonger LEND) as part of the Interprofessional Case Simulation Series experience.

MCH Navigator Self-Assessment: Trainees complete this at the beginning of the program to assess their knowledge base and skills in the MCH Leadership Competencies. Information from this self-assessment guides the development of their ILPs. Trainees create specific action plans for incorporating MCH Leadership Competencies into their everyday work and reflect on their successes throughout their training.

Policy Retrospective Assessment: Trainees complete this survey via *REDCap* at the end of their training year using retrospective pre-post methodology to assess their perceived levels of confidence, knowledge, and skill in the MCH leadership competency of Policy. Trainees are also asked to rate their level of satisfaction with the policy and advocacy curriculum and recommendations for improvement.

Leadership Showcase Evaluation Form: Each trainee has a choice between completing a Leadership Journey presentation or a Leadership e-portfolio. The Leadership Journey presentation is a 10-minute presentation towards the end of the LEND training year, following the *True North* model. A formal evaluation of the presentation, using a standard evaluation form, is conducted at the end of the academic year by the PD and AD and shared with the trainee. The Leadership e-portfolio contains work products of LEND projects, SEBM poster and abstract, and additional materials which the trainees believe demonstrate their leadership development. Trainees share their e-portfolios with each other during the last LEND session of the year to encourage long-term connection with one another. A formal evaluation of the portfolio, using a standard evaluation form, is conducted at the end of the academic year by the PD and shared with the trainee and his/her faculty mentor.

Community Leadership Project Service-Learning Reflection Paper Rubric: Trainees keep a time log to document their service-learning hours, write a reflection paper at the end of their experience, and present to the other trainees so that the cohort can learn about the particular agency/organization. Written feedback on both written and oral components is provided to each trainee based upon the REFLECT. Partner feedback is obtained through pre- and post-placement meetings between the Community Engagement Coordinator and each site's leadership team.

Early Intervention Project Reflection Rubric: Similar to the CLP, trainees keep a time log to document their project hours and provide a reflection in the form of a paper, voice-over PowerPoint presentation, or brief video that reflects on family engagement, developmental content and intervention, and family support strategies observed in the home environment. Written feedback is provided to each trainee again using the REFLECT tool.

FMP Reflective essays Rubric: Trainees write reflective essays at the end of each semester to reflect on the experience and what they learned and how their clinical activities/advocacy/research will be impacted by what they learn from their family mentor. The family discipline faculty provides written feedback to each trainee utilizing REFLECT tool framework.

Person-/Family-Centered Care (PFCC) Feedback Form: Family discipline trainees (with mentorship from Family Discipline Faculty) observe clinical activities and use this form, developed by our team and based upon the Institute for Patient- and Family-Centered Care core concepts to provide feedback to clinical trainees on their use of family-centered care skills in these interactions with patients and their families.

Anticipated evaluation barriers and how to address. A Policies and Procedures manual is provided to each trainee and reviewed during Orientation. Expectations regarding completion of various evaluations are discussed during this time. The AD/TD reviews the completion rates of individual session evaluations on a frequent basis. If completion rates are low, reminders are sent through multiple modes

including announcements on *Canvas*, as well as verbal and email reminders. If completion rates are satisfactory, positive reinforcement is provided in the same manner.

Evaluation capacity, including experience, skills, and knowledge. The LEND program has a designated DC, with 50% of her total effort dedicated to the LEND program and the remainder of her effort dedicated to UCCEDD. The current DC has been in this role since 2010. The DC reports administratively to the PD. The DC participates in the NIRS Data Coordinator's Annual Meeting and receives TA from AUCD as necessary. The DC assists LEND Leadership in implementing, monitoring, and evaluating planned activities through data collection. The DC is responsible for communicating with LEND Faculty and staff regarding the fiscal year reporting requirements as defined by HRSA/MCHB and NIRS. The DC sends communication to collect information on current trainees, projects, activities, and products that our faculty and staff have participated in during each fiscal year. The DC is responsible for entering this information into NIRS. She is responsible for building surveys in *REDCap* that are used to collect data from current trainees for program improvement, including pre- and post-training surveys on program components. The DC maintains contact information for all former LTTs and is responsible for sending the Trainee Survey to graduates at 2, 5 and 10 years post-LEND to measure LEND's impact and align with UCCEDD reporting requirements.

Continuous quality improvement

See p. 27 for plans to assess the quality of CE and TA provided by Cincinnati LEND. See p. 28-30 for plans to assess the quality of trainee experiences. See p. 32 for more information regarding how Cincinnati LEND shares project results and outputs with other awardees and the public. Cincinnati LEND monitors progress and conducts quality improvement efforts to inform and improve processes and outcomes to ensure that our program meets the **evolving needs of the workforce serving individuals with autism/DD**. Cincinnati LEND has participated in the LPQI Network since 2017 and will continue to do so in the next grant cycle. LPQI is a data collection and analysis system designed to allow participating LEND training programs to compare their training outcomes with national data. The goal is to help individual training programs improve the quality of LEND training in core MCHB leadership competencies. The LPQI data system is managed by AUCD through an MCHB-funded TA grant for the LEND programs. Participation in the LPQI Network is voluntary and open to all LEND training programs. Key components of LPQI include: (1) individual LEND programs use standardized measurement tools for core competencies such as family-professional partnerships, interdisciplinary/interprofessional team building, and policy; (2) LEND programs enter results of these measurement tools into a national, inter-institutional database; (3) AUCD analyzes the data, calculates national averages, and provides data to each LEND program; (4) individual LEND programs use data comparing their program outcomes to national benchmarks to aid in QI efforts; and (5) aggregate data is available to demonstrate improvements in training program outcomes across the network, providing evidence for return on investment of training grant funds. Dr. Smith serves as the lead on the general LPQI workgroup and LPQI research groups and attends monthly meetings with leaders across the network to improve network function and relevance. Dr. Smith and the Cincinnati LEND Policy Specialist developed the Policy measure that is currently being used across the LEND network. Dr. Smith was first author on "A Quality Improvement Network for Interdisciplinary Training in Developmental Disabilities" paper that highlighted the impact of LEND training in the three competencies described above and was published in Pediatrics in November 2022. A manuscript focused on trainee gains in policy is under peer-review for publication in *Journal of Disability Policy Studies*. For the next grant cycle, Dr. Smith and others from the LPQI workgroup will be working with AUCD on how to improve the LPQI system to capture more specific data and better meet program needs. This will include ensuring

that the measures reflect updates to the MCH Leadership Competencies and are inclusive of non-clinical trainees including family trainees and trainees with disabilities.

DDBP QI efforts have focused on improving access to care for individuals with autism/DD. Utilizing clinical supply differently and building new care models has resulted in improved and sustained wait time of 66 days for any new visit in DDBP, with shorter wait time of 30-60 days for children younger than three years old. These efforts have allowed us to systematically expand access to school-aged children seeking comprehensive evaluation of autism/DDs while maintaining quick access for such evaluation in our youngest patients. The DDBP QI team developed an innovative and responsive priority scheduling process to improve time to new patient evaluation for children with developmental concerns referred from our academic center's primary care clinics. Using QI methodology focused on flexible and creative priority scheduling practices from within the medical home, the percentage of patients referred to DDBP who scheduled and completed their initial visit within 90 days increased from 20% to 40%. This work was co-led by core faculty member, Dr. Williams-Arya (Samaan et al., 2025) with active involvement from LEND DBP and GN trainees. To improve access to culturally and linguistically appropriate services, we modified scheduling algorithms and clinic builds so that Spanish-speaking families are automatically paired with certified bilingual Spanish medical providers when they schedule a new visit. We have seen an increase in the proportion of new visits for children younger than three years completed by Spanish-speaking families.

SUSTAINABILITY

Regarding financial **sustainability**, although the depth and robustness of the program would be diminished, key elements of Cincinnati LEND could be sustained beyond the federal funding period by leveraging core funding through partnerships built by the program over the last several years. Cincinnati LEND serves a **vital** interdisciplinary training function within UCCEDD and DDBP. As a part of a larger academic medical institution (CCHMC), our program could reach a wide range of students and professionals with recorded sessions housed on *Canvas*, observations within interdisciplinary clinics, and experiential community activities. Various components of Cincinnati LEND are already sought out by divisions across the institution and community. If necessary, Cincinnati LEND could begin charging CE fees for CE opportunities and reduce stipends for trainees obtaining course credit. If federal funding was not available, Cincinnati LEND would pursue state-based funding or explore philanthropic opportunities through DDBP and MBHI. Several LEND Faculty and staff are funded through a variety of grant mechanisms which could be leveraged to continue some elements of the program including research projects and trainee leadership development through program development and evaluation.

Cincinnati LEND **shares information about our project** in a variety of ways to help other groups advance autism/DD training in other settings. This includes reports, products, and other project outputs, scholarly activity, and training methods. Faculty and trainees present posters and concurrent sessions and are invited speakers at local, state, national, and international conferences (See Table 2 on p. 16). At the national level, this includes discipline specific annual meetings such as National Society of Genetic Counselors, American Physical Therapy Association's Combined Sections, American Occupational Therapy Association, as well as interdisciplinary conferences such as AUCD, Society for Developmental and Behavioral Pediatrics, Pediatric Academic Societies, American Association on Intellectual and Developmental Disabilities and EHDI. Internationally, faculty and staff from Cincinnati LEND have presented at Association for Behavior Analysis International Conference, International Congress of Applied Psychology, and Keys for Osteogenesis Imperfecta Quality of Life Conference.

In addition to disseminating findings from various research projects in scholarly journals, the faculty publish on elements of the Cincinnati LEND curriculum including our policy curriculum (Weber

et al., 2019), leadership training (Weber et al., 2021), interdisciplinary training for behavior analysts (Vincent et al., 2025) and impact of the LEND network demonstrated by the LPQI (Smith et al., 2022). LEND faculty and staff report products, publications, and curricular materials to the LEND DC who enters the data into NIRS to assess progress towards program objectives and toward MCH Performance Measures. Recognizing the need to disseminate accessible information to a vast audience, Cincinnati LEND has worked diligently to produce products that "tell our story" including two infographics. One highlights the LEND network across the country while the other highlights Cincinnati LEND specifically. These infographics are updated on an annual basis and have been added to recruitment and interview materials, posted on social media, and shared with stakeholders, policy partners, and legislators at the local, state, and national levels. We also share information in more informal ways by attending meetings of the UCCEDD CAC and DDBP FAC.

In partnership with CCHMC's physician-hospital organization, Tri-State Health Services, Inc. and the James M. Anderson Center for Health Systems Excellence, Cincinnati LEND has also developed Community Practice Support Tools using expert consensus and informed by Best Evidence Statements, Care Practice Guidelines, and other evidence-based documents as available. These three unique tools educate providers on what to do and how to refer to appropriate services when they detect developmental concerns in children up to 12 years of age. Similar Primary Care Guidance Tools have been developed and disseminated to help caregivers understand what they can do while they wait for a developmental evaluation at DDBP including how to contact the DDBP Family Support Team.

Public Health Impact

Cincinnati LEND builds workforce capacity but also improves the system of care for children and youth with autism/DD in a variety of other ways including referrals to services, coordination of care, TA, and policy, all resulting in measurable improvements in outcomes at the local, state, regional, and national levels.

Workforce development. The most significant way Cincinnati LEND has made a strong public health impact to improve systems of care for individuals with autism/DD is through workforce development. Since 1969, over 2,250 LTTs from 28 unique disciplines have graduated from Cincinnati LEND.

CCHMC: Like many systems across the country, systems within CCHMC are overburdened. There is simply not enough supply of providers to keep up with the demand of patients/families needing to be seen. Cincinnati LEND serves as a direct pipeline to our division and institution across all clinical LEND disciplines to address this demand. LEND family discipline graduates also hold leadership roles within DDBP including LEND Family Discipline Coordinator, Family Outreach Associate, Family Support Specialists, Family Navigator, Policy Specialist, Community Engagement Coordinators for Ohio P2P and Ohio F2F. At the time of this application, there were over 75 current CCHMC employees who have been trained through Cincinnati LEND since Drs. Smith and Weber became the PD and TD in 2013.

Local: Former Cincinnati LEND trainees hold clinical positions in local clinics and schools as well as leadership positions throughout the greater Cincinnati region including the Regional Director for The Arc of Ohio, the Director of Development and co-founder of the Smith-Kingsmore Syndrome Foundation, founder and governing board member of Autism and We, Director of Clinical and Field Placement in the College of Education, Health, and Society at Miami University, Program Manager for the Freeman Center for Developmental Disabilities, Family Leadership Network Coordinator at Starfire, founder of Jason's Connection, founder of Hearts in the Park, Adolescent and Adult Program Manager of Spina Bifida Coalition of Greater Cincinnati.

National and international: The impact of Cincinnati LEND is far-reaching with former LTTs working in 29 states across the country as well as in Washington, DC, and Thailand. Many Cincinnati LEND graduates work in the top *US News and World Report's* "Best Children's Hospitals" in addition to CCHMC, including Boston Children's Hospital, Children's Hospital of Philadelphia, Texas Children's Hospital, Children's National Medical Center, and Nationwide Children's Hospital as well as top universities including University of Cincinnati, University of Kansas, University of Arkansas.

Referrals to services. Ensuring families/patients are connected to services is an integral part of Cincinnati LEND and is achieved through a variety of activities. All families who are receiving a diagnostic evaluation for their child participate in an ISS at the end of the diagnostic process. The ISSs are conducted by a DBP or a psychologist. During this session, families are provided with next steps for their child which often include referrals to various services including early intervention, allied health services, school supports, or further specialized medical services. Our clinical providers work closely with CCHMC's Hospital-Based Child Find Specialist who is supervised by LEND AD/TD, Dr. Weber. The Child Find Specialist provides education and information to all healthcare providers about eligibility requirements and serves as the referral source and liaison between CCHMC and the community. The Child Find Specialist then refers children at risk of, or with suspected developmental delays or disabilities, to Early Intervention. Our clinical providers also work closely with the DDBP Social Work team who are vital in getting families connected to services. The LEND Social Work trainee and fellow are supervised by members of the DDBP Social Work team and are actively involved in the referral process. Several years ago, the LEND program recognized that families who were participating in a diagnostic evaluation did not feel like they were provided with necessary next steps for their child. As a result of this need, the LEND program developed a Family Navigation program. The first Family Navigator hired was a former LEND family trainee who has served as our Family Discipline Coordinator for the past four years. The FN program provides care coordination and connection to educational and community services. Feedback from families has been extremely positive as evidenced by completion of multiple action plans to reach family goals. LTTs, especially Family Trainees, meet with and observe the work of the Navigator during LEND. ITT serves as a unique opportunity for our LEND faculty/trainees to connect families with services including a comprehensive diagnostic evaluation. Although CCHMC's catchment area includes counties in Northern Kentucky, patients with Kentucky Medicaid are currently not eligible for psychological services at CCHMC. The LEND program offers these patients and families the opportunity to receive a comprehensive evaluation that they likely would not otherwise be able to pursue. By participating in ITT, LEND trainees can make referrals within their own discipline but are also able to learn about referrals made by all other disciplines participating on the team.

Coordination of Care. The Cincinnati LEND program is committed to improving systems of care throughout our institution and the greater Cincinnati community. One method is through our involvement with the newly established MBHI at CCHMC. MBHI strengthens the connections between the divisions of Child and Adolescent Psychiatry, Behavioral Medicine and Clinical Psychology, and DDBP, and enables deeper collaboration across all related specialties. This integration helps ensure children and families receive the most timely and effective treatment possible. Our Associate LEND Director, Dr. Weber, is the Community Liaison lead for MBHI, while Marta Getz, our LEND SW faculty, was named the Clinical Manager for Family Navigation for MBHI. Two of the existing programs within DDBP, including FN and FAC, are serving as models for the development of similar programs within MBHI under the leadership of Dr. Weber and Ms. Getz. A recent Cincinnati LEND graduate is the Preschool Director and Part B 619 Coordinator for Adams County, a rural county outside of Cincinnati. She has created a mechanism to share Act Early information with families of children who are referred out of Part C who may or may not qualify for Part B services. She has also taken information learned in

LEND and added them to her professional development activities for her staff including understanding the process for autism diagnoses and provision of ABA services. Our LEND team has also been working with CareSource, a managed care organization that offers Medicaid coverage for children in Ohio. We have been providing consultation to CareSource care managers related to autism service delivery. We have also been working to provide guidance on the re-approval process for continuation of ABA services, which currently requires full comprehensive diagnostic evaluations that overwhelm an already overburdened system.

TA. Our LEND team has been working actively with leadership from CCHMC's Center for Patient and Family Experience to assist them in how to improve the patient and family experience for families of children with developmental disabilities who receive care at CCHMC. Our teams developed a survey that was completed by families of children with DD who are seen through DDBP, to gain a better understanding of the supports (e.g., Child Life Services, Adaptive Care Plans, social narratives, visual supports, and sensory supports) they receive through our Division so that similar supports can be incorporated across the institution.

Policy. Cincinnati LEND has been actively involved with educating policymakers at the state and national level on issues that impact children and youth with autism/DD and their families. Until January 2025, health insurance plans were not required to cover the cost of hearing aids for children in Ohio who have hearing impairments. Trainees and faculty from the LEND program played a crucial role in educating policymakers on the importance of hearing aids in enhancing quality of life for young patients. Madeline's Law became official when Ohio Governor Mike DeWine passed it into law in early January 2025. For the past few years, Cincinnati LEND has worked actively with CCHMC's Legislative Affairs office on a plan to educate policymakers in Ohio on the importance of Prescribed Pediatric Extended Care (PPEC) programs, community-based programs that address the critical needs of children with complex medical needs who may require medical childcare. These programs have been adopted in multiple states, but not yet in Ohio. LEND trainees/faculty educated policymakers during Ohio's DD Awareness and Advocacy Day in Columbus, OH in early March 2026 concerning the PPEC programs. This bill has cleared the House and is now in the Ohio Senate Health Committee. LEND trainees/faculty also educated policymakers on a bill named Joshua Alert. Named after a 6-year-old boy with autism who tragically drowned after going missing, this bill activates a coordinated statewide response using Ohio's existing alert system when a child with a developmental disability such as autism goes missing. This bill has passed both chambers of the Ohio Legislature and is currently in the final stages of the legislative process.

ORGANIZATIONAL INFORMATION

Cincinnati LEND is supported by a robust institutional infrastructure. UC is an urban, public, state university and is the eligible applicant for the Cincinnati LEND Project. CCHMC is affiliated with UC by serving as the Department of Pediatrics for the COM. CCHMC is a leading medical research and teaching hospital consistently ranking as one of "US News Best Children's Hospitals" by *U.S. News and World Report*. It is also the third-highest ranking recipient of research grants from the National Institutes of Health among pediatric institutions. CCHMC's mission is to "improve child health and transform delivery of care through fully integrated, globally recognized research, education, and innovation." CCHMC is a nonprofit pediatric health system with a strong focus on population health and access for all children, including those who are medically underserved. CCHMC provides culturally and linguistically appropriate services with infrastructure to engage community partners, conduct extensive outreach, and design programs specifically to reach children facing barriers to care. See **Attachment 3** for letters of agreement from partners within UC and CCHMC. DDBP, a large division within CCHMC where

Cincinnati LEND is housed, is comprised of approximately 130 academic faculty, staff physicians and psychologists, allied health professionals, and support staff representing a variety of disciplines. Cincinnati LEND is and will continue to be one of the major programs housed within DDBP. See **Attachment 4** for Cincinnati LEND Organizational Chart. LEND Leadership has monthly meetings with the DDBP Business Director and Financial Analyst to ensure appropriate recording of all program costs and proper accounting of federal funds based on the approved work plan and sub-award budget. LEND Leadership and the DDBP Business team have frequent communication with the Grant Administrator team in the Office of Operations and Finance at UC to ensure accurate and timely reporting of finances for the prime and sub-awards in HRSA's EHB system.

Access to established training programs

For more information about our access to training programs and our ability to recruit graduate students and other LEND trainees from various disciplines, please refer to p. 4-6.

Capacity to carry out project based on past performance

Cincinnati LEND serves a vital training role within DDBP as evidenced by DDBP's vision statement "to be the leader in improving health, quality of life, and outcomes in children and adolescents with developmental disabilities" and specific foundation of training to build leadership and capacity in the field of developmental disabilities. Interdisciplinary leadership training has been at the foundation of DDBP since its inception in the late 1960s as the Cincinnati Center for Developmental Disorders (CCDD) by Dr. Jack Rubinstein. As a University Affiliated Facility (UAF) funded through the Kennedy Administration, CCDD emphasized interdisciplinary training. As mentioned above, over 2,250 LTTs from 28 unique disciplines have graduated from Cincinnati LEND since its inception. Cincinnati LEND graduates are leaders in the field of autism/DD. According to the most recent trainee survey, 85% of Cincinnati LEND trainees report working with MCH populations in interdisciplinary settings and demonstrating leadership in the areas of academics, clinical services, public health, and/or policy/advocacy five years after completing the program.

Physical and technological resources to conduct training program

In 2010, DDBP moved into a state-of-the-art Medical Office Building (MOB) that provides approximately 120,000 square feet of office, clinic, program, and research staff space, of which DDBP occupies three of the six floors. DDBP's space includes 17 clinic examination rooms and 35 treatment rooms which all have recording and remote viewing and/or direct observation rooms (especially for training purposes). The camera system for remote viewing was updated in 2025. The first floor of the building is devoted to two EIBI classrooms for preschoolers with autism and a demonstration classroom for school-age children with autism, as well as the BIBT program. The sixth floor of MOB houses offices for faculty, staff, and trainees. Trainee space includes two large rooms with cubicle space and individual computers and phones for trainees. Having trainees located in the same areas, rather than dispersed in different locations, provides increased informal interdisciplinary interaction between trainees. The fifth floor houses several small conference rooms dedicated to DDBP and a large conference room (that seats 125) with Zoom capacity that facilitates remote training. LEND sessions typically occur in the large conference room with LTTs attending in-person. Cincinnati LEND is intentional about assessing and meeting the unique needs of all trainees we serve. We have built our program's infrastructure to ensure that the settings, materials, and curricula are accessible for all participants with varying needs. All program materials available on *Canvas* were designed with support from an Assistant Professor at UC who builds dynamic online learning environments and specializes in Universal Design for Learning and technology for inclusion. The accessibility of the program's physical spaces is frequently assessed by the LEND Faculty member who has lived experience as a person who uses a manual wheelchair in addition to feedback from members of the UCCEDD CAC. Feedback about accessibility of written materials is

provided by members of the DDBP FAC and an educational consultant who teaches the LEND session on health literacy. Technology used to facilitate adult learning and support collaborative interdisciplinary teamwork throughout many aspects of Cincinnati LEND will align with cybersecurity requirements to ensure confidentiality, integrity, and compliance with federal security expectations. Overall, day-to-day communication with trainees regarding scheduling, expectations, and assignments is conducted through *Canvas* learning management system. *REDCap* is used for research-related surveys and *Constant Contact* is employed for preferences and general surveys of trainees. Each SEBM team has access to a Google Drive, Microsoft SharePoint, or shared place within *Canvas* to share documents and resources and to discuss progress. Teams learn how to more productively use reference manager software and social bookmarking as tools for sharing results of literature and online resource searches. Zoom is used for remote learners. Interactive components such as chat box, breakout rooms, and annotation are encouraged. Sessions are recorded on Zoom and saved on a private YouTube channel to share broadly with other LTTs, DDBP faculty and staff, and community partners and for use in completion of make-up assignments for missed sessions. Other innovative educational technology is used to engage trainees in sessions, including *Canva*, *Padlet, Mentimeter,* and *Visme*.

Organizational capacity to gather, manage, and use data to inform processes and advance outcomes

See p. 31 Program Evaluation for more information regarding evaluation capacity, including experience, skills, and knowledge.

Project director, LEND faculty, and staff qualifications

Cincinnati LEND's leadership team, LEND faculty, and project staff have the training and extensive experience to carry out the project activities as well as performance reporting and program evaluations. See Biosketches for more information about Dr. Smith, LEND PD and Dr. Weber, LEND AD/TD. Cincinnati Core Faculty represents at least 10 disciplines and are role models for trainees aspiring to leadership roles in clinical work, advocacy and policy, research, and other academic areas such as teaching and training in an interdisciplinary setting. The LEND Family Faculty is essential to our program and emphasizes the importance of person-/family-centered care and family partnerships as an underlying foundation of Cincinnati LEND. The LEND Advocacy Faculty underscores Cincinnati LEND's foundation of "Nothing about us, without us." LEND Faculty work as a cohesive cohort to maintain a curriculum that meets the needs of trainees at various levels of education, clinical training, and lived experience. The initial part of every IDTC meeting is dedicated to faculty's own professional development in various areas including supervision, mental health, and interprofessional education. In addition, LEND faculty are provided with ongoing professional development through CCHMC's Office of Academic Affairs and Career Development as well as UC's Center for Clinical and Translational Science and Training. Cincinnati LEND has met the number of faculty and faculty disciplines in the Tier 4 funding level over all previous grant cycles, reflecting its successful recruitment approach. We have a multitude of relationships within CCHMC and with local universities to which we can recruit new faculty should positions need to be filled in the proposed project period. Position descriptions for key personnel are provided in **Attachment 2: Staffing Plan**. Biosketch Chart in **Attachment 2** describes each LEND faculty member and their qualifications to meet Core Faculty requirements including experience and expertise in their respective disciplines, experience in autism/DD, and qualifications to mentor graduate and postgraduate students and serve as leaders in the field of autism/DD.

Acronym Chart

AD/TD: Associate Director/Training Director	IDTC: Interdisciplinary Training Committee
CCHMC: Cincinnati Children's Hospital Medical Center	ILP: Individualized Leadership Plan

CLP: Community Leadership Project	ISS: Information Sharing Session
DC: Data Coordinator	ITT: Interdisciplinary Training Team
DDBP: Division of Developmental and Behavioral Pediatrics	LEAD: Interdisciplinary Leadership Seminar
EBCD: Evidence-Based Case Discussions	MBHI: Mental and Behavioral Health Institute
EIP: Early Intervention Project	PD: Project Director
FMP: Family Mentoring Project	SEBM: Seminar in Evidence-based Methods
FN: Family Navigation	UC: University of Cincinnati
UCCEDD: University of Cincinnati Center for Excellence in Developmental Disabilities	

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Attachment 1: Work Plan

Objective 1. Train graduate & post-graduate students, family members, individuals with disabilities, & practicing professionals			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To increase capacity of the workforce trained to meet the complex needs of children & youth with autism/DD	LEND Leadership & Core Faculty will market the program using Curriculum Graphic, website, presentations, & videos	Y1 - Y5	Smith, Weber, LEND Faculty, Hennessy

Methodology	
Specific	We will recruit at least 29 long-term trainees from at least 12 disciplines for matriculation each year that satisfy other legislatively mandated recruitments
Measurable/Evaluation	DC will track recruitment & retention through DGIS for reporting in MCHB Performance Report
Attainable	13 Core Faculty for recruitment & mentorship; 100% retention/graduation rate (2021-2025)
Relevant	More trained professionals needed as Ohio has 249 primary care & 226 mental health care HPSAs
Time Commitment	July 1 recruitment deadline each year of the funding cycle; Recruitment varies by discipline

Objective 1a. Deliver training that includes a balance of academic, clinical, leadership, & community-based learning opportunities framed in an interdisciplinary & person/family-centered manner			
Academic: *Core Course (CORE)*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To provide education on a wide array of autism/DD topics	See Attachment 5 Syllabus	Y1 - Y5	Weber

Methodology	
Specific	All LTT participate in 30, 2h didactic sessions using adult learning methodology; Open to MTT, STT & practicing professionals for CEUs
Measurable/Evaluation	Summative Assessment: Reflective essays on at least 3 sessions at end of each semester; Session reactions are discussed in large group at final semester sessions with all LTT, MTT & STT, to provide informal evaluation of trainees' knowledge & for quality improvement to guide future recruitment/retention of speakers; Formative Assessment: Session Evaluation Form for each session for presenter ratings & key trainee learning points
Attainable	LEND Faculty & Supplemental Faculty & local, state, & national experts are recruited to present didactic material & lead interactive sessions
Relevant	Topics reflect current, necessary information for MCH Leaders & emerging focus areas that may vary annually
Time Commitment	Trainees: 2h per week; Faculty/Speakers: 2-4h per session for preparation & delivery of material; Recruitment of speakers occurs at least 6 months prior to their scheduled presentation in LEND

Academic: *Seminar in Evidence-Based Methods (SEBM)*			
Purpose	**Content**	**Timeline**	**Responsible Staff**

To develop effective leaders with high quality critical thinking skills & science-based judgment who will know how to use evidence-based practices & data for continuous quality improvement in clinical & community-based practice	See Academic p. 15	Y1 - Y5	Weber

Methodology	
Specific	All LTT will participate in the SEBM course & introductory didactic sessions; They complete a research-based project as a member of an interdisciplinary team; All teams led by LEND faculty; Open to MTT
Measurable/Evaluation	SEBM Self-assessment; Course survey; Faculty Assessment; Presentation Evaluation Form (Faculty & Self)
Attainable	At least 5 interdisciplinary SEBM projects annually led by LEND Faculty/Cincinnati Children's/UC Faculty
Relevant	Research topics span developmental perspectives & populations (e.g., early intervention, Down syndrome)
Time Commitment	Trainees: 2-4h per week Aug-April; Faculty: 2-4h per week Aug-April & 10-20h preparing project in summer

Academic: *Evidence-Based Case Discussion (EBCD)*

Purpose	Content	Timeline	Responsible Staff
To provide case-based learning opportunities on autism/DD across the lifespan	See Academic p. 17	Y1 - Y5	Schworer & LeCras

Methodology	
Specific	All LTT participate in 7 EBCD; Series open to MTT, STT, & practicing professionals for CME/CEUs
Measurable/Evaluation	Feedback on quality of responses/synthesis of material; Session Evaluation Form
Attainable	All expert interdisciplinary teams are clinicians who work together to treat children with the conditions discussed; Family members & advocates are invited & offered financial compensation for their time
Relevant	Common developmental concerns seen in clinical practice & educational settings are covered
Time Commitment	Trainees: 2h in class; Expert interdisciplinary team members: 2h prep; 2h in class; Family/advocate panelists: 1h in class

Clinical: *Interdisciplinary Training Team*

Purpose	Content	Timeline	Responsible Staff
To provide a comprehensive, multidisciplinary clinical service model that is person-/family-centered & culturally & linguistically appropriate	Evaluation materials, interviews & instruments used by all disciplines; overview of interdisciplinary roles & responsibilities	Y1 - Y5	Getz, LeCras, Mays

Methodology	
Specific	All LTT participate in this course. Trainees are divided into 2 ITT teams. Each semester, one family is recruited to see all clinical members & a family trainee from Team #1. A second family is recruited to see all clinical members & a family trainee from Team #2 (4 cases total for the academic year).
Measurable/Evaluation	ITT Evaluation Form
Attainable	While each family participates in 9 or 10 evaluations across 4 or 5 mornings, families receive all services free of charge (we also do not bill insurance). Families recruited from DDBP referral lists from which we receive over 700 unique referrals per month; therefore, obtaining 4 families is highly attainable.

Relevant	Fall semester cases: school-aged children with educational, behavioral, or genetic concerns; Spring semester: children under 3 years (rule-out autism)
Time Commitment	Trainees: 2h per week for meetings & 2-4h per evaluation per semester; Faculty: 2h per week for meetings & 2-4h of supervision; Coordinators: 5-10h per family for recruitment & communication

Clinical/Leadership/Person-/Family-Centered: *Interprofessional Case Simulation Series*

Purpose	Content	Timeline	Responsible Staff
To provide multidisciplinary case-based learning opportunities across LEND programs	See Academic p. 10	Y1-Y5	Weber

Methodology

Specific	All LTT participate in 4 case sessions in the series, two in Fall semester & two in Spring semester
Measurable/Evaluation	Attitudes Towards Healthcare Teams Survey & Interprofessional Collaborative Competencies Attainment Survey
Attainable	At least two options for each case are offered so trainees can select which one works in their schedule
Relevant	First case: early intervention-related, second: school-aged child, third: medical complexity, & fourth: young adult case relevant for moving to adult care & employment
Time Commitment	Trainees: up to 2h per case (8 total); Faculty: 2-3h preparation & review of surveys & up to 2h per case

Clinical: *Observations Across Disciplines Project*

Purpose	Content	Timeline	Responsible Staff
To provide opportunities for trainees to observe diagnostic services & evidence-based interventions	Clinical Observations of at least 4 of 10 disciplines	Y1 - Y5	Atzinger

Methodology

Specific	All LTT participate in this project which includes 4 observations of clinical disciplines (other than their own)
Measurable/Evaluation	Observations Across Disciplines Rubric
Attainable	Two observations per semester allow LEND Faculty to evaluate growth of knowledge & communication skills over time
Relevant	Trainees may select clinics/disciplines that are most relevant to their future practice, research, or advocacy
Time Commitment	Trainees: 10-20 hours/trainee for entire project which includes 4 clinical observations (usually 2-4 hours each) plus reflection writing time; Faculty: 35-40 hours/year to read submissions, provide feedback, & coordinate

Leadership: *Leadership Seminars/Didactics*

Purpose	Content	Timeline	Responsible Staff
To prepare LEND graduates to take on future leadership roles	See Academic p. 8	Y1 - Y5	Smith

Methodology

Specific	All LTT participate in this course; Open to MTT, STT, & practicing professionals for CME/CEUs
Measurable/Evaluation	LEND Leadership Self-Evaluation Form & Interdisciplinary Practice Self-Assessment
Attainable	All LTT are provided with all books & materials well in advance of sessions

Relevant	Topics included are important & relevant for leaders in MCH professions
Time Commitment	Trainees: Approximately 2h per week for trainees for attendance at in-class sessions & to complete leadership exercises/activities; Faculty: 2-4h per session for preparation & 2h in class

Leadership: *Individualized Leadership Plans (ILPs)/Mentoring*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To provide individualized mentorship opportunities	MCH Leadership Competencies	Y1 - Y5	Smith

Methodology	
Specific	All LTT develop an ILP with goals in Lived Experience in MCH, Policy, & Community Health Factors plus 2 other MCH Competencies of their choice (5 total competencies plus self-care goals)
Measurable/Evaluation	Complete MCH Navigator Self-Assessment; ILP submitted in October & April
Attainable	In-class sessions are available for peer review of goals; mentorship meetings built into LEND Faculty FTE
Relevant	This process encourages trainees to utilize MCH Leadership Competencies for professional goal development even after completion of LEND training
Time Commitment	Trainees: 5-10h for MCH Navigator completion & ILP development; Trainees & Faculty: 30 minutes per trainee for each mentoring session (at least 1.5 hours/year/trainee)

Leadership: *Policy & Advocacy Training*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To increase knowledge & skills related to policy-making processes	See Academic p. 13	Y1 - Y5	Ayers, Weber, Gerhardt, Harris

Methodology	
Specific	All LTT participate in four, 2h sessions & in Ohio's DD Awareness & Advocacy Day; interested trainees apply to attend Disability Policy Seminar; Open to MTT, STT, & practicing professionals for CME/CEUs
Measurable/Evaluation	LPQI Policy measure & Policy & Advocacy Retrospective Assessment
Attainable	LEND Faculty have published in this area: Weber, Smith, Ayers & Gerhardt (2019)
Relevant	Curriculum provides flexibility to teach trainees disability policy & basics of educating v. lobbying
Time Commitment	Trainees: 8-10h total for in-class sessions & experiential activities; Faculty: 2-4h preparation & 2h in class per session; All: full day for Ohio Advocacy Day

Leadership: *Disciplinary Presentations*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To provide trainees a teaching opportunity; build interdisciplinary respect; offer faculty mentoring that promotes reflective practices & facilitates personal & professional growth	See Academic p. 9	Y1 - Y5	Weber

Methodology	
Specific	All LTT present 10-minute oral presentation on discipline-specific information; Open to MTT & STT

Measurable/Evaluation	Presentation Evaluation Form; Self-evaluation Form
Attainable	Faculty support each discipline to create an effective & concise presentation
Relevant	Trainees focus on their discipline's role in the care for children/adolescents with autism/DD
Time Commitment	Trainees: 1-2h preparation; Faculty: 1h for presentation review; All: 4h for presentations

Leadership: *Leadership Showcase (Leadership Journey or e-Portfolio)*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To prepare LEND graduates for future leadership roles & offer faculty mentoring that promotes reflective practices & facilitates personal & professional growth	Summary of LEND projects & leadership experiences, CV/resume, personal statement	Y1 - Y5	Smith

Methodology	
Specific	All LTT choose either to prepare an e-portfolio or a leadership journey presentation
Measurable/Evaluation	Completion & sharing with LEND Faculty & other trainees through showcase
Attainable	There are many free e-portfolio websites & trainees have many projects & accomplishments to highlight
Relevant	Supports trainees' professional development & employment search
Time Commitment	Trainees: 2-6h preparation; Faculty: 5h for review; All: 6-8h for showcase

Community-based: *Community Leadership Project (CLP)*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To provide experiential training in community-based settings	Service-learning goals vary by setting	Y1 - Y5	Bonds

Methodology	
Specific	LTT have the choice to participate in this project; They select a community agency serving individuals with autism/DD & complete a minimum of 15 hours of service-learning
Measurable/Evaluation	Reflective assignment rubric
Attainable	Our program has relationships with >10 community-based organizations for placement of trainees
Relevant	Trainees work on program development & evaluation & quality improvement to support improvement of & access to services in the local community
Time Commitment	Trainees: 15h of service learning & 2-4h on reflective essays; Coordinator: 50h per year communicating with community agencies & grading reflective assignments

Community-based: *Early Intervention Project (EIP)*			
Purpose	**Content**	**Timeline**	**Responsible Staff**
To offer field placements in state/local agencies	EI home visit or evaluation, Transdisciplinary team meeting, an Individualized Family Service Plan	Y1 - Y5	Bonds

Methodology

Specific	LTT have the choice to participate in this project to observe various EI activities
Measurable/Evaluation	Reflective assignment rubric
Attainable	We maintain partnerships with EI administrators at the 4 nearest Ohio counties
Relevant	Experiential activities in community-based programs & public health services available to MCH populations
Time Commitment	Trainees: 8-10h observing meetings/visits & 2-4h completing assignment/year; Coordinator: 50h per year for coordination & grading reflective assignments (plus CLP)

Person/Family-Centered: *Family Mentoring Project*

Purpose	Content	Timeline	Responsible Staff
To have trainees learn from family members of children with autism/DD to learn how to practice person-/family-centered care	See Curriculum p. 11-12	Y1 - Y5	Trenkamp

Methodology

Specific	All LTT are paired up & matched with a child/adolescent with a disability & their family for mentoring
Measurable/Evaluation	Reflective essays rubric
Attainable	Families are recruited & compensated for their time & efforts; Recruitment through DDBP's Family Advisory Council, a group of at least 16 family members annually
Relevant	Opportunities for trainees to learn family priorities to improve their family-centered care
Time Commitment	Trainees & families: 4 visits of 1-2h each (4-8h per year); Faculty coordinator: 60h of prep & communication with families

Person/Family-Centered: *Family-Centered Care (FCC) Observations Assignment*

Purpose	Content	Timeline	Responsible Staff
To provide clinical trainees direct feedback on their P/FCC skills	Lived Experience in MCH applied to disciplinary clinical work	Y1 - Y5	Trenkamp

Methodology

Specific	LTT in clinical disciplines are observed by Family Faculty & at least one Family Trainee for one clinical encounter; All Family Trainees observe 3-5 clinical trainees & provide FCC feedback
Measurable/Evaluation	P/FCC Feedback Form
Attainable	Clinical trainees have at least 200 clinical hours & Family Trainees can observe during morning LEND hours
Relevant	Observations focused on clinical trainees' skills in shared decision making, assisting with prioritization of needs, tailoring recommendations, & connecting family to resources
Time Commitment	Clinical trainees: 30-60-minute meeting to review feedback; Family trainees: 2-4h observing & completing form per trainee & 30-60 minutes to review feedback/clinical trainees (3-5 clinical trainees each)

Objective 1b. Increase the number of current & future providers trained to screen, evaluate, diagnose or rule out, & develop & provide evidence-based interventions to the target population across the lifespan.

Clinical: *Disciplinary Clinics*

Purpose	Content	Timeline	Responsible Staff
To provide a wide variety of clinical opportunities for trainees to learn best practices for autism screening & diagnosing autism/DD, evidence-based interventions at various levels of support needs across the lifespan	Instruction on clinical screening, evaluation, diagnosis, & treatment for individuals with autism/DD	Y1 - Y5	Smith, Weber, LEND Faculty, Clinical Supervisors

Methodology	
Specific	All clinical trainees will participate in disciplinary work under the supervision of licensed professionals
Measurable/Evaluation	LPQI for Lived Experience in MCH & Interdisciplinary/Interprofessional Team Building; Discipline-specific competency evaluations; LTT track own clinical hours
Attainable	Approximately 15 clinics across main hospital, satellite sites, & The Freeman Center
Relevant	Variety facilitates trainees' clinical competency to serve the target population across the lifespan with a broad array of autism/DD presentations
Time Commitment	Trainees will participate in clinical activities between 8-16h per week (varies based upon discipline) either on a 12-month basis (Fellows & psychology trainees) or a 9-month basis (master's-level)

Objective 1c. Engage trainees at varying levels of intensity through long-, medium-, & short-term training opportunities

Purpose	Content	Timeline	Responsible Staff
To train professionals, family members & advocates to increase knowledge & skills around autism/DD	Same offerings as for LTTs	Y1 - Y5	Smith, Weber & LEND Faculty

Methodology	
Specific	Menu of options ("LEND Lite") disseminated through various marketing channels & professional partnerships/events to recruit at least 5 AMTT, 15 MTT & 70 STT in addition to annual LTT
Measurable/Evaluation	Recruitment- Tracking annually the # of professional community events attended, disciplinary presentations given, reach of social media through analytics, & number of LEND "menu" disseminated; Participation- Tracking annually the # of LTT, AMTT, MTT & STT, their hours, disciplines & in which portions of LEND they participated in NIRS; Educational outcomes for all trainees- see Program Evaluation
Attainable	Social media & email blasts; information sessions with discipline groups at CCHMC & through community agencies
Relevant	Trainees who will later provide care for individuals with autism/DD have opportunities to gain education & exposure through participation in LEND in conjunction with residency program, internship, fellowship, etc.
Time Commitment	MTT & STT: hours vary; Faculty: hours vary by discipline for recruitment & training purposes

Objective 1d. Engage cohorts of trainees that satisfy other legislatively mandated requirements

Purpose	Content	Timeline	Responsible Staff
To recruit & train professionals, family members & advocates that are culturally competent & ethnically diverse	See Academic p. 9	Y1-Y5	Smith, Weber

Methodology	
Specific	All LTT participate in four, 2h sessions; Open to MTT, STT, & practicing professionals for CME/CEUs

Measurable/Evaluation	Complete MCH Navigator Self-Assessment; ILP submitted in October & April
Attainable	DDBP Building BRIDGES program led by LEND grad who co-develops session programming with LEND Leadership
Relevant	Self, others, & wider community-focused across 4 sessions; flexible to accommodate new research & recommendations
Time Commitment	Trainees: 8h; Faculty: 12-20h preparation & refinement of sessions per year

Objective 2. Increase the number of interdisciplinary diagnostic evaluations provided by LEND Faculty & trainees

Purpose	Content	Timeline	Responsible Staff
To build workforce capacity to screen, evaluate, diagnose or rule out autism/DD	Clinical placements- vary by discipline roles & responsibilities	Y1 - Y5	Smith & Weber

Methodology

Specific	LTT, MTT & STT will participate in clinical observations of & participation in interdisciplinary clinics; Increase number of diagnostic evaluations complete by LEND Faculty & trainees by 2% from previous year
Measurable/Evaluation	Tracking # of diagnostic evaluations completed by LEND Faculty & trainees/year in NIRS
Attainable	DDBP receives >700 referrals/month; >10 interdisciplinary clinics in which faculty & trainees participate
Relevant	Expand competent workforce to quickly & efficiently diagnose individuals with autism/DD to improve health & quality of life by linking with necessary interventions & resources
Time Commitment	Trainees: at least 4h per week observations &/or participation in clinics; Faculty: varies by discipline

Objective 3. Include individuals with lived experience, including family members & individuals with autism & other developmental disabilities, as faculty & trainees

Purpose	Content	Timeline	Responsible Staff
To ensure training is guided by lived experience; to build a workforce that is responsive to the real needs of their communities	See Narrative	Y1-Y5	Smith, Weber, Ayers, Trenkamp, Music, Hennessy

Methodology

Specific	Recruit least one Advocacy trainee/year & at least two Family trainees/year; Retain current Advocacy & Family LEND Faculty positions
Measurable/Evaluation	DC will track recruitment for reporting in MCHB Performance Report
Attainable	Inclusion of family trainees since 2007; Advocacy trainees since 2013; Current Faculty in both disciplines; Weber co-leads DDBP Family Advisory Council; Ayers leads UCCEDD CAC
Relevant	Essential to shape training, service delivery, & systems-level improvements
Time Commitment	July 1 recruitment deadline each year of the funding cycle; Recruitment varies by discipline

Objective 4. Provide continuing education (CE) opportunities for practicing professionals

Purpose	Content	Timeline	Responsible Staff
To share new information & enhance skills of practicing professionals dedicated to caring for children & youth with autism/DD	See CE p. 22	Y1 - Y5	Smith, Weber, Music & Hennessy

Methodology	
Specific	We will host at least 3 CE opportunities/year reaching at least 1,000 practicing professionals plus at least 500 caregivers
Measurable/Evaluation	Individual session & overall conference evaluations; Tracking # of professionals in NIRS
Attainable	Long-standing collaborative relationship with Interprofessional Continuing Education (IPCE), DDBP caregiver education program, & Empowering Families Symposium
Relevant	Building community capacity is necessary to augment training of specialized professionals
Time Commitment	Varies on CE offering

Objective 5. Provide technical assistance (local, state, & national) to programs & agencies to improve system of care for people with disabilities

Purpose	Content	Timeline	Responsible Staff
To strengthen MCH systems by providing TA for clinical services, program development & evaluation, needs assessments, policy & guidelines formulation	Varies by committee, task force, board or advisory group	Y1 - Y5	Smith, Weber & LEND Faculty

Methodology	
Specific	Across the local, state, & national levels, LEND & related faculty will participate in at least 50 committees, task forces, boards of directors, & advisory groups annually
Measurable/Evaluation	Tracking of each faculty members' activities per quarter in NIRS
Attainable	LEND Faculty are well established experts in national organizations within their own disciplines as well as sought out for TA for disability-specific at national, state, & local levels
Relevant	We have current, evidence-based research & information to disseminate to our partners at all levels & expertise to share on boards, committees, & task forces
Time Commitment	Faculty: varies by committee, task force, board or advisory group

Attachment 2: Staffing Plan and Job Descriptions

POSITION DESCRIPTION: LEND Project Director (PD)

Position Definition:

Under the administrative direction of the DDBP Division Director at CCHMC and in collaboration UC, the Project Director (PD) will have day-to-day direct responsibility for grant activities with the assistance of the LEND Associate/Training Director (A/TD) and LEND Faculty. The PD will be a steward for all funds under the LEND program, including optional supplements, and will ensure that grant funds are used properly. The PD will attend LEND Director and related meetings. In addition, the PD will work directly with the federal Project Officer. The PD will hold an academic rank of at least Assistant Professor of Pediatrics and will be academically responsible to the Chairman of the UC Department of Pediatrics. All LEND Project Faculty will be administratively responsible to the LEND PD during their time allocated to the Project.

Administrative Responsibilities:

Recruit & retain LTTs with leadership potential. Develop LEND Project goals and objectives and implement them consistent with local, state, and national MCH needs. Maintain collaborative relationships with UC Departments and Programs, other local colleges and universities, other LEND Programs and UCEDDs, community partners and local, state, and federal policy and program partners. Prepare and submit grant applications and required reports, and represent LEND at appropriate local, state, and national meetings.

Education and Training Responsibilities:

Responsible for interdisciplinary LEND Curriculum, specialized clinical training, community partnerships, CE, and TA. Design and implement an evaluation strategy to measure performance on program objectives, including trainee performance.

Research Responsibilities:

Conduct research projects in collaboration with LEND and other Faculty. Prepare and submit abstracts for meeting presentations and manuscripts for publication.

Minimum Qualifications:

Education: M.D. from accredited school of medicine, Board certified in pediatrics OR health professional with a doctorate degree representing one of the LEND core disciplines.

Experience: Minimum of 5, preferably 7 years of experience in programs serving children with autism/DD and 3 years of experience in administrative capacity. Experience should include working in interdisciplinary setting, university teaching, research, and grant-writing.

Knowledge, Skills, and Abilities:

Leadership in graduate education, program management, and establishing and maintaining innovative partnerships; skills for planning, organizing, administering, implementing and evaluating interdisciplinary programs; instructional methodology skills; skills for clinical supervision and teaching at the university level; skills for identifying funding sources, preparing successful funding applications, and providing required reporting for funded projects; skills for conducting research and publishing. In addition, comprehensive knowledge of principles and

practices of pediatrics, child development, developmental disabilities, including autism; of community organizations and relevant federal and state policy pertaining to children with autism/DD and of person/family-centered care, life course theory, the public health perspective.

POSITION DESCRIPTION: LEND Associate Director /Training Director

Position Definition:
Under the administrative direction of the LEND PD, the Associate Director/Training Director (AD/TD) will assist the LEND PD in the implementation of the LEND Project, and in cooperation with the LEND Faculty, assist in directing/coordinating the development, implementation, and evaluation of the LEND Program. The AD/TD will hold an academic rank of at least Assistant Professor of Pediatrics and will be academically responsible to the Chairman of the UC Department of Pediatrics.

Education and Training Responsibilities:
In collaboration with the LEND PD and the Core Faculty, assist in development, implementation, and evaluation of the Interdisciplinary LEND Curriculum. Provide didactic and interdisciplinary training. Participate in the interdisciplinary evaluation of trainees in conjunction with LEND Faculty.

Research Responsibilities:
Design and conduct research projects related to interdisciplinary training aspects of the LEND Project both individually and in collaboration with LEND and/or university faculty. Prepare and submit publications based on research/demonstration activities.

Administrative Responsibilities:
Assist the LEND PD in the implementation and evaluation of the LEND Project. Formulate, in cooperation with LEND PD and Core Faculty, training program policies. Participate as a member of the LEND IDTC. Assist the LEND PD in recruitment & retention of LTTs with leadership potential. Assist in the preparation of grant applications, progress reports, and national presentations related to the LEND Project as requested. Represent the LEND Project at meetings and conferences at the local, state, and national level.

Minimum Qualifications:
Education: Doctoral degree in a health-related or education-related profession.
Experience: Minimum of 5 years professional experience, preferably in an interdisciplinary setting serving children and youth with autism/DD. In addition, at least 3 years of experience in an administrative capacity concerning graduate education and training in the health or education professions, including experience with coordinating interdisciplinary educational programs. Public health and/or policy-related experience/education is highly desirable.

POSITION DESCRIPTION: LEND Core Faculty

Position Summary:
LEND Faculty will be administratively responsible to the LEND PD during the time allocated to the LEND Project. They will be academically responsible to the Dean/Department Head of the UC College/Department or affiliated university through which they have an appointment. LEND Faculty will participate as a member of the LEND IDTC.

Roles and Responsibilities:
Share responsibility for planning, designing, implementing, coordinating, and evaluating interdisciplinary and disciplinary training elements of the LEND program. Ensure supervision of all training and service elements of their discipline with special emphasis on autism/DD. Help select trainees and define criteria for recruiting trainees from their discipline. Serve as mentors to LEND LTTs from their discipline or related disciplines. Coordinate between the program and their professional associates, academic affiliates, clinical departments, and discipline counterparts in state and community programs. Engage in scholarship related to MCH populations, especially those with autism/DD.

Minimum Qualifications:
LEND Faculty must meet the minimum standards of education, experience, and certification/licensure accepted by their respective professions. A Doctoral degree from an accredited program, as well as post-doctoral training, will be required in all medical specialties and psychology. Except for the Family and Advocacy Faculty, a minimum of a master's degree in the discipline from an accredited program (Doctoral degree preferred) will be required. A minimum of 5 years of professional experience in their discipline will be required, including experience with children with autism/DD and 3 years of experience in graduate/post-graduate education and training. Additional experience will include working in an interdisciplinary setting and a history of scholarly activity.

Knowledge, Skills, and Abilities:
Demonstrated leadership and teaching and clinical experience in providing health and related services to individuals with autism/DD and their families; Knowledge and experience in person-/family-centered care; Ability to role model best practices in providing interdisciplinary and/or disciplinary services that are person-/family-centered, community-based, coordinated, and culturally and linguistically appropriate; Ability to collaborate to plan, organize, administer, implement and evaluate the disciplinary training program within the interdisciplinary LEND Project; Ability to work cooperatively and effectively with people from a wide variety of disciplines and backgrounds.

POSITION DESCRIPTION: LEND Family Faculty

Position Summary:
LEND Family Faculty will be administratively responsible to the LEND PD during the time allocated to the LEND Project. The Family Faculty is responsible for ensuring the inclusion of families and the family perspective in the program and assist in the development of curriculum pertaining to person-/family-centered care. LEND Family Faculty will participate as a member of the LEND IDTC in developing, implementing, reviewing, and evaluating elements of the LEND Curriculum and for coordinating interdisciplinary and disciplinary training elements and their evaluation within the Family discipline.

Roles and Responsibilities:
Serve as chief representative of Family discipline in the LEND program. Define appropriate criteria for recruitment and selection of trainees in the Family discipline. Serve as mentors to LTTs. Recruit family members as speakers at LEND sessions. Coordinate the Family Mentoring Project (FMP) including recruitment of family mentors, matching trainees with families, and monitoring trainee performance. Provide community presentations on family involvement within the LEND program. Provide feedback to clinical trainees using *Family-Centered-Care Observation Feedback Tool.* Participate in family advocacy groups in the community and collaborate with other community-based

organizations and agencies on family training and support. Represent the Family discipline on internal program, policy, or committees. Participate in LEND network workgroups related to the Family discipline.

Minimum Qualifications:

Required: Bachelor's degree (per CCHMC requirements), Parent/sibling of a child/adult with autism/DD; Knowledge of community networks of families of children with autism/DD

Knowledge, Skills, and Abilities:

See above for description of knowledge, skills, and abilities for Core Faculty

POSITION DESCRIPTION: LEND Advocacy Faculty

Position Summary:

LEND Advocacy Faculty will be administratively responsible to the LEND PD during the time allocated to the LEND Project. The Advocacy Faculty is responsible for ensuring the inclusion of Advocates and the Advocate perspective in the program and assist in the development of curriculum pertaining to the Advocates' needs, goals, and aspirations. LEND Advocacy Faculty will participate as a member of the LEND IDTC in developing, implementing, reviewing, and evaluating elements of the LEND Curriculum and for coordinating interdisciplinary and disciplinary training elements and their evaluation within the Advocacy discipline.

Roles/Responsibilities:

Serve as chief representative of Advocacy discipline in the LEND program. Evaluate and refine the Advocate Curriculum as needed, according to evaluation results. Coordinate recruitment of advocates for the advocate LEND positions; review applications; participate in interview and selection process. Provide disciplinary supervision for Advocate Discipline LEND trainees, including meeting with their trainee on a regular basis and cultivate self-advocacy skills and progress on their ILP. Function as mentor and advisor to ensure that the trainee has access to appropriate services and resources to aid in their success. Recruit advocates as speakers for LEND sessions. Provide community presentations on advocacy involvement within the LEND program. Actively participate in advocacy groups in the community.

Minimum Qualifications:

Education: Bachelor's degree preferred (per CCHMC requirements)

Required: Lived experience with disability and advocacy; Experience working with individuals and families with autism/DD

Knowledge, Skills, and Abilities:

See above for description of knowledge, skills, and abilities for Core Faculty

Other Personnel:

Data Coordinator (Hennessy): The current DC has been in this role since 2010 and dedicates 50% of her total effort to the LEND program and the remainder of her effort dedicated to UCCEDD. She reports administratively to the PD, participates in the NIRS Data Coordinator's Annual Meeting, and receives TA from AUCD as necessary. The DC assists LEND Leadership in implementing, monitoring, and evaluating activities through data collection. The DC is responsible for communicating with LEND Faculty and staff regarding the fiscal year reporting requirements. The DC collects information on current trainees, projects, activities and products that our faculty and staff

participate in each fiscal year and enters this information into NIRS. She is responsible for building surveys in *REDCap* that are used to collect data from current trainees for program improvement, including pre- and post-training surveys on program components. The DC maintains contact information for all former LTTs and is responsible for sending the Trainee Survey to graduates who are post LEND at 2, 5 and 10 years to measure LEND's impact.

Program Coordinator (Music): Provides overall administrative support to the LEND Program. She coordinates daily office operations including technology needs, maintains Canvas LMS and LEND training calendar, coordinates and schedules trainee interviews, handles travel for trainees and faculty, monitors daily attendance and make-up assignments, maintains scheduling of meetings and room assignments, documents meeting minutes, keeps inventory of supplies, plans and oversees special events, programs, conferences, and symposiums, and works with outside vendors as necessary. The PC is a 2021 graduate of the Cincinnati LEND program.

LEND Clinical Supervisors (Donovan, Goldsbury, Mattingly, Luneke): Provide clinical supervision in the disciplines of occupational therapy (Donovan), physical therapy (Goldsbury), audiology (Mattingly), and social work (Luneke) and for the Interdisciplinary Training Team and other discipline-specific clinical activities.

Policy Specialist (Gerhardt/Harris): This position, currently filled by a LEND Family discipline graduate, attorney, and mother of a young lady with Down syndrome, assists the LEND program in strengthening policy activities. Roles and responsibilities include: leading didactic sessions on policy; providing updates to trainees on current DD-related legislative topics & issues; preparing trainees for policy and awareness events; coordinating and attending legislative meetings to educate state and federal legislators on public policy changes impacting individuals with autism/DD and their families; preparing policy briefs; and meeting with local, state, and national policy partners. A LEND Advocacy graduate who is an autistic adult is onboarding to assume this role upon the current policy specialist's retirement.

Community Engagement Coordinator (Bonds): This position, filled by a LEND Community Engagement graduate who is a licensed OT, coordinates the Community Leadership Project (CLP) and Early Intervention Project (EIP).

In addition to the positions funded by the LEND grant, Cincinnati LEND receives in-kind support from faculty and clinicians in DDBP who perform a variety of functions including teaching LEND sessions as content experts and performing special education evaluations as a part of the ITT. In addition, DDBP faculty who are fully funded by various funding sources serve as SEBM team leaders and provide research mentorship to Cincinnati LEND.

Biosketch Chart

Name/Discipline/# of years as LEND Faculty Education and Certification/Licensure	Leadership Positions/Teaching	Clinical Experience/Research Experience
Carrie Atzinger Genetic Counseling (GC); 16 yrs University of Cincinnati, MS Board-Certified and Licensed Genetic Counselor	*ITT Supervisor & Coordinator of Observations Across Disciplines project;* Co-director of the Cincinnati GC Graduate Program; Site visitor for the Accreditation Council of Genetic Counselors; Various roles within NSGC including secretary/treasurer on Board of	Research interests: clinical supervision effectiveness and training; student and supervisor self-efficacy; Published > 23 peer-reviewed papers; Co-created Genetic Counseling Self-Efficacy Scale and Genetic Counseling Supervisory Self-Efficacy Scale; book chapter on "Evaluation in Supervision" in *A Practical Guide to Clinical Supervision in Genetic Counseling*

	Directors & co-leading the NSGC Leadership Development Program	
Marta Getz Social Work; 9 yrs University of Cincinnati, MSW, LISW-S Clinical Manager	*Course Instructor for ITT;* LISW Supervision group facilitator & mentor, Division of Social Services, Clinical Advancement mentor; Clinical Manager in MBHI	Clinical: 16 years as social worker in DDBP with content expertise in Autism/DD & Spina Bifida, 1.5 years Clinical Manager in MBHI, creating, developing family navigation program; supervising family navigators
Karen Harpster Occupational Therapy; 14 yrs The Ohio State University, PhD, OTR/L Associate Professor	*SEBM Team Lead;* Leads research training for both OT and PT graduate students. Supervision of undergraduate and graduate students.	Clinical: Evaluates and treats children with NDD, focusing on cerebral visual impairment (CVI) and cerebral palsy (CP). Leader of the OT team in the multidisciplinary CVI clinic and directs early detection efforts for CP and CVI at CCHMC. Research: Funded for 2 RCTs and an NIH R01 focused on early detection of CVI and developmental outcomes. > 34 peer-reviewed publications and >125 presentations both nationally and internationally.
Sally LeCras Physical Therapy; 8 yrs Arcadia University, MSPT/DPT Licensed PT Board-Certified Clinical Specialist in Pediatrics Assistant Professor-Affiliated, UC Leadership Certificate in Health Care Leadership, 2023 through APTA LAMP Institute of Leadership	*Course Instructor for ITT, Course Instructor for Evidence Based Case (EB Case)*; Director of PT Residency Program; Chair of APTA's Pediatrics Residency & Fellowship Special Interest Group (SIG); Leader, APTA's Clinical Practice Guideline Development Group for Idiopathic Toe Walking; Guest lecturer/Adjunct Instructor at Mount St. Joseph University, University of Cincinnati and University of Dayton	Clinical: Provides outpatient physical therapy services and evaluation and treatment in multi-disciplinary Cerebral Palsy Clinic; Cerebral Palsy Mentor for staff therapists; Research: 6 publications, including a clinical practice guideline and two book chapters; Associate Editor of Pediatric Physical Therapy Journal.
Lindsay Mays Psychology; 7 yrs PhD, Palo Alto University Licensed Clinical Psychologist Staff Psychologist III *Cincinnati LEND graduate, 2015	*Course Instructor for ITT*; Provides lectures on multidisciplinary diagnostics and autism/DD in UC Social Work Department; Professional Mentor for New Hires in NDBP, Supervisor for NDBP Postdoctoral Fellows, Member of BMCP IHE Clinical Workgroup	Clinical: Conducts multi-disciplinary diagnostic evaluations; Research: investigator on study evaluating a technology assisted intervention for children that are Deaf/HoH; investigator on study to establish neurodevelopmental profiles informing interventional strategies for children diagnosed with CVI; 5 publications on clinical care in autism/DD.
Naveen K. Nagaraj Audiology; 1st year as LEND faculty PhD, Hearing Science CCC A, Licensed Audiologist (Ohio, Utah) Associate Professor-Affiliated UC	Course Instructor for AuD and undergraduate programs, teaching courses in Amplification, Electrophysiology, Research Methods, and Diagnostic Audiology; former faculty member at Utah State University, University of Arkansas for Medical Sciences, and Manipal University, India	Clinical: experience in India and Australia in assessment and management of hearing loss and auditory processing across the lifespan; Research: Examining developmental and acquired listening difficulties through a cognitive hearing science framework that integrates auditory processing,

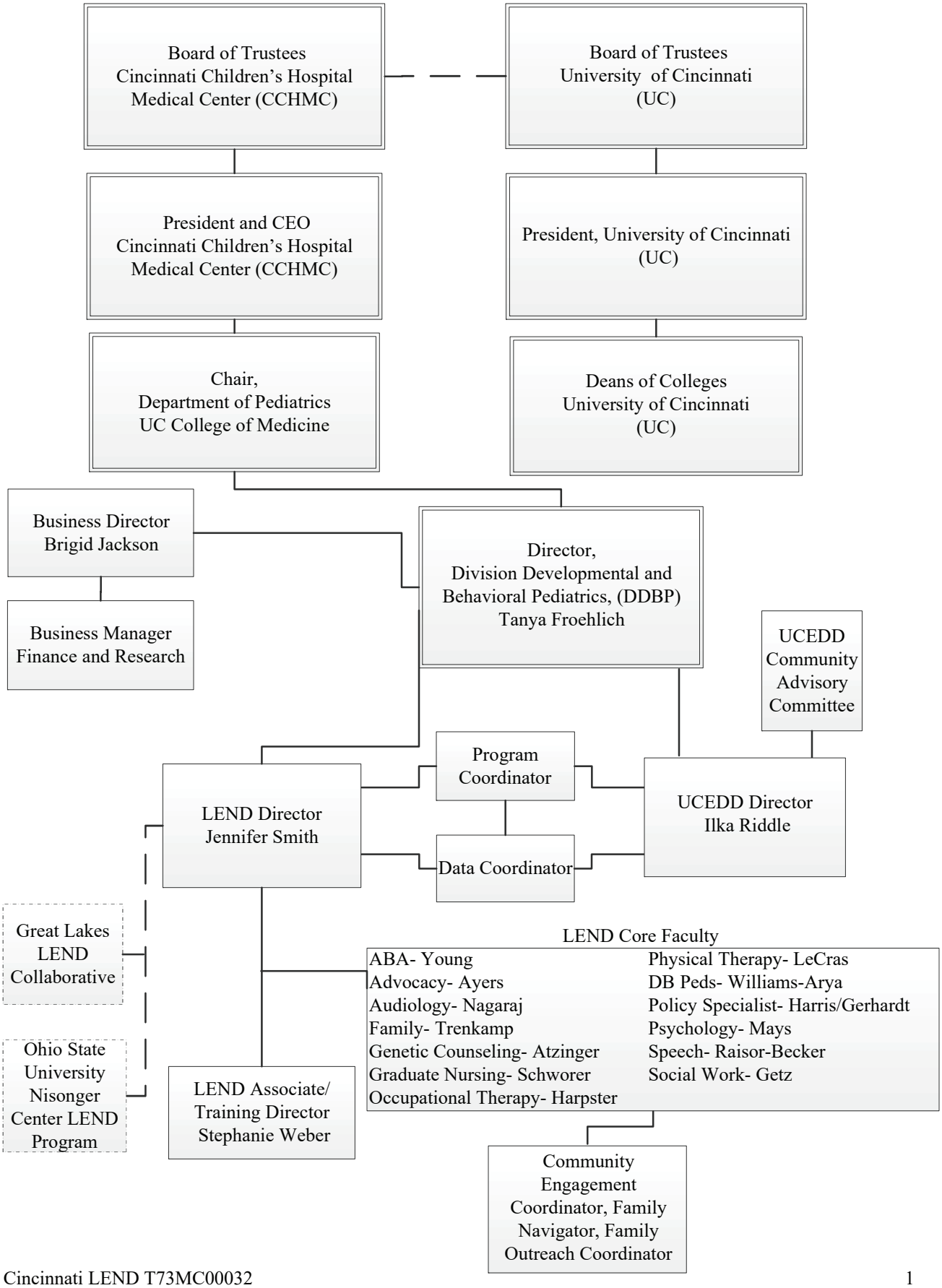
		working memory, attention, & language mechanisms. > 30 peer reviewed publications and four chapters.
Lesley Raisor-Becker Speech-Language Pathology; 6 yrs University of Cincinnati, PhD; Assistant Professor CCC-SLP	*Core SLP faculty for ITT*; Leads the Language and Literacy Enrichment Group (LLEG), training and supervising graduate students in the design of interventions for preschool age children with speech and language disorders. Managed U.S. Department of Education Personnel Preparation Grant: Preparing Early Intervention Providers for the Opioid Epidemic.	Clinical: 25 years practicing as a pediatric SLP providing assessment of language and literacy skills in preschool and school-aged children. Research: Collaborates on NIH R01 grant examining the developmental profiles of children diagnosed with CVI; Developed contextualized intervention protocols targeting phonological awareness and early literacy skills in at-risk preschool children.
Saoirse Schworer Graduate Nursing; 1 yr University of Cincinnati, MSN Board Certified Family Nurse Practitioner	*Course Instructor for Evidence Based Case Seminar;* Recruitment and supervision of graduate nursing students and established nurse practitioner trainees; member of DDBP Culture Club and Family Advisory Council	Clinical: Developmental Nurse Practitioner in DDBP; HUB team member for ECHO Autism STAT
Elizabeth Trenkamp Family; 4 yrs *Cincinnati LEND graduate, 2014	*Lead for Family Mentoring Project and Family Centered Care Observation Project*; Active member of the AUCD LEND Family Discipline Network; MBHI Family Navigator peer consultant	Clinical: DDBP Family Navigator serving > 1,300 families since 2015; CCHMC Schwartz Rounds Panelist; DDBP Family Support Team; Family Navigator Training for Head Start Preschoolers with Mental Health Concerns
Hannah Young Applied Behavior Analysis (ABA); 1 yr Ph.D., School Psychology M.Ed., Foundations in Behavior Analysis Licensed Psychologist, Certified Ohio Behavior Analyst, Board Certified Behavior Analyst-Doctoral	*LEND Faculty in Applied Behavior Analysis (ABA)*; Teaching Assistant, UC School Psychology Program (2019–2022); Program Manager for Intensive Behavior Consultations, a triage program developed to streamline families' access to Moderate to Severe Behavior programming; Behavior Response Coordinator for the DDBP Behavioral Response Team (BRT)	Clinical: Outpatient psychology therapy services through the Brief Intensive Behavior Therapy (BIBT) program, serving children and adolescents with autism/DD and moderate to severe behavioral presentations; Research: 7 peer-reviewed articles, 2 book chapters, 20 annual convention presentations.
Pam Williams-Arya DBP, 11yrs West Virginia University, MD Associate Professor UC *Cincinnati LEND graduate, 2009	*SEBM Team Lead & ITT supervisor; Core Course instructor on prenatal development;* Clinical supervisor for DBP fellows; HUB team member for ECHO SCOPE (Supporting Children of the Opioid Epidemic) and ECHO Autism for Allied Health; HUB team lead facilitator for ECHO Autism for Primary Care Providers; Scholarly oversight committee member for multiple DBP fellows	Clinical: Decade as primary care pediatrician before returning for DBP fellowship; Pediatric lead in the Next Step clinic for preschool-aged children with prenatal opioid exposure; expertise in evaluation and management of autism/DD; supports initiatives of Ohio Act Early Ambassador Project; Research: access to diagnostic evaluations, developmental profiles of children with prenatal opioid exposure, sleep and behavior challenges in children with Down syndrome, and preliteracy skills in children who are Deaf/HoH.

Attachment 3: Agreements with other entities

(Due to space limitations, all are on file at Cincinnati LEND program and available upon request)

- Tanya Froehlich, MD, MS, Division Director of DDBP, Thelma and Jack H. Rubinstein Chair and Professor of Pediatrics, UC College of Medicine, CCHMC
- Laurel K. Leslie, MD, MPH, Institute Director, Mental and Behavioral Health Institute, Cincinnati Children's Hospital Medical Center, Research Professor, College of Medicine, UC
- Ilka Riddle, PhD, Director, UC UCEDD, CCHMC, Cincinnati, Ohio
- Amy Turner-Iannacci, family member representing DDBP Family Advisory Council, DDBP, CCHMC
- Lyndsay Nash, Director, Ohio Department of Developmental Disabilities, Columbus, Ohio
- Bruce T. Vanderhoff, MD, MBA, Director, Ohio Department of Health, Columbus, Ohio
- Fawen Zhang, PhD, Professor and Chair, Department of Communication Sciences and Disorders, College of Allied Health Sciences, University of Cincinnati Medical Center
- Valerie Miller, PhD, MS, OTR/L, Assistant Professor, Occupational Therapy Program, College of Allied Health Sciences, UC
- Stacey Priya Raj, PhD, Associate Professor, Associate Director of Clinical Training, Clinical Psychology Doctoral Program, Xavier University, Cincinnati, Ohio
- Jennifer H Green, PhD, Clinical Professor & Coordinator of Clinical Traineeships, Department of Psychology, Miami University, Oxford, Ohio
- Andrea Witwer, PhD, Nisonger Center LEND Director; Columbus, Ohio
- Rachel Womack, PhD, MSW, Kentucky Leadership Education in Neurodevelopmental Disabilities (LEND) Director, The Human Development Institute, University of Kentucky
- Great Lakes LEND Collaborative (GLLC): University of Illinois-Chicago, University of Minnesota, University of Wisconsin-Madison, Indiana University, Ohio State University, Michigan LEND, and CCHMC/UC
- Tiffany Madden, M.A., CCC-SLP, Program Director, OCALI Center for the Young Child
- Sara Birkofer, Associate Director of Wellness & Accessibility, Cincinnati Art Museum
- Sadie Everett, Coordinator of Regional Autism Advisory Council (RAAC) of Southwest Ohio, Cincinnati, Ohio

Attachment 4: Organizational Chart Cincinnati LEND



Attachment 5: Cincinnati LEND Syllabus and Curriculum Graphic

Program Description:

The Cincinnati LEND Program employs the *Interdisciplinary LEND Curriculum (ILC)* to provide at least 360 hours of instruction over the course of the academic year through five components: Interdisciplinary Leadership Seminar (LEAD), Interdisciplinary Training Team (ITT), Core Course (CORE), Seminar in Evidence-based Methods (SEBM), and Disciplinary and Clinical Experiences.

The goals of the ILC are to assist trainees to:
1) Increase knowledge and skills in providing services to children and youth with autism and other developmental disabilities.
2) Reach a higher level of leadership knowledge and skills in the competency areas of the *MCH Leadership Competencies (v.5)*

Course Objectives:

Disciplinary and Clinical Experiences (Every T/TH 8:00am-12:00pm; some disciplines may have additional hours based on program)
Goal: To develop high quality clinicians, researchers, and future policymakers with specialty knowledge and skills in autism/DD.
Objectives: As a result of participating in this disciplinary and clinical experiences, trainees will:
1. Implement clinical skills for screening, evaluating, diagnosing or ruling out, and developing and providing evidence-based interventions to children and youth with autism/DD (clinical trainees).
2. Analyze public policy related to individuals with autism/DD and create effective educational materials for various audiences (non-clinical public health/family/advocacy trainees).
3. Formulate and test research hypotheses in collaboration with individuals with lived experiences (non-clinical research trainees).

Leadership Seminar (LEAD) (Every Tuesday from 1:00pm-3:00pm)
Goal: To develop leaders in the MCH field.
Objectives: As a result of participating in this course, trainees will:
1. Develop Individualized Leadership Plans (ILPs) based upon the MCH Leadership Competencies with faculty mentorship.
2. Communicate roles and responsibilities of one's own discipline.
3. Discuss the process for public policy development impacting individuals with autism/DD and their caregivers.
4. Educate various audiences on clinical practices, programs, or systems that benefit individuals with autism/DD and their families.
5. Demonstrate improved skills in person/family-centered care and providing culturally and linguistically appropriate services.
6. Identify and communicate one's own leadership style.

Interdisciplinary Training Team (ITT) (Every Tuesday from 3:00pm-5:00pm)
Goal: To develop effective interdisciplinary clinicians/professionals.

Objectives: As a result of participating in this course, trainees will:
1. Compare and contract own discipline roles and responsibilities to at least three other disciplines.
2. Evaluate children for developmental concerns and communicate findings and recommendations based upon diagnostic evaluations.
3. Demonstrate improved skills in person/family-centered care, conflict-resolution/negotiation, and group facilitation skills.

Core Course (CORE) (Every Thursday from 1:00pm-3:00pm)
Goal: To educate professionals broadly on the full range of neurodevelopmental disabilities (NDD), with a focus on autism
Objectives: As a result of participating in this course, trainees will:
1. Demonstrate improved understanding of diagnostic criteria and clinical presentation of various NDD.
2. Describe Life Course Theory as applies to individuals with autism/DD across the lifespan.
3. Analyze public health programs and systems for individuals with autism/DD and their families.
4. Identify and describe best practices for person/family-centered care.

Seminar in Evidence Based Methods (SEBM) (Every Thursday from 3:00pm-5:00pm)
Goal: To develop effective researchers and consumers of research in the autism/DD field.
Objectives: As a result of participating in this course, trainees will:
1. Demonstrate improved mastery of critical thinking, science-based judgement, research methods, and evidence-based practice.
2. Describe how their team project relates to broader agendas at various levels to benefit children with autism/DD and their families.
4. Demonstrate improved mastery of PowerPoint; poster-making; searching scholarly literature; searching and extracting data from publicly available databases; reading and summarizing scholarly literature; questionnaire and survey design; skills relevant to the collection, analysis and dissemination of data; and writing of abstracts, manuscripts, and/or grants.
5. Describe essential elements of strategies to protect human subjects involved in research and their confidential information.

Course Policies:

Class Attendance and Participation
Attendance and active participation is mandatory at ALL LEND sessions in order to fulfill both learning and stipend requirements. Active participation is measured by at least one comment verbally or in the Zoom chat box per session. **Please note: each 2-hour LEND curriculum session is considered 1 session (1-3pm =1 session and 3-5pm = 1 session). If you miss a full day of LEND, you have missed 2 sessions. It is the trainee's responsibility to sign in at the beginning of every LEND session to document attendance. On Zoom, type name into the chat box. In person, if you are in a seat by 5 minutes after the start of the session, you will be marked as in attendance by the LEND Coordinator. If trainees need to miss a LEND session for any reason, they must contact ALL of the following by email in ONE email as soon as possible prior to a missed day/session(s): Dr. Smith (Jennifer.Smith4@cchmc.org); Dr. Weber (Stephanie.Weber@cchmc.org);); Sarah Music

(Sarah.Music@cchmc.org) and Core Faculty member/clinical supervisor. Any absence will require a make-up assignment submitted no later than 2 weeks from date of missed session.

Access
Cincinnati LEND is committed to providing all trainees with equitable access to learning opportunities. University of Cincinnati's Accessibility Resources will be utilized to arrange for reasonable accommodations for trainees with an identified physical, psychological or cognitive disability. Trainees are encouraged to contact the LEND PD and TD to arrange for a confidential meeting to discuss services and accommodations. Contact should be initiated as soon as possible to allow adequate time for accommodations to be arranged.

Required Texts and Materials:
All materials will be available on the Cincinnati LEND's learning management system site called *Canvas* prior to LEND Orientation for the Fall semester and by December 1 for Spring semester.

Assignments:

Submitting Work
All written assignments must be typed and double-spaced unless otherwise specified. Page length guidelines are based on 12-point Times New Roman or Arial font 1-inch margins expectation. Other options are available to show trainee knowledge and skill development based on the various projects. These include, but are not limited to, creation of videos, slide decks, voice recordings, or other creative submissions following approval from the course director or project coordinator.

Make-Up Assignments
Four options for Make-up Assignments when a trainee misses a session are available on *Canvas*.

Sample LEND Schedule – Key	
1:00-3:00pm	Teal
3:00-5:00pm	Purple
1:00-5:00pm	Blue
All Day Event	Pink
Key session topics	*Italicized text*
Taught by Core Faculty	*
Taught by Invited Expert	**
Trainee as Co-presenter	^

Note: Trainees participate in discipline-specific clinical, research, or policy work every Tuesday and Thursday from 8:00am to 12:00pm.

Weeks 1-15 focus primarily on MCH Leadership Competencies Conceptual Framework areas of *Self* and *Others* as well as Leadership Competencies *Knowledge* and *Foundational Skills*.

Weeks 16-30 cover Conceptual Framework areas of *Others* and *Wider Community* and Leadership Competencies *Knowledge* and *Advanced Skills*.

Tuesday	Thursday
Week 1:	
LEAD: Values in Disabilities Book Club Discussion* *Identity-first & person-first language, disability representation*	Core: Values in Disabilities/The History of Disabilities* *Disability models, disability policy, history & key disability advocates*
LEAD: Leadership #1: True North* *Authentic leadership, Personal Leadership Development plan*	SEBM: Team Project Introductions* *Brief overview of team project opportunities for research*
Week 2:	
LEAD: Presentation and Writing Skills for MCH Leaders* *Effective & communication for tailored audiences, using plain language, accessibility, & health literacy principles*	Core: Intellectual/Developmental Disabilities* *Conceptualizations of ID & DD, signs, symptoms, & clinical diagnostic processes & limitations, epidemiology of ID & DD*
LEAD: In-depth review of LEND Projects* *Detailed descriptions of all LEND assignments with opportunities for question & answer*	SEBM: Evidence-Based Cycle & Summarizing Literature* *Building critical thinking skills, understanding research methods, introduction to science-based judgment*
Week 3:	
LEAD: Person- & Family-Centered Care (P/FCC)* *Defining and introducing how to practice P/FCC, soliciting input, shared decision-making, strengths-based approaches*	Core: Life Course Perspective* *Human development, risk & protective factors, adverse experiences, key transitions and influences*
ITT: Case Overviews & Initial Meeting* *Introduction to interdisciplinary care, defining roles, responsibilities, group processes, mandated abuse reporting*	SEBM: Research for LEND Leaders Part 1* *Advantages of qualitative & quantitative methods, developing research questions, summarizing and analyzing literature*
Week 4:	
LEAD: StrengthsFinder* *Self-reflection; impact of values, beliefs, communication, & experiences on one's personal & professional leadership style*	Core: Universal Design for Learning** *Key principles & history of UDL, applications in community, education, & employment*
LEAD: Empathy + Belonging #1: Examining Experiences** *Appreciating strengths & differences in perspectives of individuals & communities*	SEBM: Research for LEND Leaders Part 2* *Participatory action research, continuous quality improvement*
Week 5:	
LEAD: Disciplinary Presentations #1 *Trainees present individually or in small disciplinary groups*	Core: Community Health Factors & Public Health* *Systems of care, medical home, public health strategies, programs, & achievements, the social environment*
LEAD: Language Access** & Health Literacy**	SEBM: Team Meeting* *Team building, identifying strengths, delegating tasks*

Culturally & linguistically appropriate services, health literacy principles, differences in communication needs	

Week 6:

LEAD: Disciplinary Presentations #2 *Trainees present individually or in small disciplinary groups*	Core: Prenatal Development*^ *Brain structure & function, environmental toxins/exposures, vulnerable periods, neurocognitive development*
ITT: Team 1, Meeting 1* *Shared vision, active listening & rapport-building, diagnosing autism/DD*	SEBM: Team Meeting* *Developing research questions, project-specific tasks*

Week 7:

LEAD: Policy & Advocacy #1* Public Policy Overview *Title V Maternal and Child Health Services Block Grant Program, MCH- and DD-specific legislation*	Core: Early Identification of Autism/DD* *Child Find, Early Intervention, developmental milestones & "Learn the Signs. Act Early." Effective screening tools*
ITT: Team 2, Meeting 1* *See Week 6*	SEBM: Team Meeting* *Project-specific tasks*

Week 8:

LEAD: Empathy + Belonging #2: Examining Culture** *Culturally and linguistically appropriate services, strategies that address community factors for positive outcomes*	Core: Genetics & Genetic Syndromes* *Genetic technology, past, current, emerging roles of genetics & genetic counselors in disability community*
ITT: Team 1, Meeting 2* *Shared leadership, value of multiple perspectives, effective facilitation of groups, diagnosing autism/DD*	SEBM: Team Meeting* *Project-specific tasks*

Week 9:

LEAD: Policy & Advocacy #2: Advocacy Can Be Fun!** *Policy-making processes at state & national levels, methods for educating policymakers about needs of MCH populations*	Core: Trauma-Informed Interactions** *Trauma-informed care, resiliency models, risk & protective factors*
ITT: Team 2, Meeting 2* *See Week 8*	SEBM: Team Meeting* *Project-specific tasks*

Week 10:

LEAD: Sharing Difficult News #1* *Communicating in challenging situations, SPIKES approach*	Core: Autism** *Diagnostic criteria, differential diagnosis, epidemiology of autism, best practices for autism screening and diagnosis*
ITT: Team 1, Meeting 3*	SEBM: Team Meeting*

Managing team dynamics, consensus-building, diagnosing autism/DD	*Project-specific tasks*

Week 11:

EB Case: Attention-Deficit/Hyperactivity Disorder* *Diagnostic criteria, differential diagnosis, multidisciplinary evaluation & treatment approaches*	Core: Evidence-based Practice (EBP) in Autism** *EBP at various levels of support needs across lifespan, learning theory, behavioral & communication strategies*
ITT: Team 2, Meeting 3* *See Week 10*	SEBM: Team Meeting* *Project-specific tasks*

Week 12:

EB Case: Spina Bifida & Myelomeningocele* *Neural tube development, risk factors, multidisciplinary evaluation & treatment approaches*	Core: Environmental Strategies & EBPs in Autism/DD*^ *Functional Behavior Assessment & Behavior Improvement Plans, visual supports, strategies any discipline can use*
ITT: Team 1, Case Conference* *Summary of complex information, diagnosis and recommendations plan, assemble team members for final session with patient & family*	SEBM: Team Meeting* *Project-specific tasks*

Week 13:

EB Case: Down syndrome* *Chromosomal differences, risk factors, multidisciplinary evaluation & treatment approaches*	Core: Receptive & Expressive Language Development*^ *Milestones for speech sound & language development, assessing language, family-centered interventions*
ITT: Team 2, Case Conference* *See Week 12*	SEBM: Team Meeting* *Project-specific tasks*

Week 14:

EB Case: Fragile X* *Chromosomal differences, risk factors, multidisciplinary evaluation & treatment approaches*	No LEND Sessions- Thanksgiving
LEAD: Leadership #3* *Leadership styles, self-reflection on strengths & growth areas*	

Week 15:

LEAD: Leadership #4: Emotional Intelligence/Empathy* *Emotional self-awareness & regulation, empathy mapping*	Core: Semester Wrap-Up *Core Essay Discussion, conference report-back*
ITT: Final Session*	SEBM: Team Meeting* *Project-specific tasks*

Strengths & limits of ITT approach, addressing clinical & community program & policy challenges	

Week 16: (Return from Winter Break)

LEAD: Five Dysfunctions of a Team* *Communication & leadership styles, strategies for effective team building*	Core: Special Education + Learning Disabilities** *IDEA, FAPE, educational assessment process, medical diagnosis v. educational classification*
ITT: Observations Across Disciplines and Supervision & ITT Case Overviews*^ *Learning from observation, models & methods of supervision, providing feedback, navigating challenges in supervision*	SEBM: Team Meeting* *Project-specific tasks*

Week 17:

LEAD: Conflict-Resolution #1* *Thomas-Kilmann conflict modes, self-reflection, strategies of effective negotiation*	Core: Hearing Loss, Deafness, & Deaf Culture*^ *Current research in best practices, policies & state systems, EHDI*
LEAD: Empathy + Belonging #3: Examining Interactions** *Strategies for managing challenging team dynamics*	SEBM: Team Presentations^ *Presentations on research progress to date*

Week 18:

LEAD: Charting the Life Course + Medical Home** *Components of medical home, care coordination, CtLC framework & tools, support for transitions of care*	Core: Supporting Siblings**^ *Current research on strengths & needs of siblings across life course, support & resource initiatives, policy & program needs*
ITT: Team 1, Meeting 1* *Tools for screening & diagnosing autism/DD, empathy maps*	SEBM: Team Presentations^ *Presentations on research progress to date*

Week 19:

LEAD: Negotiation & Conflict-Resolution #2* *Psychological safety, application of strategies & techniques*	Core: Genetics*^ *Current ethical, legal, & social issues in genetics & public health, family perspectives & P/FCC*
ITT: Team 2, Meeting 1* *See Week 18*	SEBM: Team Meeting* *Project-specific tasks*

Week 20:

LEAD: Empathy + Belonging #4: Examining Systems *Clinical & public health systems, approaches to address community factors in MCH settings*	Core: Apraxia, Augmentative & Alternative Communication** *Diagnostic criteria, modes/types of communication, benefits of AAC, funding considerations*
ITT: Team 1, Meeting 2*	SEBM: Team Meeting*

Strategies to enhance team dynamics, empathy mapping	*Project-specific tasks*

Week 21:

LEAD: Policy & Advocacy #3: Legislative Meetings* *Analyze data & policies, use evidence & storytelling to present to key decision makers*	Core: Mental Health #1*^ *Dual diagnosis, diagnostic overshadowing, assessment tools in autism/DD, epidemiology*
ITT: Team 2, Meeting 2* *See Week 20*	SEBM: Team Meeting* *Project-specific tasks*

Week 22:

LEAD: Professionalism Escape Room** *Ethical decision-making, competency frameworks, team-based problem-solving*	Core: Mental Health #2*^ *Treatment approaches for anxiety & depression, suicide prevention & screening, systems-level considerations*
ITT: Team 1, Meeting 3* *Roles for disciplines other than one's own, empathy mapping*	SEBM: Team Meeting* *Project-specific tasks*

Week 23:

DD Awareness and Advocacy Day *Trainees & Faculty participate in event & legislative visits at Ohio Statehouse*	Core: Ethics* *Ethical principles, engaging those with lived experience, ethical dilemmas in autism/DD*
	SEBM: Team Meeting* *Project-specific tasks*

Week 24:

EB Case: Feeding Disorders* *Diagnostic criteria, differential diagnosis, multidisciplinary evaluation & treatment approaches*	Core: Healthy Relationships and Sexual Health in DD** *Puberty, sexuality, & building healthy relationships, resources & programs, systems-level considerations*
ITT: Team 2, Meeting 3* *See Week 22*	SEBM: Team Meeting* *Project-specific tasks*

Week 25:

EB Case: Developmental Coordination Disorder** *Diagnostic criteria, differential diagnosis, multidisciplinary evaluation & treatment approaches*	Core: Cerebral Palsy^ *Diagnostic criteria, screening & early identification, functional classifications, interventions & equipment*
ITT: Team 1, Case Conference* *Summary of complex information, diagnosis & recommendations plan, assemble team members for final session with patient & family, empathy mapping*	SEBM: Team Meeting* *Project-specific tasks, abstract development, dissemination strategies*

Week 26:

EB Case: Visual Impairments* *Diagnostic criteria, differential diagnosis, multidisciplinary evaluation & treatment approaches*	Core: Moving to Adult Medical Services** *Clinical and systems approaches to healthcare transitions from youth to adult systems, guardianship, shared decision-making*
ITT: Team 2, Case Conference* *See Week 25*	SEBM: Team Meeting* *Project-specific tasks, presentation & poster-making, dissemination strategies*

Week 27:

LEAD: Sharing Difficult News #2* (with actors) *Interdisciplinary standardized patient experiences; six clinical scenarios plus debrief discussion*	Core: Moving through Systems to Adulthood** *Clinical & systems approaches to transitions from youth to adult services, employment, housing, transportation*
	LEAD: Leadership Showcase* *Presentations by trainees: leadership journey or e-portfolio*

Week 28:

LEAD: Community Leadership Project/Early Intervention Final Session^ *Presentations by trainees on project experiences*	SEBM: MCH Poster Symposium with Nisonger Center *MCH Joint Poster Symposium hosted at Cincinnati Children's in Y1, Y3, Y5 and at Nisonger Center in Y2 & Y4*
ITT: Both Teams, Final Session* *Broader systems approach, leading teams effectively*	

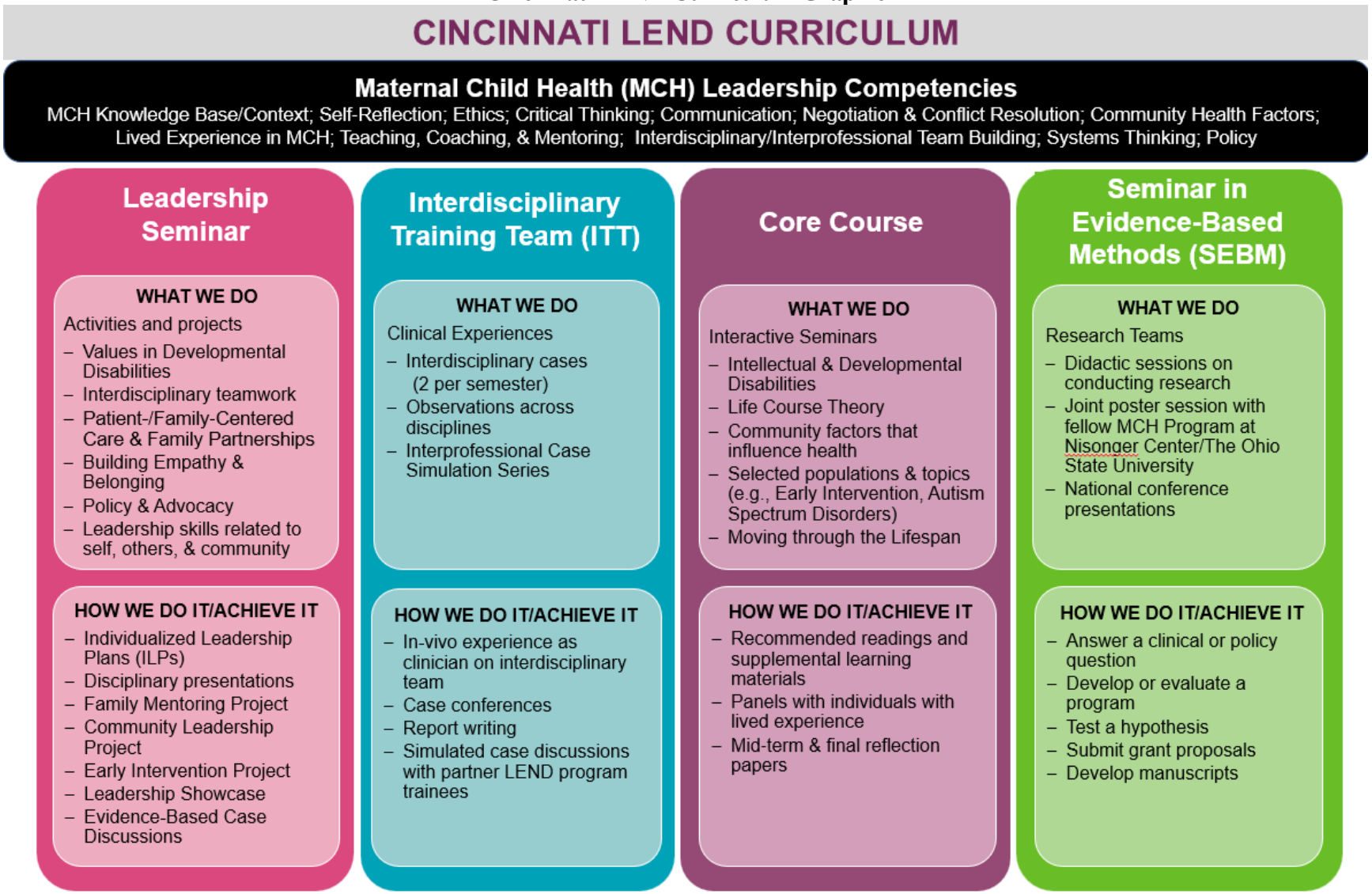
Week 29:

LEAD: Policy & Advocacy #4 Final Session* *Elected official panel, systems-level policy scenarios*	Core: Emerging Issues
LEAD: Administrative Roles* *Mission, vision, & goals of organizations, business & administrative principles related to planning, funding, budgeting, staffing, & evaluation*	LEAD: Leadership Showcase^ *Presentations by trainees: leadership journey or e-portfolio*

Week 30:

LEAD: Person-/Family-Centered Care Final Session* *Key takeaways from FMP & P-/FCC Observation Project*	Core: Semester Wrap-Up* *Core Essay Discussion, conference report-back, final surveys*
LEAD: Leadership Showcase^ *Presentations by trainees: leadership journey or e-portfolio*	Final LEND Session* *Field Trip to Cincinnati Art Museum*

Cincinnati LEND Curriculum Graphic



Attachment 6:
Funding Tier Calculation for LTTs over first four years of 2021-2026 Grant Cycle

LEND Funding Year	Number of Long-Term Trainees Reported in HRSA performance reports	Number of disciplines per year
July 1, 2021 – June 30, 2022	29	16
July 1, 2022 – June 30, 2023	27	15
July 1, 2023 – June 30, 2024	29	16
July 1, 2024 – June 30, 2025	32	13
Average number of LTTs for the 1st 4 years of grant cycle	29.25	15

Project Narrative for LEND Emerging Issues Supplement

Development of a novel behavioral intervention for children with prenatal opioid exposure to improve behavioral health outcomes, family functioning, and readiness to learn

The focus of this LEND Emerging Issues Supplement application by Cincinnati LEND is the development of a behavioral treatment package specific to the needs of children with prenatal opioid exposure (POE). Our team aims to develop and pilot an innovative treatment approach to improve mental and behavioral health outcomes in young children and their caregivers. The much higher rates of behavioral challenges experienced by children with POE can lead to negative health outcomes and decreased school-readiness. This proposal outlines how Cincinnati LEND plans to leverage a decade of previous research and clinical care and partner with caregivers with lived experience to develop and test a behavioral treatment package for children aged 3-8 years and their primary caregivers. We aim to include LEND trainees in this project to achieve our overall goal and meet our objectives to expand the workforce capacity in this subset of children with developmental and behavioral concerns.

PROBLEM

Greater Cincinnati, spanning Ohio, Kentucky, and Indiana, has been disproportionately impacted by the opioid crisis, with overdose death rates (40.5 per 100,000) exceeding the state average (30.5) between 2009–2018 (MacKinnon & Privitera, 2020). As opioid use during pregnancy has increased (Syvertsen et al., 2021), so has the need to address the developmental consequences of POE and neonatal opioid withdrawal syndrome (NOWS) (OhioMHAS, 2024). NOWS is associated with prematurity, low birthweight, and small for gestational age (Graeve et al., 2022), while neuroimaging studies show reduced brain volumes and white matter abnormalities in exposed infants (Wu et al., 2025). Although long-term outcomes remain understudied, emerging evidence links POE to increased risk for vision and language deficits, sensory processing challenges, executive dysfunction, and elevated rates of anxiety, ADHD, and emotional dysregulation (Lee et al., 2020).

To address these needs, Dr. Weber and colleagues developed an initial asynchronous training course for early childhood care and education professionals with support from the AUCD Focused Assistance to Support Training (FAST) project mechanism. Following the success of this work, the team partnered with Wyoming Institute for Disabilities (WIND) to create Project SCOPE (Supporting Children of the Opioid Epidemic), a federally funded (2019–2022) national training initiative using the Project ECHO model. The program has reached over 10,000 professionals nationally and 2,000 regionally across disciplines including early childhood, healthcare, and education. Project SCOPE continues through state and institutional support. Through case discussions and community input, it became clear that children with POE have distinct, complex developmental and psychosocial needs, often shaped by caregiver substance use disorder, disrupted caregiving arrangements, and trauma exposure. In response, Cincinnati Children's established a care pathway for children transitioning from high-risk follow-up clinics in Neonatology. In 2023, Dr. Weber's team launched the Next Step Clinic, a multidisciplinary evaluation clinic for children ages 2.5–6, providing individualized recommendations based on comprehensive assessments from psychology, occupational therapy, speech-language pathology, and developmental-behavioral pediatrics. The clinic was informed by caregiver listening sessions and has become a key training site for LEND trainees and other health professionals.

Among the first 100+ children evaluated, common concerns included attention, impulsivity, and behavioral challenges, with elevated rates of complex ADHD (30%) compared to national

estimates (~10%). Additional concerns included sensory processing difficulties (46%), speech-language deficits (26%), and executive functioning challenges (40%). Intervention needs were substantial, with frequent recommendations for behavioral therapy (72%), occupational therapy (46%), and speech therapy (42%).

Behavioral concerns were the most frequently reported by caregivers, yet **evidence-based interventions tailored to children with POE and their caregivers remain limited**. Building on prior research and direct work with families in recovery settings with a previous LEND research team, key themes identified by caregivers included: (1) validating emotions in themselves and their children; (2) managing complex feelings and child behaviors; (3) discussing substance use in developmentally appropriate ways; (4) accessing practical resources (e.g., mental health, education, housing); and (5) increasing community awareness of POE impact in children. These insights directly inform our proposed goals and intervention approach.

GOALS and OBJECTIVES

The proposed project aims to create replicable intervention strategies to be tested by mental health providers to have widespread impact on future research and clinical care for this population and ultimately mitigate the adverse impact of POE on children and their families. Our improved understanding of various behavioral intervention models and the specific behavioral needs and priorities of families affected by the opioid crisis will be used to design a more effective and targeted behavioral intervention package. We aim to prevent behavioral and mental health disorders from progressing to diagnosis or escalating in severity level through early recognition and intervention in this high-risk community-based population.

A caregiver-child treatment package utilizing components of other evidence-based, trauma-informed intervention programs has not been researched on children with POE to date. Our team aims to test intervention modules that have previously been researched with similar populations (e.g., children with trauma histories, who are adopted, who have developmental delays/disabilities) to identify best practices for dissemination. Our team will work with caregivers with lived experience, to develop a modular treatment package leveraging strategies from Child Parent Psychotherapy, Parent-Child Interaction Therapy and Applied Behavior Analysis. This work will emphasize improved behavioral health outcomes through reduction in behavioral symptomology, enhanced caregiver-child interactions, and increased family quality of life. Through the implementation phase, we will identify the components with highest feasibility for caregivers, effectiveness to decrease mental and behavioral health concerns, and scalability to be used within other behavioral health clinics.

METHODOLOGY

We will develop treatment modules which will focus on externalizing and internalizing behaviors, emotional dysregulation, and sensory processing difficulties highly prevalent in children aged 3-8 years with POE. To start, **we have an already identified qualified team** whose members are skilled in utilizing trauma-informed approaches and working with this vulnerable population. An interdisciplinary team led by child psychology (Dr. Weber), developmental and behavioral pediatrics (Dr. Williams-Arya), and occupational therapy (Elsie Bush, Cincinnati LEND graduate and OT for Next Step Clinic) will develop a 6 to 12-module treatment package. **LEND Trainees will be involved in all levels of the project** either through the SEBM course, their disciplinary work, or Community Leadership Project. This model will provide a structured opportunity to bring together interdisciplinary trainees, caregivers of children with disabilities, and self-advocates to support the development of treatment modules across the LEND year. **We will work alongside caregivers with lived experience**

and professionals from other disciplines including peer recovery and nursing. Content development will be informed by feedback gained from previous group level assessments involving caregivers with lived experience as well as ongoing collaboration from at least three biological caregivers with lived experience of substance use disorder. Trainees will focus on creating module facilitation materials, including visual supports, home programming resources, and session content grounded in current evidence-based practice. All our work will be approved by our hospital's Institutional Review Board to ensure ethical practices in human subjects research.

Based upon research to date, anticipated topics for each module are detailed below. The number of sessions within each module will be of evaluation focus in the project. Of particular focus will be to identify the strategies and timing of interventions related to developmentally appropriate education on the impact of substance use disorders. Caregivers who participate in the treatment package with their child will be compensated $100 per session for their time and effort for the project.

Proposed Module Topics:

1) **Emotional Awareness**: Build caregiver and child emotional awareness; address caregiver trauma; discuss mindfulness and self-care.
2) **Emotion Communication**: Teach emotion labeling, validation, and introduce co-regulation.
3) **Empathy Expression**: Foster empathy, acceptance of temperament differences, and caregiver curiosity.
4) **Predictable Routines**: Establish structured routines with proactive sensory strategies.
5) **Sensory Environment**: Adapt environments and routines to reduce sensory-driven dysregulation.
6) **Co-Regulation Through Play**: Use collaborative play to teach co- and self-regulation skills.
7) **Positive Parenting**: Promote special play time, praise, and positive language.
8) **Relationship Nurturing**: Strengthen relationships; build stress management and coping skills.
9) **Felt Safety & Sensory Awareness**: Introduce felt safety and increase caregiver understanding of the child's sensory profile.
10) **Interoceptive Awareness**: Help children recognize body cues and emotional states.
11) **Limit Setting**: Support consistent boundaries, routines, and sensitive parental authority.
12) **Self-Advocacy & Modeling**: Model and reinforce self-regulation and self-advocacy skills.

EVALUATION

While the evaluation plan will be formalized in partnership with caregivers with lived experience and LEND trainees on the team, we propose the following: To test feasibility, we will assess the practicality of implementing behavioral health intervention for children with POE and their families by examining recruitment, retention, and adherence rates. Acceptability will be measured by participant satisfaction and engagement with intervention strategies through surveys and interviews. Preliminary effectiveness will be measured by examining initial changes in behavioral health outcomes and family quality of life, such as reduced symptoms of emotion dysregulation and increased parenting competence. Participants will be asked to complete a post-intervention evaluation to report their satisfaction with the sessions, the knowledge gained, intent to utilize new behavioral management knowledge and strategies, as well as improved caregiving confidence. Questionnaires will be distributed through individualized web links using the REDCap research management system. REDCap is HIPAA and FERPA secure and is the

gold standard for internet-based healthcare research. We aim to disseminate findings through national presentations and publications.

BUDGET and BUDGET JUSTIFICATION NARRATIVE: UC prime award
These supplemental funds will be a sub-award from UC to CCHMC. This total support reflects that all personnel are administratively located at CCHMC and that program activities are carried out through DDBP of CCHMC. Costs were estimated using data regarding current salaries, fringe benefit rates, and indirect costs for each of the faculty and trainees. The UC grants management team and DDBP business team have an established plan and system for monitoring this sub-award contract.

BUDGET and BUDGET JUSTIFICATION NARRATIVE: CCHMC sub-award
All costs will be covered by the supplement sub-award from UC to CCHMC and are shown in the table and narrative below.

Budget Item	**Total**
Personnel	$31,250
Consultants for content development	$6750
Participant compensation	$6,000
Supplies	$2,000
Indirect (8%)	$4,000
	TOTAL: $50,000

Personnel: We are seeking personnel support in the amount of $31, 250 for Dr. Weber (5% FTE), who will lead the project and provide project implementation, and a project coordinator (3% FTE) who will be responsible for day-to-day project management and operations. Other personnel time will include Dr. Williams-Arya (2% FTE) and Elsie Bush (10% FTE) for content development and intervention implementation.

Consultants: We plan to have three biological family members with lived experience (60 hours @ $25.00/hour = $4,500), one certified peer-recovery specialist (25 hours @ $25.00/hour = $750), and one nurse care manager (30 hours: $1,500 honorarium). Funds are requested in the amount of $6,750 to compensate them adequately for their time and effort.

Participant Compensation: We plan to test the intervention package with five families who will be provided with $100/session or up to $1200 total for their time and effort. Funds are requested in the amount of $6,000 for participant compensation.

Supplies: We request supply funds in the amount of $2,000 to support materials development for recruitment materials as well as any necessary treatment materials to be used as part of the behavior intervention.

OMB Number: 4040-0001
Expiration Date: 01/31/2029

R&R SUBAWARD BUDGET ATTACHMENT(S) FORM

Instructions: On this form, you will attach the R&R Subaward Budget files for your grant application. Complete the subawardee budget(s) in accordance with the R&R budget instructions. Please remember that any files you attach must be a PDF document.

Click here to extract the R&R Subaward Budget Attachment

Important: Please attach your subawardee budget file(s) with the file name of the subawardee organization. Each file name must be unique.

1) Please attach Attachment 1	CCHMC	Add Attachment	Delete Attachment	View Attachment
2) Please attach Attachment 2		Add Attachment	Delete Attachment	View Attachment
3) Please attach Attachment 3		Add Attachment	Delete Attachment	View Attachment
4) Please attach Attachment 4		Add Attachment	Delete Attachment	View Attachment
5) Please attach Attachment 5		Add Attachment	Delete Attachment	View Attachment
6) Please attach Attachment 6		Add Attachment	Delete Attachment	View Attachment
7) Please attach Attachment 7		Add Attachment	Delete Attachment	View Attachment
8) Please attach Attachment 8		Add Attachment	Delete Attachment	View Attachment
9) Please attach Attachment 9		Add Attachment	Delete Attachment	View Attachment
10) Please attach Attachment 10		Add Attachment	Delete Attachment	View Attachment

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: JZD1HLM2ZU83 **Enter name of Organization:** Children's Hospital Medical Center

Budget Type: ☐ Project ☒ Subaward/Consortium **Budget Period:** 1 **Start Date:** 07/01/2026 **End Date:** 06/30/2027

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith		166,329.00	6.00			83,164.00	24,035.00	107,199.00
Project Role:	PD/PI										
	Steph		Weber		148,191.00	5.04			62,240.00	17,987.00	80,227.00
Project Role:	Training Director										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 187,426.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
13	Core Faculty	12.59			109,184.00	31,554.00	140,738.00
4	LEND Clinical Supervision Staff	2.40			16,045.00	4,637.00	20,682.00
4	Other Personnel	13.20			75,809.00	21,909.00	97,718.00
6	LEND Trainees	11.70			63,456.00	6,693.00	70,149.00
27	**Total Number Other Personnel**					**Total Other Personnel**	329,287.00

Total Salary, Wages and Fringe Benefits (A+B) 516,713.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	5,500.00
2. Foreign Travel Costs	
Total Travel Cost	5,500.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	5,400.00
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	5,400.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	9,971.00
2.	Publication Costs	
3.	Consultant Services	11,781.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other: Panelist Reimbursement	1,340.00
9.	Other: Presenter Honorariums ($250/ person)	2,000.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	25,092.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	552,705.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	552,705.00	44,216.00
		Total Indirect Costs	44,216.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	596,921.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	596,921.00

L. Budget Justification

(Only attach one file.) 1246-CCHMC Budget Justification Narrat | Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: JZD1HLM2ZU83 **Enter name of Organization:** Children's Hospital Medical Center

Budget Type: ☐ Project ☒ Subaward/Consortium **Budget Period:** 2 **Start Date:** 07/01/2027 **End Date:** 06/30/2028

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith		171,319.00	5.04			71,954.00	20,795.00	92,749.00
Project Role:	PD/PI										
	Steph		Weber		152,637.00	4.80			61,055.00	17,645.00	78,700.00
Project Role:	Training Director										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 171,449.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
13	Core Faculty	12.58			113,675.00	32,852.00	146,527.00
4	LEND Clinical Supervision Staff	2.10			17,037.00	4,924.00	21,961.00
4	Other Personnel	16.20			91,993.00	26,586.00	118,579.00
6	LEND Trainees	11.70			63,558.00	6,722.00	70,280.00
27	**Total Number Other Personnel**					**Total Other Personnel**	357,347.00

Total Salary, Wages and Fringe Benefits (A+B) 528,796.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	5,000.00
2. Foreign Travel Costs	
Total Travel Cost	5,000.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	1,000.00
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	1,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	3,024.00
2.	Publication Costs	
3.	Consultant Services	11,781.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other: Panelist Reimbursement	1,340.00
9.	Other: Presenter Honorariums ($250/ person)	1,500.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	17,645.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	552,441.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	552,441.00	44,195.00
		Total Indirect Costs	44,195.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	596,636.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	596,636.00

L. Budget Justification

(Only attach one file.) 1246-CCHMC Budget Justification Narrat | Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: JZD1HLM2ZU83 **Enter name of Organization:** Children's Hospital Medical Center

Budget Type: ☐ Project ☒ Subaward/Consortium **Budget Period:** 3 **Start Date:** 07/01/2028 **End Date:** 06/30/2029

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith		176,458.00	4.80			70,583.00	20,399.00	90,982.00
Project Role:	PD/PI										
	Steph		Weber		157,216.00	4.32			56,598.00	16,357.00	72,955.00
Project Role:	Training Director										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 163,937.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
13	Core Faculty	12.58			116,408.00	33,642.00	150,050.00
4	LEND Clinical Supervision Staff	2.10			17,548.00	5,071.00	22,619.00
4	Other Personnel	16.20			94,753.00	27,384.00	122,137.00
6	LEND Trainees	11.70			63,664.00	6,753.00	70,417.00
27	**Total Number Other Personnel**					**Total Other Personnel**	365,223.00

Total Salary, Wages and Fringe Benefits (A+B) 529,160.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	5,000.00
2. Foreign Travel Costs	
Total Travel Cost	5,000.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	1,000.00
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	1,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	2,876.00
2.	Publication Costs	
3.	Consultant Services	11,781.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other: Panelist Reimbursement	1,340.00
9.	Other: Presenter Honorariums ($250/ person)	1,000.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	16,997.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	552,157.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	552,157.00	44,173.00
		Total Indirect Costs	44,173.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	596,330.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	596,330.00

L. Budget Justification

(Only attach one file.) 1246-CCHMC Budget Justification Narrat | Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: JZD1HLM2ZU83 **Enter name of Organization:** Children's Hospital Medical Center

Budget Type: ☐ Project ☒ Subaward/Consortium **Budget Period:** 4 **Start Date:** 07/01/2029 **End Date:** 06/30/2030

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith		181,752.00	4.80			72,701.00	21,011.00	93,712.00
Project Role: PD/PI											
	Steph		Weber		161,933.00	3.84			51,818.00	14,976.00	66,794.00
Project Role: Training Director											

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 160,506.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
13	Core Faculty	12.58			119,223.00	34,455.00	153,678.00
4	LEND Clinical Supervision Staff	2.10			18,075.00	5,224.00	23,299.00
4	Other Personnel	16.20			97,595.00	28,205.00	125,800.00
6	LEND Trainees	11.70			63,773.00	6,784.00	70,557.00
27	**Total Number Other Personnel**					**Total Other Personnel**	373,334.00

Total Salary, Wages and Fringe Benefits (A+B) 533,840.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	3,035.00
2.	Foreign Travel Costs	
	Total Travel Cost	3,035.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	
3.	Travel	
4.	Subsistence	
5.	Other	
	Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	1,000.00
2.	Publication Costs	
3.	Consultant Services	11,781.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other: Panelist Reimbursement	1,340.00
9.	Other: Presenter Honorariums ($250/ person)	1,000.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	15,121.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	551,996.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	552,996.00	44,160.00
		Total Indirect Costs	44,160.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	596,156.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	596,156.00

L. Budget Justification

(Only attach one file.) 1246-CCHMC Budget Justification Narrat | Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: JZD1HLM2ZU83 **Enter name of Organization:** Children's Hospital Medical Center

Budget Type: ☐ Project ☒ Subaward/Consortium **Budget Period:** 5 **Start Date:** 07/01/2030 **End Date:** 06/30/2031

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Jennifer		Smith		187,205.00	4.13			64,474.00	18,633.00	83,107.00
Project Role:	PD/PI										
	Steph		Weber		166,791.00	3.84			53,373.00	15,425.00	68,798.00
Project Role:	Training Director										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 151,905.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
	Secretarial/Clerical						
13	Core Faculty	12.58			122,122.00	35,293.00	157,415.00
4	LEND Clinical Supervision Staff	2.10			18,617.00	5,380.00	23,997.00
4	Other Personnel	16.20			100,523.00	29,051.00	129,574.00
6	LEND Trainees	11.70			63,887.00	6,817.00	70,704.00
27	**Total Number Other Personnel**					**Total Other Personnel**	381,690.00

Total Salary, Wages and Fringe Benefits (A+B) 533,595.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

	Funds Requested ($)
1. Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	3,000.00
2. Foreign Travel Costs	
Total Travel Cost	3,000.00

E. Participant/Trainee Support Costs

	Funds Requested ($)
1. Tuition/Fees/Health Insurance	
2. Stipends	
3. Travel	
4. Subsistence	
5. Other	
Number of Participants/Trainees — **Total Participant/Trainee Support Costs**	

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	1,000.00
2.	Publication Costs	
3.	Consultant Services	11,781.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Other: Panelist Reimbursement	1,340.00
9.	Other: Presenter Honorariums ($250/ person)	1,000.00
10.		
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	15,121.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	551,716.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	551,716.00	44,137.00
		Total Indirect Costs	44,137.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

DHHS, Olulola Oluborode, Phone (214-767-3261)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	595,853.00

J. Fee

Funds Requested ($)

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	595,853.00

L. Budget Justification

(Only attach one file.) 1246-CCHMC Budget Justification Narrat | Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

	Totals ($)	
Section A, Senior/Key Person		835,223.00
Section B, Other Personnel		1,806,881.00
Total Number Other Personnel	135	
Total Salary, Wages and Fringe Benefits (A+B)		2,642,104.00
Section C, Equipment		
Section D, Travel		21,535.00
1. Domestic	21,535.00	
2. Foreign		
Section E, Participant/Trainee Support Costs		7,400.00
1. Tuition/Fees/Health Insurance		
2. Stipends		
3. Travel	7,400.00	
4. Subsistence		
5. Other		
6. Number of Participants/Trainees		
Section F, Other Direct Costs		89,976.00
1. Materials and Supplies	17,871.00	
2. Publication Costs		
3. Consultant Services	58,905.00	
4. ADP/Computer Services		
5. Subawards/Consortium/Contractual Costs		
6. Equipment or Facility Rental/User Fees		
7. Alterations and Renovations		
8. Other 1	6,700.00	
9. Other 2	6,500.00	
10. Other 3		
11. Other 4		
12. Other 5		
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	2,761,015.00
Section H, Indirect Costs	220,881.00
Section I, Total Direct and Indirect Costs (G + H)	2,981,896.00
Section J, Fee	
Section K, Total Costs and Fee (I + J)	2,981,896.00

Budget Justification Narrative: UC prime award

Cincinnati LEND has a strong history of producing leaders who work with and on behalf of children and youth with autism/DD and their families. The proposed budget plan for 2026-2031 builds on the program's strengths and on outcomes accomplished through a long-standing affiliation between UC and CCHMC. The expenses for each year of the project are projected to be the same across the 5-year project period except for adjustments due to increases in fringe benefits and annual merit increases.

The following funds are requested for resources needed to accomplish the outlined LEND training objectives (see **Attachment 1: Work Plan**) by supporting the proposed project activities. We have allocated funds accordingly so that core faculty and key personnel have enough devoted time to achieve project objectives.

We have ensured that the budget and budget narrative align with the stated requirements. Based on the calculation tool in **Attachment 6,** Cincinnati LEND has met the minimum number of required trainees (average of 29.25 LTT per year) in the first four years of the previous grant cycle (2021-2026) to qualify for Funding Level Tier 4. Cincinnati LEND will continue to meet the minimum number of trainees (29), and number of trainee disciplines (12) and faculty disciplines (10) to qualify for Tier 4 funding for all 5 years of the upcoming budget cycle. We have allocated funds to financially compensate trainees with disabilities and family trainees.

University of Cincinnati Prime Award: Total project costs: $734,000 = $731,058 direct costs + $2,942 Indirect costs (8% of Direct Costs + 8% first $25K of Subaward)

Faculty/Other Personnel: $7,978
We request support for one position that is salaried through UC: the position of LEND core faculty member (for the speech language pathology discipline) Lesley Raisor-Becker at 5% effort. Specific responsibilities include: participating as a member of the LEND's IDTC and providing clinical supervision of SLP trainees through ITT.

Participant/Trainee Support Costs: $122,368
Stipends: Funds in the total amount of **$122,368** are requested to provide stipends to trainees from various disciplines including two or three family trainees and one or two trainees with disabilities who will compensated at the same amount as Master's Level trainees ($6,095).

Master's Level trainees will be from various disciplines: Applied Behavior Analysis (1), Clinical Counseling (1-2), Community Engagement (1), Disability Studies (2), Genetic Counseling (2-3), Public Health (1), Social Work (1-2), Speech-Language Pathology (3), other (e.g., Adult Services, Child Life, etc. - # to vary across years).

In addition, three or four pre-doctoral trainees in Psychology will be provided a stipend in the amount of $9,981. The number of stipends supported by the UC prime award will vary depending on the composition of the annual cohort. All post-doctoral trainees are CCHMC employees and funded through the CCHMC sub-award. A stipend in the amount of $1,000 is requested to fund one McNair Scholar to participate in an SEBM project. There are no costs associated with training short- and/or medium-term trainees as these trainees do not receive a stipend.

Other Direct Costs: $600,713

Consultant services: ($3,792) Jason Harris, an autistic advocate and graduate of Cincinnati LEND, will be onboarding during the first year of the grant with the intent of assuming the role of policy specialist upon the retirement of our current policy specialist, Jane Gerhardt.

Sub-awards/Consortiums/Contractual Costs: **$** (CCHMC sub-award): $596,921

The sub-award to CCHMC is the only Other Direct Cost. The detailed sub-award budget justification is provided in the separate budget justification narrative. A large proportion of total support requested is on the sub-award reflecting that most faculty and some trainees are administratively located at CCHMC and that program activities are carried out through DDBP of CCHMC. Costs were estimated using data regarding current salaries, fringe benefit rates, and indirect costs for each of the faculty and trainees. The UC grants management team and DDBP business team have an established plan and system for monitoring this sub-award contract.

Budget Justification Narrative: CCHMC sub-award

As described in the prime award budget justification narrative, most personnel costs for the project are included in the sub-award from UC to CCHMC. The FY27 (begins 07/01/26) fringe benefit rate for CCHMC employees is projected to be 28.9% for faculty and staff and includes retirement, medical insurance, tuition reimbursement, life insurance and unemployment compensation. Table 1 shows the various expenses on the CCHMC sub-award.

Table 1.

	CCHMC
A. + B. TOTAL PERSONNEL SENIOR/ KEY PERSON AND OTHER PERSONNEL	446,564
D. Faculty and Staff Travel	
Travel	5,500
E. TRAINEE EXPENSES	
Stipends/Salary Support	80,130
Travel	5,400
Other	0
Total Trainee Expenses	85,529
F. TOTAL "OTHER DIRECT COSTS"	
"Other" including Consultant Services	15,111
G. TOTAL DIRECT COSTS (A through F)	552,705
H. Indirect (8% of Direct Costs + 8% first $25K of Subaward)	44,216
I. Total Project Costs	**596,921**
TOTAL PROJECT COSTS (2026-2031)	**2,981,897**

Senior/Key Personnel:
Key Personnel for the project include Jennifer Smith, PsyD, BCBA-D, the Project Director. Dr. Smith will dedicate 50% of her time to the LEND project to ensure fulfillment of expected requirements of the program. Stephanie Weber, PsyD, MPH, the Associate/Training Director will dedicate 42% of her time to the LEND project to ensure fulfillment of expected requirements of the program.

Other Personnel:
LEND faculty and other personnel: The dedicated portion of time funded by the MCH Training grant supports faculty and other personnel's participation in activities to fulfill training program goals and objectives through training program curriculum development, implementation, and evaluation; recruitment of trainees; training; clinical supervision; evaluation of trainees; trainee follow-up; development, implementation and evaluation of continuing education and technical assistance projects; collection and analysis of program evaluation data; dissemination of training program models and products; and establishment and maintenance of partnerships with community-based organizations and with Title V and other related programs. See below for faculty, clinical supervisors, and other personnel's FTE commitment to the LEND program to meet project objectives.

LEND Core Faculty	Percent FTE funded by MCHB funds	LEND Core Faculty	Percent FTE funded by MCHB funds
LEND Director (Smith)	50%	Audiology (Nagaraj)	2%
LEND Associate/Training Director (Weber)	42%	Nursing (Schworer)	3%
Self-Advocacy (Ayers)	5%	Physical Therapy (LeCras)	6%
Genetic Counseling (Atzinger)	9%	Psychology (Mays)	5%
Social Work (Getz)	0%– in-kind	Occupational Therapy (Harpster)	5%
Family (Trenkamp)	100%	Developmental Behavioral Pediatrics (Williams-Arya)	16.67%
Speech Language Pathology (Raisor-Becker)	8%	Policy Specialist (Gerhardt)	10%
Hannah Young (ABA)	5%		
LEND Clinical Supervisors		**LEND Clinical Supervisors**	
Occupational Therapy (Donovan)	10%	Audiology (Mattingly)	5%
Physical Therapy (Goldsbury)	5%	Social Work (Luneke)	2%
Other Personnel		**Other Personnel**	
Program Coordinator (Music)	50%	Family Outreach Associate (Turner-Iannacci)	10%
Data Coordinator (Hennessy)	50%	Katie Bonds (Community Engagement)	5%

Trainees: Long-term trainees at various levels (Master's, Pre-Doctoral, and Post-Doctoral) who participate in residencies, fellowships, and externships are employees of CCHMC and will be funded through the CCHMC sub-award via salary support (in parentheses after each position). These include a Pediatric Physical Therapy Resident ($16,206), a Pediatric Occupational Therapy Resident ($16,206), a Developmental-Behavioral Pediatrics Post-Doctoral Fellow ($16,206), a Social Work Fellow ($8,558), an Audiology Extern ($8,558), and an Advanced Clinical Counseling Trainee ($4,415). Trainees from these disciplines are expected to participate each year. In addition, a Psychology trainee from Miami University may participate in the LEND program during certain years, with payment of $9,981 being invoiced per training year.

Travel: $5,500

Required Faculty travel: Support is requested in the amount of $2,750 for the Project Director and Associate Director/Training Director to attend the AUCD Annual Meeting (including LEND recipient meeting) in Washington, DC: $1,375/person. Support is requested in the amount of $2,750 for the Project Director and Associate Director/Training Director to attend the Autism CARES recipient meeting every other year in Washington, DC: $1,375/person.

Participant/Trainee Support Costs: $85,529

1. Tuition/Fees/Health Insurance: N/A
2. Stipends: $80,130 is requested for salary support provided to trainees who are CCHMC employees. See above under Other Personnel for description.
3. Travel: $5,400 is requested for trainee travel to the AUCD Annual Meeting or Disability Policy Seminar (DPS) in Washington, DC to engage with LEND network activities. $1,000 is requested for trainee travel to Columbus, Ohio for the Ohio Developmental Disabilities Awareness and Advocacy Day. This travel increases trainees' knowledge regarding current state policies regarding disability and increases leadership skills by providing the opportunity to meet with and educate state legislators.

Other Direct Costs: $15,111

1. Materials and Supplies: $9,971
   $5,000 is requested for program supplies to support the Interdisciplinary LEND Curriculum including StrengthsFinder, True North field books, Thomas Kilmann Inventory, Crucial Conversations book, and other leadership and disability culture books used throughout the year. $4,000 is requested to support hosting of the Empowering Families Symposium described in the CE section of the project narrative. This will include A/V equipment, accessibility accommodations, and other supplies as needed. There are no costs associated with other CE activities due to in-kind support. $971 is requested to cover subscription expenses for services such as Zoom, Canva, Padlet, Mentimeter, and Visme to offer virtual learning options and enhance learner experience. Publication Costs: N/A
2. ADP/Computer Services: N/A
3. Sub-awards/Consortiums/Contractual Costs: N/A
4. Equipment or Facility Rental/User Fees: N/A
5. Alterations and Renovations: N/A
6. Other: <u>Family Participants:</u> $1,340 is requested to reimburse family mentors who participate in the Family Mentoring Project (FMP) = 12 mentors at $100 each, as well as family members who serve as panelists as part of the Evidence-Based Case Discussion sessions = 7 panelists at $20 each.
7. Other: <u>Presenter Honorariums</u>: $2,000 is requested to provide honoraria for expert presenters for Leadership Seminar or CORE Courses who are outside of CCHMC.

Contracted Services Costs: $1,800

Consultant Services: **$1,800**

<u>Actors:</u> $1,800 is requested to fund professional actors from the local area to assist in a LEND session focused on sharing difficult news. Actors are being utilized in this session to provide the trainees with realistic role-playing experiences on how to approach discussing difficult topics with families. This session will be video-recorded and made available to future LEND trainees as well as a broader group of students and staff members.